



MILKI HEALTH SCIENCE TUTORIAL CENTER



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- ✓ **Barnoota Dabalataa Kennu**
- ✓ **Exit Exam/ COC'f isin qopheessuu**
- ✓ **Leenjiwwan gaggabaaboo kennuu fi kkf**



Teessoon: Magaalaa Naqamtee
Gamoo OSSA Darbii 3^{ffaa}

**Obstetrics I,II,III &gynecology final comprehensive examination for
4th year regular midwifery students**

Instructions

- 1) read the instructions carefully
- 2) switch of your mobile
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✓ 50MQC-50%

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Time allowed-60min

Prepared by: worku .m

PART-I –Multiple choices

**Choose the best answer from the give alternative and encircle
it (each 1pt)**

1. A 30years old gravid II Para I mother is presented at 38weeks +5days with marked decrease in fetal movement for two days. The attending gynecologist did the standard biophysical profile (BPP) for thirty minutes and found: 3 fetal gross body movement, 2 episode of extremity extension and subsequent return to flexion, 2 episode of rhythmic breathing lasting 30 seconds, 2 accelerations of 15 beats/min for 20 minute, a single vertical pocket of amniotic fluid that measures 2-cm . What will be the BPP score?
 - a. 6
 - b. 8
 - c. 9
 - d. 10 A
2. For the case scenario described under Q1, which one of the management option is appropriate
 - a. The biophysical profile is reassuring and the test can be repeated in a week timeo A
 - b. The biophysical profile is unequivocal and other means of fetal testing is recommended
 - c. The biophysical profile is poor and prompt cesarean delivery is recommended
 - d. The biophysical profile is poor and the test should be repeated in 4-6hours time
3. A 30years old mother who conceived after ovulation inductions has delivered to twin female babies at 36weeks spontaneously. The babies were in a separate sac (diamniotic) but single placenta (monochorionic). Which one of the following is true about this pregnancy?
 - a. It is unlikely to have single placenta and double amniotic cavity and most likely the placenta is also separate but fused
 - b. This is a monozygotic twinning with possible division within the first three days of fertilization
 - c. This is a dizygotic twinning that is possibly induced by ovulation induction

- d. This is a monozygotic twinning with possible separation between day 3 and day 8 A
4. A 32-year-old primigravid with regular 28-day menstrual cycle and on no prior contraception was having uneventful course of the pregnancy. Her gestational age from LNMP is 42weeks+2days. Term sized uterus, cervix 2cm dilated, 40% effacement, medium and posterior. Station is at -3cm and the pelvis is grossly adequate. Ultrasound was done and BPP is reassuring and estimated fetal weight of 3200gm. What is the Bishop Score?
- Unfavorable A(3)
 - Intermediate
 - Favorable
 - Incomplete
5. Referring to Q4 which one of the following management options is the best?
- Continue monitoring with weekly BPP
 - Induction of labor
 - Priming with prostaglandin and induction A
 - Cesarean delivery
6. A 24year- old Primigravida mother on ZDV 300mg twice a day since 28weeks of gestation was admitted at 37weeks with the cervical dilatation of 5cm. Membrane was intact. She brought the laboratory results: CD4 count 520, viral RNA 400copies/ml. Which one of the following is **in appropriate** measure?
- Start on ZDV 600mg, nevirapine 200mg and lamivudine 150mg A
 - Follow the progress of labor with partograph
 - Keep the membrane intact and avoid frequent pelvic examination
 - Continues ARV prophylaxis no need of additional drug because the lab result is normal
7. The post partum care of the case described under Q6 involves all of the following **except**
- Single dose of NVP and ZDV for 4weeks for the neonate A
 - Combivir (AZT+3TC) for 7days for the mother

- c. Dried blood spot (DBS) at 6weeks for the neonate to test for DNA PCR
 - d. Exclusive breast feeding or exclusive replacement feeding
8. All of the following are true about severe polyhydramnios, **except**
- a. Is diagnosed when single vertical pocket of amniotic fluid exceeds 24cm
 - b. Is usually due to severe fetal malformations
 - c. Can be managed by amniocentesis
 - d. Carries risk of abruption
 - e. Carries risk of cord compression A
9. Which of the following **is false** about Asymmetric intrauterine growth restriction (IUGR)
- a. The insult usually occurs in the last phasea
 - b. Chromosomal abnormalities are the common causes A
 - c. Is usually a benign condition
 - d. It mainly affects the cell size than cell number
10. A 25-year-old mother was admitted in second stage of labor and delivered to her second baby by SVD. Up on reviewing her chart her blood group and Rh is A-ve and that of her husband is A+ve. She took anti-D during her previous delivery and indirect Coombs test is negative. The blood group and Rh of the baby O+ve. Which one of the following **is true** about the case?
- a. It is impossible have O+ve baby if both parents are A blood type A
 - b. No need for anti-D as she received it and immunized during the previous delivery
 - c. The ABO compatibility in this case is protective for Rh isoimmunization
 - d. It is possible have O+ve baby if both parents are A blood type
11. The **most** definitive principal management of eclampsia is
- a. Delivery A
 - b. Drug management
 - c. control convulsion
 - d. mechanical management
12. Which of the following is not a dysmaturity syndrom
- a. Failure of fetal grow
 - d. Clear amniotic fluid A

- b. Loss of subcutaneous tissue e. Long nails
- c. Wrinkled skin

13. Accurate diagnosis of post term pregnancy is made by accurate dating ie if LNMP is supported by the following criteria :except

- a. +ve pregnancy test by 6wks from the LNMP
- b. Bimanual examination that in the second trimester to confirm gestation age A
- c. Fetal heart tone at 20wks by fetoscope
- d. Confirmatory by U/S prior to 26wks of gestational age

14. Which of the following **is not** true about determination of zygosity

- a. Twin- twin transfusion is almost common in dz A
- b. Twins of opposite sex are almost always DZ
- c. Monochorionic placentation indicate monozygosity
- E. Dichorionic twins of similar sex may be DZ or Mz

15. All are the indications of caesarean section for retained second twin **except**

- a. Fetal macrosomia d. borderline fetal heart sound
- b. cervix retracting and not dilating e Cord prolapse
- c. Hypotonic uterine dysfunction A

16. 45. The following are prerequisites for trial of scar, Vaginal Birth After Cesarean, **except**

- a. Clinically adequate pelvis
- b. Estimated fetal weight less than 4kg
- c. Previous successful vaginal delivery A
- d. Facility for managing rupture
- e. Maternal consent

17. All of the following are component of active management third stage labor **except**

- d. 10IU oxytocin IM or ergometrine 2mg IM or IV
- e. Controlled cord traction (CCT) with each contraction
- f. CCT when there is sign of placental separation A

- g. Uterine massage
- 18 . Failed vacuum is considered in all except
 - a. Failure to descend with each pull
 - b. More than two pop offs
 - c. Maternal refusal for the application
 - d. Failure to deliver after half an hour of application
 - e. none

19. The presenting diameter of mento anterior face presentation is

- A. Occipito frontal
- B. Sub occiptobregmatic
- C. Sub mentobregmatic A
- D. Sub mentovertical
- E. Biparietal

19. The best predictor for success of vaginal birth after cesarean section (VBAC) is

- A. Previous vaginal delivery
- B. Prior indication for Breech presentation
- C. Previous successful VBAC
- D. Non recurring
- E. Previous indication for fetal distress

20. What is **McRobert maneuver** to release shoulder dystocia

- A. Rotation of post shoulder to deliver anterior shoulder
- B. Abduction of shoulders
- C. Flexion of mother's knees and supra pubic pressure
- D. Rotation and extraction of anterior shoulder
- E. Repositioning of fetal head back in to the uterus and C/S

21. A woman 30 years old GII Para I EFW of 2kg, presents with face presentation. How do you manage her delivery?

- A. Induction of labor
- B. Internal rotation to make mentum anterior position
- C. Observation to allow spontaneous rotation
- D. C/S
- E. Forceps assisted delivery

22. Which criterion applies to low forceps?

- A. The fetal head leading point should be on station ≥ 2
- B. The fetal head leading point should be above station ≥ 2
- C. The fetal head is on the pelvic floor
- D. Sagittal suture is ant-post
- E. Rotation is less than 45 degree

23. In breech presentation what is the name of the maneuver. The shoulder is grasped by one hand and the legs are grasped by the other hand then the newborn is pulled toward mother's abdomen?

- A Pinard
 - B modified Prague
 - C Bracht
 - D Meuriceu
 - E Burns marshal
24. Vesico vaginal fistula following obstructed labor should be repaired
- A. 10 days later
 - B. Within 3 month A
 - C. At any time
 - D. 6 month
 - E. Immediately
25. In occiput posterior position
- A. Vaginal birth cannot occur
 - B. The woman should not give birth in squatting position
 - C. Vaccum extraction should not be performed
 - D. The expulsive phase may be prolonged A
 - E. The presenting diameter is mento vertical
26. A 23-year-old woman (gravida 2, para 2) calls her physician 7 days postpartum because she is concerned that she is still bleeding from the vagina. It would be appropriate to tell this woman that it is normal for bloody lochia to last up to
- A. 2 days A
 - B. 5 days
 - C. 8 days
 - D. 11 days
 - E. 14 days
27. . A 27-year-old has just had an ectopic pregnancy. Which of the following events would be most likely to predispose to ectopic pregnancy?
- A. Previous tubal surgery A
 - B. Pelvic inflammatory disease (PID)
 - C. Use of a contraceptive uterine device (IUD)
 - D. Induction of ovulation
 - E. Exposure in utero to diethylstilbestrol (DES)

28. The most important reason that PID must be recognized and treated promptly is prevention of:

- A. Infertility
- B. Ectopic Pelvic pain syndrome
- C. pregnancy
- D. Tubo-ovarian abscess
- E. Pelvic adhesive disease

29. Which of the following **is not** positive sign of pregnancy?

- A. Identification of fetal heart tones
- B. Positive pregnancy test
- C. Perception of fetal movement by the examiner
- D. Identification of fetus by ultrasound

30. Of the followings are component of reassuring fetal heart pattern **except**

- A. A baseline FHR of 120-160 bpm.
- B. Absence of FHR deceleration.
- C. Age appropriate FHR acceleration.
- D. Normal FHR variability(5-25 bpm).
- E. late deceleration phase

31. which one of the following **is not** Features of sinusoidal fetal heart rate pattern

- A. Normal base line HR with regular oscillations
- B. Amplitude of 5-15 bpm
- C. 2- 5 cycles per min frequency
- D. Fixed long term variability
- E. Absence of acceleration

32. Which of the following **is not important** for the Diagnosis of fetal distress.

- A. Moderate to severe deceleration with absent beat to beat variability.
- B. Early deceleration with absent beat to beat variability.
- C. Fetal bradycardia
- D. Hypoxia with metabolic acidosis

33. in which type of breech presentation Ischial tuberosities, sacrum and its spines, genitals and the anus are usually palpable during vaginal examination.

- A. frank breech
- B. Complete breech
- C. footling breech
- D. Extended breech

34. Refer urgently if the following specific breech-related conditions are identified: **except**

- A. Footling breech
 - B. Estimated fetal weight of > 3500 gm
 - C. Previous CS, Rh-immunization,
 - D. Elderly multiparous women
 - E. Uterine dysfunction, prolonged labor or failure to progress in labor
35. which one of the **following is not recommended** during Assisted Vaginal breech delivery

- A. Minimum manipulations
- B. Place patient in a lithotomy position
- C. Empty bladder
- D. Ascertain cervix is Partially dilated
- E. Bear down with contractions
- F. Deliver spontaneously up to the umbilicus

36. Descent Zero out of fifth means

- A. The head is floating
- B. The head is fixed
- C. Station is Zero
- D. The whole head has entered the pelvic cavity
- E. The head is crowning

37. A 21-year-old has difficulty voiding 6 h postpartum. The least likely cause is which of the following?

- A. Preeclampsia
- B. Infusion of oxytocin after delivery
- C. Vulvar hematoma

- D. Urethral trauma
- E. Use of general anesthesia

38. What anesthetic agent is most ideal for replacing an inverted uterus?

- A. Spinal analgesia
- B. Thiopental
- C. Succinylcholine
- D. Halothane

39. The treatment of metritis should include

- A. IV ampicillin or IV gentamicin or IV metronidazole
- B. iv ampicillin, plus iv gentamicin and iv metronidazole
- C. a combination of oral antibiotics
- D. a broad spectrum oral antibiotic

40. When performing abdominal aortic compression to control postpartum hemorrhage, the point of compression is

- A. just below and slightly to the right of the umbilicus
- B. just below and slightly to the left of the umbilicus
- C. just above and slightly to the right of the umbilicus
- D. just above and slightly to the left of the umbilicus A

41. Findings diagnostic of cephalopelvic disproportion are

- A. cervical dilation plotted to the right of the alert line on the partograph
- B. uterine contractions in the latent phase with an unengaged fetal head
- C. Secondary arrest of descent of the head in the presence of good contraction A
- D. grade 3 molding of the fetal head

42. The most common clinical presentation of ectopic pregnancy is

- A. Pain A
- C. Amenorrhea
- B. Bleeding
- D. Adenexal mass

43. The most common clinical presentation of ectopic pregnancy is

- A. Pain
- C. Amenorrhea
- B. Bleeding
- D. Adenexal mass

44.. Single out, the clinical sign which is an indication of expectant

Management of ectopic pregnancy

- A. HCG level <1000 mlu/ml A
- C. Non viable fetus.

- B. HCG Level <1500mIU/ml D. Unruptured tube
45. One of the following is true about pelvic inflammatory disease
- A. 85 % of PID occurred following Gynecological & obstetrical procedure
 - B. 90 % of the causative organism are normal flora of the vagina
 - C. Hematological spread is common
 - D. PID is common during pregnancy b/c of maternal immunity
46. One of the following is **not true** about spontaneous abortion
- A. Mostly occur in the first trimester
 - B. The Incidence decrease when gestational age increase
 - C. Risk decrease with increasing maternal age
 - D. Risk increase with increasing paternal age
47. Which classification is the best for management of abortion?
- A. Etiological classification
 - B. Clinical classification
 - C. depend on its GA
 - D. depend on its legality
48. Which of the following **is true** about the anatomic changes of pueriperium
- A. Uterus changes from 1000gm to 100gm
 - B. The endometrial lining regenerates rapidly
 - C. Uterine fundus palpates little below the symphysis
 - D. The uterus is palpable at 6 weeks A
49. when do you say asphyxia of the fetus based on the APGARscore interpretation.
- A. APGARscore less than 04 in the first minut
 - B. APGARscore less than 05 in the first minut
 - C. APGARscore less than 05 in the last 5th minut
 - D. APGARscore less than 07 in the last 5th minut
50. One of the following fetal component of partograph is record every **30** minutes
- A. Fetal heart beat A D.contraction
 - B. Maternal pulse

C. Blood pressure

14. A 40-year-old Para VII woman presented family planning clinic with her husband for delaying her next pregnancy. After health care provider counsel them her husband agreed for vasectomy.

What is the most important information needed to perform the procedure?

- (A) Diabetes mellitus
(B) HIV status
(C) History of previous surgery
(D) Their plan to have other baby

15. A 21-year-old lady comes to family planning clinic to use contraceptive. After counseling, she decides to use a family planning method which has importance to prevent pregnancy as well as HIV/AIDS.

What would be the best method for this woman?

- (A) Cervical cap
(B) Condom
(C) Diaphragm
(D) Spermicidal

16. A 32-year-old woman comes to your health center to use family planning method. She had no history of diabetes and hypertension. After counseling, she decides to use a regular method of contraceptive method which has low estrogen and progesterone hormone.

What would be most likely side effect of this method?

- (A) Bleeding patterns changes
(B) Risk of Endometrial cancer
(C) Cause of ovarian cancer
(D) Risk of symptomatic PID

17. A 32-year-old Para V woman presented to a family planning clinic after six days of IUCD insertion. At the time of insertion the procedure was very painful. Currently she has abdominal pain and negative urine HCG.

What is the best diagnosis for this client?

- (A) Partial expulsion of the strings
(B) Uterine perforation
(C) Missing of Strings
(D) Uterine mass

18. A 44-year-old woman presented at family planning clinic for delaying her future pregnancy. She had seven children; hence she wants to delay her pregnancy. On examination her blood pressure was 140/100mmHg and has no abnormal vaginal discharge. The health provider counseled the woman about the available contraceptive methods.

What is the best method for this woman?

- (A) Implants
(B) Intrauterine devices
(C) Monthly injectables
(D) Tubal ligation

55. A 30-year-old gravid HIV positive mother with gestational age of 33 weeks came to ANC clinic with complain of head ache for two days duration. History revealed that the head ache was continuous and associated with fever and sweating for two days duration. She had also nausea and vomiting including loss of appetite. Lab investigation was reactive for PF. Physical examination showed that: BP 90/60 mmHg, PR 90 B/min, RR 24, T° 38°C and MUAC 18 cm.

What nutritional care plan should be applied for this patient?

- (A) Nutritional care plan A
(B) Nutritional care plan B
(C) Nutritional care plan C
(D) Nutritional care plan D

56. A 35-year-old HIV positive gravid woman came to ANC unit for follow up based on her appointment. She started ART drugs one month back. During the assessment of her adherence level it is found that she missed two doses ART drugs because of illness.

What is the ART level of adherence?

- (A) Excellent
(B) Good
(C) Fair
(D) Poor

57. A gravid woman who is on ART for two years came for ANC visit on her 28 weeks of gestation. On history, it is found that her husband who is living with her had pulmonary TB and is currently on anti TB drugs. She had no fever, cough and night sweating. The Physical examination shows appropriate weight increment according to gestational age.

What is the best drug for this client?

- (A) Isoniazid
(B) Erythromycin
(C) Co-trimoxazole
(D) Anti-TB drug regimen

58. A gravid mother who is linked to PMTCT clinic one month back complains of having a problem taking the ART drugs. During revision of history it is revealed that her husband is not supportive of her taking the drugs.

What is the best management option for this problem?

- (A) Litigation
(B) Stop the drug until he got agreed
(C) Provide scientific information
(D) Take the drug without his knowledge

43. A 36-weeks Gravida III Para II woman admitted to a hospital with a diagnosis of ante partum hemorrhage secondary to placenta previa. Ultrasound scanning result revealed that the placenta located posteriorly and reaches the margin of the internal cervical os but does not cover it.

What is the degree of placenta previa based on ultrasound finding?

- (A) Type -I (C) Type-III
(B) Type-II (D) Type-IV

44. A 10-week Primigravida woman is admitted in a hospital with a diagnosis of hyperemesis gravidarum. She had history of nausea and villous vomiting with an episode of three times per day. And laboratory investigation is sent for urinalysis.

What is the most likely laboratory finding for this problem?

- (A) High chloride level (C) Presence of ketone
(B) Low specific gravity (D) Presence of protein

45. A 42-year-old, Gravida II Para I with gestational age 22-week by LNMP came to the hospital with chief complaint of blurring of vision and epigastric pain for two days duration. She had associated frontal headache which is unable to relieve with regular analgesics. The urine output measures 70 ml /hour. The diastolic blood pressure also 110 mm Hg on two occasions. In addition, urinalyses showed trace Proteinuria.

What is the most likely diagnosis?

- (A) Eclampsia (C) Severe pre-eclampsia
(B) Mild pre-eclampsia (D) Gestational hypertension

46. A 30-week primigravida woman presented with abdominal distention started three weeks back. Abdomen is markedly enlarged and looks globular with fullness at the flanks. The fundal height is 38 weeks with fluid thrill. The ultrasound result showed that the amniotic fluid volume is 11 cm by the largest vertical pocket and absence of fetal skull.

What is the most likely cause of abdominal distension?

- (A) Anencephaly (C) Esophageal atresia
(B) Hydropsfetalis (D) Open spinal bifida

47. A 32 -week Gravida IV Para III woman came in the nearest hospital for her 2nd ANC visit. She has flank pain and shortness of breathing for the last two weeks. Physical examination revealed a 40 weeks sized gravid uterus with barrel shaped abdomen. Many fetal parts are palpable. Ultrasound result showed that amniotic fluid with the largest vertical pocket measures 6cm.

What will be the most likely diagnosis?

- (A) Fibroid (C) Polyhydraminous
(B) Ovarian cyst (D) Multiple pregnancy

89. A multi Gravida woman with GA of 40 weeks admitted at 2:00 pm with the diagnosis of active first stage of labor. On P/E cervix 4 cm dilated, membrane intact, contraction= 3c/10'/45" and FHB was 144 BPM. At 6: 00 pm the woman was re-evaluated and there was no fetal and maternal abnormality.

What will be the minimum expected cervical dilation at this time?

- (A) 5cm (C) 7cm
(B) 6cm (D) 8cm

90. A- Gravida II Para I woman came with a complaint of pushing down sensation for 8 hours duration. After assessing her and fetal condition, she was admitted with a diagnosis of second stage of labor. The midwife was attending the delivery. After delivery of the fetal head there was tightly attached umbilical cord around the neck.

What will be the immediate management?

- (A) Doing episiotomy to deliver easily
(B) Double clamp and cut the cord
(C) Facilitate delivery with vacuum
(D) Slip it over the fetus's head

91. A 34-year-old woman gave birth to a 2.8 kg male neonate at health institution. 10 IU Oxytocine was given intramuscularly and controlled cord traction was tried. But the placenta was not delivered within 15 minutes.

What will be the next management?

- (A) Observe for additional 15 minutes
(B) Repeat another dose of Oxytocine
(C) Manual removal of placenta
(D) Massage the uterus

92. A 32-year-old woman gave birth to a 3.4 kg male neonate at health center. 10 IU Oxytocine was given intramuscularly and controlled cord traction was tried. But the placenta was not delivered within 30 minutes. There is no sign of PPH.

What will be the next management?

- (A) Manual removal of placenta
(B) Refer for further management
(C) Give additional 10 IU Oxytocine IM
(D) Continuing controlled cord traction for 30 minutes

64. A 37-weeks Gravida II Para I woman was admitted in obstetrics ward for the diagnosis of active first stage of labor. She had 4 antenatal care visits and all recommended laboratory investigation has been done.

What laboratory investigation is most relevant at admission?

- (A) Blood group & Rh
- (B) VDRL
- (C) Hepatitis B
- (D) Hematocrit

65. A pregnant woman gave birth with spontaneous vaginal delivery. After examination of placenta, the health care provider evaluates the perineum and extended vaginal tear to the rectal mucosa, muscle and anal sphincter was identified. What is the degree of tear?

- (A) 1st
- (B) 2nd
- (C) 3rd
- (D) 4th

66. A 25-year-old primigravida woman presented with complaint of pushing down pain. On pelvic examination one foot inferior to the buttock was identified..

Which operative delivery is best indicated for this woman?

- (A) Cesarean
- (B) Vacuum
- (C) Forceps
- (D) Version

67. A 35-year-old G-III-P-II woman presented with true labor at 38+5 weeks of gestation at 2 o'clock local time. On physical examination cervical dilation was 6 cm, fetal position was left occiputo-anterior and amniotic membrane was ruptured. After re-evaluating her at 6 o'clock you found full cervical dilatation. She has not been given any regional anesthesia. But at 7:30 o'clock she is exhausted to push adequately and still in labor with station of +1 and FHB 160 beats/minute.

What is the most likely operative delivery indicated for this woman?

- (A) Cesarean
- (B) Vacuum
- (C) Forceps
- (D) Version

68. A primigravida woman is in 2nd stage of labor at 33+2 weeks of gestation. On P/E uterine contraction-3/10'/40-50'', station- 0, FHB-120 beats/minute, BP 120/110 and protein urea of 3+. Shortening 2nd stage of labor was mandatory.

What is the most likely operative delivery indicated for this woman?

- (A) Cesarean
- (B) Vacuum
- (C) Forceps
- (D) Version

98. A 28-year-old Gravida III Para II woman with GA of 40 weeks admitted at 2:00 pm with the diagnosis of active first stage of labor. At admission cervix was 5 cm dilated, amniotic membrane was intact. She had 3 uterine contractions in 10 minute lasting 40 seconds each. Partograph was started. At 6:00 pm she was re-evaluated and cervical dilatation was 9 cm.

Where will be cervical dilation plotted on the partograph?

- (A) At alert line
(B) At the action line
(C) At the left of the alert line
(D) At the right of the alert line
99. A 38-weeks Gravida I Para 0 women was admitted in obstetrics ward for the diagnosis of active first stage of labor. After 4 hours of admission, she has 3 strong contractions, decent 1/5th; fetal heart beat (FHB) 140 beats/minute, cervix 10 cm dilated, vertex presentation, occiputo-anterior position and station +1. After two hours of full cervical dilatation, FHB fluctuates between 160 and 180/minute and station 2+.

What is the most appropriate decision that a health care provider should take?

- (A) Perform episiotomy and use fundal pressure
(B) Position the mother left lateral to improve fetal heart beat
(C) Secure IV line and refer the mother to hospital
(D) Use vacuum extractor to assist vaginal delivery
100. A 37-year-old Gravida V Para IV woman came to the hospital at 37 weeks of gestation with a complaint of pushing down pain for 12 hours duration. on physical examination BP 80/10 mmhg, pulse rate 120/ minute, and she was disoriented, abdomen was distended, uterine contraction is not detectable and fetal part is easily palpable abdominally. Laparotomy was decided to manage.

What makes laparotomy to be preferable?

- (A) Disorientation
(B) Uterine rupture
(C) Obstructed labor
(D) Poor uterine contraction

THE END

81. A 38-weeks Gravida II Para I woman was admitted in obstetrics ward for the diagnosis of active first stage of labor? On per vaginal examination, cervix 8 cm dilated, membrane ruptured, station -1, cord is easily palpable by fingers before the fetal presenting part. What is the most likely diagnosis?
- (A) Cord prolapse
(B) Occult prolapse
(C) Cord presentation
(D) Cord entanglement
82. A 37-weeks Gravida IV Para III woman was admitted in obstetrics ward for the diagnosis of active first stage of labor. On physical examination uterine contraction is 5/10'/40-55'', the cervix dilated fully, station is -2, position is left occiputo anterior, membrane ruptured, uterus is tender, and FHB 130beats/minute. Suddenly, the uterine contraction ceases, the woman bleeds profusely and fetal heart beat absent. What is the diagnosis?
- (A) Abruptio placenta
(B) Uterine rupture
(C) Placenta previa
(D) Vasa previa
83. A 38-weeks Gravida III Para II woman came with a complaint of profuse vaginal bleeding for 2 hours duration. On physical examination BP 80/60 mmhg, PR 120 beat/minute, there is no uterine contraction but severe abdominal tenderness, non-reassurance fetal heart beat, fetal part is easily palpable abdominally, cervix was 9 cm dilated and there is non-clotted vaginal bleeding. Based on the findings uterine rupture was diagnosed. What is the primary clinical finding used for the diagnosis?
- (A) Feto-maternal distress
(B) Profuse vaginal bleeding
(C) Diffuse abdominal tenderness
(D) Abdominal palpation of fetal parts
84. A 26year-old pregnant mother is presented with complaint of passage of fluid through the vagina at 36+5 weeks of gestation. On physical examination Temp: 38 Oc, RR: 30 breath/minute, Pulse 120 beat/minute and FHB 185 beat/minute, The uterus was tender, cervix admits tip of finger, amniotic membrane is ruptured and there is foul smelling vaginal discharge. What is the diagnosis?
- (A) Term PROM with chorio-amnionitis
(B) Term PROM without chorio-amnionitis
(C) Preterm PROM with chorio-amnionitis
(D) Preterm PROM without chorio-amnionitis

48. A 37-year-old Primigravida lady came to gynecology OPD at GA of 12-weeks with a complaint of repetitive vomiting. History revealed that she is unable to perform regular activities. Physical examination findings include BP-90/50 mmHg and PR-100 beat per minute and dry coated tongue. Laboratory investigation shows Urine ketone +3. What is the most appropriate initial management in this case?
- (A) Rehydrate with IV fluid
 - (B) Start nasogastric feedings
 - (C) Provide IV antiemetic drugs
 - (D) Assess serum level of electrolyte
49. A 28-year-old Gravida-IV and Para- III mother presented with absence of fetal movements for one day duration at gestational age of 32 weeks. Physical examination revealed that fundal height of 30 cm and fetal heart beat not heard on fetoscope. What is the most likely next management for this client?
- (A) Conduct laboratory test
 - (B) Perform ultrasound scan
 - (C) Tell her that her fetus is dead
 - (D) Admit for observation
50. A 32-year-old Gravida-2, Para-1 woman on her 32 weeks of gestation presents with a complaint of fast growing abdomen associated with fatigue and dyspnea. Abdominal examination shows fundal height of -36cm and Fetal heart beat is not audible with fetoscope. Ultrasound shows singleton live fetus with cranium absent and Large Vertical Pocket -10cm. What is the most possible management?
- (A) Terminate the pregnancy
 - (B) Advise to take bed rest
 - (C) Perform amnioreduction
 - (D) Provide indomethacin
51. A 27-year-old Gravida-III and Para-II mother from remote area is presented with one episode of vaginal bleeding at GA of 34 weeks. The bleeding is dark red and soaks her underwear only. Physical examination showed BP-110/70mmHg, PR- 88 beat per minute, and RR-20 breath per minute, T° -36 $^{\circ}$ C, no uterine contraction and Fetal heart rate - 136beat per minute. Ultrasound examination revealed placenta implanted on uterine fundus. What is the most likely management that should come next for this client?
- (A) Admit for observation
 - (B) Reassure and send home
 - (C) Advise and appoint her after a week
 - (D) Send her home after 4 hours evaluation

33. A 36-weeks Gravida III Para II woman came to nearest hospital with complaint of vaginal bleeding for three days duration. The bleeding was painless, bright red, sudden and scanty in amount. She has no history of falling accident but had one cesarean scar on her last pregnancy.

Which obstetric palpation should be avoided until the cause of bleeding is ruled out?

- (A) Deep palpation (C) Fundal palpation
(B) Pelvic palpation (D) Lateral palpation

34. A 34-year-old pregnant woman came for 1st ANC visit. She is counseled and tested for HIV and found to be reactive.

What additional laboratory investigation should be ordered for this woman?

- (A) Complete blood count (C) VDRL
(B) CD4 count (D) HCT

35. A 36-week Gravida II and Para I client came for second visit with a complaint of discomfort under the ribs for one week duration. Abdominal examination revealed presence of round hard and smooth mass occupying the fundus and soft, broad, indefinite and non ballotable mass occupying the lower pole of the uterus.

What is the most likely fetal presentation?

- (A) Breech (C) Cephalic
(B) Brow (D) Face

36. A midwife wanted to conduct a research to detect nutritional deficiency by taking blood samples from pregnant woman in the community. During analysis of the blood samples showed microcytic hypo-chromic red blood cells in around 10% of the samples.

What nutritional deficiency is suspected for this finding?

- (A) Iron deficiency (C) Folic acid deficiency
(B) Calcium deficiency (D) Vitamin B₁₂ deficiency

37. A pregnant woman on her 25 weeks of gestation comes to the health center for her ANC visit. History revealed that her husband was recently diagnosed and starts treatment for Hepatitis B infection. Laboratory result showed that she is free from Hepatitis B infection.

What is the most possible management for this client? When should she get the first vaccine?

- (A) Re-assess Hepatitis B status after three month
(B) Initiate Hepatitis B vaccination immediately
(C) Initiate Hepatitis B treatment immediately
(D) Initiate Hepatitis B treatment after delivery

52. A 38-year-old primigravida lady presented with severe headache at gestational age of 38-week. She has blurring of vision and epigastric pain. Physical examination revealed BP-150/100mmHg, PR- 88beat per minute, RR-20breath per minute and no uterine contraction. Laboratory investigation indicated +2urine protein.

What would be the most appropriate initial management for this woman?

- (A) Induce labor
(B) Administer MgSo₄
(C) Administer Hydralazine
(D) Admit her to high risk ward
53. A 37-year-old Gravida-V and Para-IV came to ANC clinic for 4th visit at GA of 37 weeks. Physical examination revealed:
- stable vital sign
 - same weight measurement as previous visit
 - Fundal height-31cm
 - FHB -136/min
 - No uterine contraction
 - Cephalic presentation.

Vaginal examination indicated the cervix is soft, 1cm dilated, anterior position and 50% effaced. Fetal head is at station 0.

What is the most appropriate management for this woman?

- (A) Prepare her for c/s
(B) Admit her to high risk ward
(C) Perform rupture of membrane
(D) Send home with advise to come when labor starts
54. A 27-year-old primigravida lady comes to a clinic for 1st ANC visit at gestational age of 22weeks. Physical examination revealed stable vital signs, fundal height-28cm and two fetal heart rate (FHR) heard (FHR1=132 and FHR 2=144beat per minute).

What is the most possible management for this woman?

- (A) Admit for observation
(B) Appoint her after 8 weeks
(C) Advice on additional calorie intake
(D) Perform ante partum fetal surveillance

19. A 24-year-old woman comes to family planning clinic to delay the next pregnancy. She registered her menstrual cycle from September up to February which was 21, 28, 23, 26, 27 and 33 respectively. She was counseled about each method and she prefers to use natural methods of family planning.

What would be the best method for this woman?

- (A) Lactation amenorrhea method
- (B) Calendar rhythm method
- (C) Standard day method
- (D) Withdrawal method

20. A 26-year-old woman come with complain of nausea, irregular bleeding, breast tenderness, headache and weight change to your hospital. The woman has started to use combined oral contraceptive pills three weeks ago.

What would be the appropriate management for this woman?

- (A) Stop using combined oral contraceptive pills and change by another method
- (B) Take each pill at any time every day to help reduce irregular bleeding
- (C) Stop using COCs because side effects are the sign of illness
- (D) Take pills with food or at bedtime to help avoid nausea.

21. A 21-year-old woman came to family planning clinic to prevent pregnancy after she got raped six days ago.

What would be the best emergency contraceptive method for this woman?

- (A) Ulipristal acetate
- (B) Cooper bearing IUD
- (C) One dose regimen of COC
- (D) Two dose regimen of POP

22. A 23-year-old woman came to family planning clinic to use emergency contraceptive pills after having unprotected sexual contact before three days. A health care provider counseled her and she decided to use copper bearing Intrauterine device

What would be the best time to start this method for this woman?

- (A) After 14 days with backup
- (B) After seven days with back up
- (C) Within 14 days without back up
- (D) Within seven days without back up

23. A 40 year-old Para VI woman presented at family planning clinic after four years of LNG-IUCD insertion. On examination she was infected by HIV, currently develops advanced clinical disease and no diabetes mellitus.

What is the best management for this client?

- (A) Leave the LNG-IUCD and monitor for pelvic inflammatory disease
- (B) Leave the LNG-IUCD and monitor for sexually transmitted disease
- (C) Remove the LNG-IUCD and monitor for pelvic inflammatory disease
- (D) Remove the LNG-IUCD and monitor for sexually transmitted disease

69. A 26-year-old Gravida-II Para-I woman is admitted with the diagnosis of true labor at 3 o'clock local time. The woman has no bad obstetric history. At admission cervical dilatation was 4cm, amniotic membrane was ruptured, station -2, pelvic was adequate, uterine contraction 2/10'/20-25", and FHB 140beat/minute. During re-evaluation at 7 o'clock cervical dilatation was 5cm, station -1, uterine contraction 2/10'/30-35" and FHB 130bpm.

What next step is expected to be done?

- (A) Induction (C) Cesarean delivery
(B) Augmentation (D) Vacuum delivery

70. A Gravida II Para I woman came with a complaint of pushing down pain for 10 hours duration. Physical examination revealed 8 cm dilated edematous cervix, amniotic membrane is ruptured, prolapsed cord. U/S finding showed amniotic fluid index of 4cm and low placental attachment. Footling breech was diagnosed.

What clinical feature associated with the diagnosis?

- (A) Oligohydramnious (C) Prolapsed cord
(B) Edematous cervix (D) Placenta previa

71. A Gravida I Para O woman with gestational age of 38 weeks was admitted in obstetrics ward for the diagnosis of second stage of labor. On physical examination BP 110/70mmhg, PR 82 b/m, RR 23 b/m, adequate uterine contraction and FHB 140 b/m. After 1 hour of admission she gave birth to a 3.8 kg female neonate. Following expulsion of the placenta, profused vaginal bleeding has occurred. While evaluating her, BP-85/40mmhg, Pulse-130(weak), RR-35breath/minute, pale conjunctiva, and scanty urine output. Shock was diagnosed.

What is the most likely clinical finding that helped to diagnose shock?

- (A) Rapid breathing (C) Low blood pressure
(B) Pale conjunctiva (D) Scanty urine output

72. A pregnant woman came to delivery ward with complaint of pushing down pain for the last 4 hours on 21/06/2011 E.C. Her last normal menstrual period was from June 1 - 6, 2010 E.C. On physical examination uterine contraction 3/10'/25-35", cervical dilation 3 cm.

What would be the most likely diagnosis?

- (A) Post-term labor (C) Preterm labor
(B) Post date labor (D) Term labor

77. A 38-year Gravida I Para 0 women was admitted in obstetrics ward for the diagnosis of active first stage of labor at 4:00 AM. She is exhausted for pushing long period of time. A midwife follows the labor with partograph and the fetal heart beat pattern line indicates that the last plotted point is 90 b/m in the absence of uterine contraction.

What is the most likely diagnosis?

- (A) Fetal distress
(B) Normal heart beat
(C) Tachycardic heart beat
(D) Reassuring fetal heart beat

78. A 21-year-old primi-gravid woman with gestational age of 38 weeks came with a complaint of pushing down pain for 25 hours duration. On physical examination uterine contraction 3/10'/50-55'', descent 4/5, FHB 135 beats/minute. On vaginal examination the cervix dilates fully, station is -3, position is right occiputo anterior and the parietal bone overhangs the symphysis pubis displacing the thumb and you feel prominent ischial spine. Based on this finding prolonged labor was diagnosed.

What is the most likely cause of prolonged labor?

- (A) Mal position and Mal presentation
(B) Cephalo-pelvic disproportion
(C) Uterine dysfunction
(D) Poor maternal effort

79. A 30-year-old G-II-P-I woman is admitted in maternity ward for the diagnosis of active first stage of labor. On pelvic examination cervix is 5 cm dilated, fetal ischial tuberosities, sacrum and anus was appreciated but no other part of the fetus is palpable.

What type of breech presentation is it?

- (A) Incomplete
(B) Complete breech
(C) Footling breech
(D) Frank breech

80. A 28-year-old Gravida I Para 0 woman presented unconsciously to health facility at 37+3 weeks of gestation. On physical examination, BP 80/50 mmhg, PR 100 BM but week, RR 30 breaths/minute and cold clammy skin. Cervix was 5 cm dilated and there was profuse non clotted vaginal bleeding. After thorough evaluation shock was diagnosed.

What type of shock is it?

- (A) Septic
(B) Cardiogenic
(C) Anaphylactic
(D) Hypovolemic

43. A 36-weeks Gravida III Para II woman admitted to a hospital with a diagnosis of ante partum hemorrhage secondary to placenta previa. Ultrasound scanning result revealed that the placenta located posteriorly and reaches the margin of the internal cervical os but does not cover it.

What is the degree of placenta previa based on ultrasound finding?

- (A) Type -I (C) Type-III
(B) Type-II (D) Type-IV

44. A 10-week Primigravida woman is admitted in a hospital with a diagnosis of hyperemesis gravidarum. She had history of nausea and villous vomiting with an episode of three times per day. And laboratory investigation is sent for urinalysis.

What is the most likely laboratory finding for this problem?

- (A) High chloride level (C) Presence of ketone
(B) Low specific gravity (D) Presence of protein

45. A 42-year-old, Gravida II Para I with gestational age 22-week by LNMP came to the hospital with chief complaint of blurring of vision and epigastric pain for two days duration. She had associated frontal headache which is unable to relieve with regular analgesics. The urine output measures 70 ml /hour. The diastolic blood pressure also 110 mm Hg on two occasions. In addition, urinalyses showed trace Proteinuria.

What is the most likely diagnosis?

- (A) Eclampsia (C) Severe pre-eclampsia
(B) Mild pre-eclampsia (D) Gestational hypertension

46. A 30-week primigravida woman presented with abdominal distention started three weeks back. Abdomen is markedly enlarged and looks globular with fullness at the flanks. The fundal height is 38 weeks with fluid thrill. The ultrasound result showed that the amniotic fluid volume is 11 cm by the largest vertical pocket and absence of fetal skull.

What is the most likely cause of abdominal distension?

- (A) Anencephaly (C) Esophageal atresia
(B) Hydropsfetalis (D) Open spinal bifida

47. A 32 -week Gravida IV Para III woman came in the nearest hospital for her 2nd ANC visit. She has flank pain and shortness of breathing for the last two weeks. Physical examination revealed a 40 weeks sized gravid uterus with barrel shaped abdomen. Many fetal parts are palpable. Ultrasound result showed that amniotic fluid with the largest vertical pocket measures 6cm.

What will be the most likely diagnosis?

- (A) Fibroid (C) Polyhydraminous
(B) Ovarian cyst (D) Multiple pregnancy

9. A 23-year-old primigravida lady came to a hospital with a complaint of vaginal bleeding of one day duration at a gestational age of 10 weeks and she also had an abdominal pain. On physical examination, the midwife finds uterus is not palpable abdominally, cervix admits 6f finger, and there is active bleeding.

What is the most appropriate management option?

- (A) Medical
(B) Dilatation and Curettage

- (C) Evacuation and Curettage
(D) Manual Vacuum Aspiration

10. A mother came to a hospital with a complaint of involuntary urine loss of two weeks duration. She also had fever, headache and urinary frequency. On physical examination, her pulse rate is 100/min, temperature is 38.7°C , and involuntary urine leakage. Urinalysis showed many WBC in urine.

What is the initial management for this patient?

- (A) Give antibiotic drugs
(B) Counsel about life style modification
(C) Surgical management of the incontinence
(D) Appoint the mother to return when the problem is worse

11. A mother with molar pregnancy is treated with suction curettage after being amenorrheic for the last 10 weeks. On the procedure, evacuation completeness was assured with ultrasound. Then the patient is discharged after her vital sign become in the normal range.

How often the serum β -hCG level should be assessed for this woman?

- (A) Every week until undetectable
(B) Every one month until undetectable
(C) Every three week until undetectable
(D) Every three months until undetectable

12. A 25-year-old, Para-II woman came to a hospital with a complaint of absence of menses for six months duration. On investigation, she is negative for serum β -hCG. Two weeks back she took oral progesterone but the menses did not start.

What is the most appropriate next management option for this client?

- (A) Give oral estrogen
(B) Repeat progesterone

- (C) Do chromosomal analysis
(D) Measure serum gonadotropins

14. A 40-year-old Para VII woman presented family planning clinic with her husband for delaying her next pregnancy. After health care provider counsel them her husband agreed for vasectomy.

What is the most important information needed to perform the procedure?

- (A) Diabetes mellitus (C) History of previous surgery
(B) HIV status (D) Their plan to have other baby

15. A 21-year-old lady comes to family planning clinic to use contraceptive. After counseling, she decides to use a family planning method which has importance to prevent pregnancy as well as HIV/AIDS.

What would be the best method for this woman?

- (A) Cervical cap (C) Diaphragm
(B) Condom (D) Spermicidal

16. A 32-year-old woman comes to your health center to use family planning method. She had no history of diabetes and hypertension. After counseling, she decides to use a regular method of contraceptive method which has low estrogen and progesterone hormone.

What would be most likely side effect of this method?

- (A) Bleeding patterns changes (C) Cause of ovarian cancer
(B) Risk of Endometrial cancer (D) Risk of symptomatic PID

17. A 32-year-old Para V woman presented to a family planning clinic after six days of IUCD insertion. At the time of insertion the procedure was very painful. Currently she has abdominal pain and negative urine HCG.

What is the best diagnosis for this client?

- (A) Partial expulsion of the strings (C) Missing of Strings
(B) Uterine perforation (D) Uterine mass

18. A 44 year-old woman presented at family planning clinic for delaying her future pregnancy. She had seven children; hence she wants to delay her pregnancy. On examination her blood pressure was 140/100mmHg and has no abnormal vaginal discharge. The health provider counseled the woman about the available contraceptive methods.

What is the best method for this woman?

- (A) Implants (C) Monthly injectables
(B) Intrauterine devices (D) Tubal ligation

48. A 37-year-old Primigravida lady came to gynecology OPD at GA of 12-weeks with a complaint of repetitive vomiting. History revealed that she is unable to perform regular activities. Physical examination findings include BP-90/50 mmHg and PR-100 beat per minute and dry coated tongue. Laboratory investigation shows Urine ketone +3. What is the most appropriate initial management in this case?
- Rehydrate with IV fluid
 - Start nasogastric feedings
 - Provide IV antiemetic drugs
 - Assess serum level of electrolyte
49. A 28-year-old Gravida-IV and Para- III mother presented with absence of fetal movements for one day duration at gestational age of 32 weeks. Physical examination revealed that fundal height of 30 cm and fetal heart beat not heard on fetoscope. What is the most likely next management for this client?
- Conduct laboratory test
 - Perform ultrasound scan
 - Tell her that her fetus is dead
 - Admit for observation
50. A 32-year-old Gravida-2, Para-1 woman on her 32 weeks of gestation presents with a complaint of fast growing abdomen associated with fatigue and dyspnea. Abdominal examination shows fundal height of -36cm and Fetal heart beat is not audible with fetoscope. Ultrasound shows singleton live fetus with cranium absent and Large Vertical Pocket -10cm. What is the most possible management?
- Terminate the pregnancy
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51. A 27-year-old Gravida-III and Para-II mother from remote area is presented with one episode of vaginal bleeding at GA of 34 weeks. The bleeding is dark red and soaks her underwear only. Physical examination showed BP-110/70mmHg, PR- 88 beat per minute, and RR-20 breath per minute, T° -36 $^{\circ}$ C, no uterine contraction and Fetal heart rate - 136beat per minute. Ultrasound examination revealed placenta implanted on uterine fundus. What is the most likely management that should come next for this client?
- Admit for observation
 - Reassure and send home
 - Advise and appoint her after a week
 - Send her home after 4 hours evaluation

52. A 38-year-old primigravida lady presented with severe headache at gestational age of 38-week. She has blurring of vision and epigastric pain. Physical examination revealed BP-150/100mmHg, PR- 88beat per minute, RR-20breath per minute and no uterine contraction. Laboratory investigation indicated +2urine protein.

What would be the most appropriate initial management for this woman?

- (A) Induce labor (C) Administer Hydralazine
(B) Administer MgSo₄ (D) Admit her to high risk ward

53. A 37-year-old Gravida-V and Para-IV came to ANC clinic for 4th visit at GA of 37 weeks. Physical examination revealed:

- stable vital sign
- same weight measurement as previous visit
- Fundal height-31cm
- FHB -136/min
- No uterine contraction
- Cephalic presentation.

Vaginal examination indicated the cervix is soft, 1cm dilated, anterior position and 50% effaced. Fetal head is at station 0.

What is the most appropriate management for this woman?

- (A) Prepare her for c/s
(B) Admit her to high risk ward
(C) Perform rupture of membrane
(D) Send home with advise to come when labor starts

54. A 27-year-old primigravida lady comes to a clinic for 1st ANC visit at gestational age of 22weeks. Physical examination revealed stable vital signs, fundal height-28cm and two fetal heart rate (FHR) heard (FHR1=132 and FHR 2=144beat per minute).

What is the most possible management for this woman?

- (A) Admit for observation
(B) Appoint her after 8 weeks
(C) Advice on additional calorie intake
(D) Perform ante partum fetal surveillance

LET CODE: 824

27. A Para-II woman came for post natal care after delivery of her baby 18 days ago. On assessment, she gave birth at this facility but had not received any antenatal care and she declined HIV testing during labor but then agreed to be tested after childbirth. Blood was taken for testing to detect HIV antibodies.

What is the most appropriate initial test for the woman?

- (A) ELISA test
(B) DNA-PCR test
(C) Rapid HIV test
(D) Western Blot test

28. A 25-year-old multigravida woman has presented to labor and delivery room with passing down pain. Her serologic test result was negative. The duty midwife has been following her condition using a Partograph for the last four hours. On examination, her vital signs were reassuring and the absence of leakage confirms that fetal membranes were still intact. She has a positive rapid HIV screening result.

What is the next plan that the midwife should consider?

- (A) Allow freedom of movement
(B) Do digital pelvic examination
(C) Do artificial rupture of membrane
(D) Continue following fetal/maternal condition

29. A 32-year-old pregnant woman came with a complaint of productive cough for 15 days duration. She was tested for HIV and found to be positive at her first ANC visit. She has associated night sweating and chest pain. Otherwise, she has no other abnormal clinical findings.

What is the best drug of choice for this patient?

- (A) Ethambutol
(B) Isoniazid
(C) Pyrazinamide
(D) Rifampicin

30. A 28-week pregnant woman came for her first ANC visit to the nearby health facility. She requested to have HIV testing and the result found to be positive. She has pulmonary tuberculosis and is taking rifampicin containing anti-TB drug regimen. Her hemoglobin is 12 gm/dl. After extensive discussion with the provider, she decided to start ART.

What is the most appropriate ART drug regimen for this client?

- (A) AZT + 3TC + EFV
(B) AZT + TDF + 3TC
(C) TDF + 3TC + EFV
(D) TDF + 3TC + NVP

77. A 38-year Gravida I Para 0 women was admitted in obstetrics ward for the diagnosis of active first stage of labor at 4:00 AM. She is exhausted for pushing long period of time. A midwife follows the labor with partograph and the fetal heart beat pattern line indicates that the last plotted point is 90 b/m in the absence of uterine contraction.

What is the most likely diagnosis?

- (A) Fetal distress
(B) Normal heart beat
(C) Tachycardic heart beat
(D) Reassuring fetal heart beat

78. A 21-year-old primi-gravid woman with gestational age of 38 weeks came with a complaint of pushing down pain for 25 hours duration. On physical examination uterine contraction 3/10'/50-55'', descent 4/5, FHB 135 beats/minute. On vaginal examination the cervix dilates fully, station is -3, position is right occiputo anterior and the parietal bone overhangs the symphysis pubis displacing the thumb and you feel prominent ischial spine. Based on this finding prolonged labor was diagnosed.

What is the most likely cause of prolonged labor?

- (A) Mal position and Mal presentation
(B) Cephalo-pelvic disproportion
(C) Uterine dysfunction
(D) Poor maternal effort

79. A 30-year-old G-II-P-I woman is admitted in maternity ward for the diagnosis of active first stage of labor. On pelvic examination cervix is 5 cm dilated, fetal ischial tuberosities, sacrum and anus was appreciated but no other part of the fetus is palpable.

What type of breech presentation is it?

- (A) Incomplete
(B) Complete breech
(C) Footling breech
(D) Frank breech

80. A 28-year-old Gravida I Para 0 woman presented unconsciously to health facility at 37+3 weeks of gestation. On physical examination, BP 80/50 mmhg, PR 100 BM but week, RR 30 breaths/minute and cold clammy skin. Cervix was 5 cm dilated and there was profuse non clotted vaginal bleeding. After thorough evaluation shock was diagnosed.

What type of shock is it?

- (A) Septic
(B) Cardiogenic
(C) Anaphylactic
(D) Hypovolemic

BOOKLET CODE: 024

18. After taking an informed consent for episiotomy, a midwife planned to prepare a 10 ml of 0.5% lidocaine from 10ml of 2% lidocaine solution.

What is the right amount to be taken by the midwife to prepare the drug?

- (A) 5 ml lidocaine and 5 ml distilled water
 - (B) 2 ml lidocaine and 8 ml distilled water
 - (C) 4 ml lidocaine and 6 ml distilled water
 - (D) 2.5 ml lidocaine and 7.5 ml distilled water
19. A one-year-old infant brought by his mother to a health center with a complaint of redness of the eye. On examination, swollen eyes and pussy discharge was identified. The midwife planned to provide eye ointment.

What is the most preferred site for administration of this medication for this infant?

- (A) In the upper eye
 - (B) In the conjunctival sac
 - (C) In the outer side of the lower eyelid
 - (D) Over the eyelids from medial to lateral side
20. A pregnant woman at term was admitted to a maternity ward with the diagnosis of severe pre-eclampsia. After administration of loading dose of magnesium sulphate, the midwife inserted urinary catheter to monitor urine output.

What is the most appropriate time interval to re-evaluate the urine output for this woman?

- (A) Every one hour
 - (B) Every two hours
 - (C) Every three hours
 - (D) Every four hours
21. A 24-hour-old male neonate transferred to NICU with the diagnosis of early neonatal sepsis. On examination, his temperature was 38.5°C , sucking reflex absent and unable to swallow. A midwife working in a neonatal ward inserts a naso-gastric tube after proper positioning of the neonate and plan to check for proper placement.

What is the most appropriate technique to check the proper placement?

- (A) Confirm tube placement by x-ray
- (B) Aspirate sample of gastric contents using syringe
- (C) Hear through the stethoscope as the air is injected
- (D) Confirm whether the tube is inserted until taped mark

Ethiopian Health Professionals' Licensure Examination

Midwifery
Hamle 2011/July 2019

Part One

Time Allowed: 2:30 hours

Number of Items: 100

Directions: Each of the following questions is followed by four possible alternative answers. Read each question carefully and blacken the letter of your best choice on the separate answer sheet provided.

1. A woman during preconception time comes to a health center and gets anthropometric assessment. The anthropometric assessment reveals that she weighs 70 kilograms and measures 150 cm of height.

What is the Calculated Body Mass Index?

- (A) 35.6 kg/m²
(B) 31.1 kg/m²

- (C) 21.3 kg/m²
(D) 17.6 kg/m²

2. A health care provider assessed a rape survivor for HIV risk. The result showed high HIV exposure.

Which feature of the assessment is indicative for the degree of risk?

- (A) Short lived injuries
(B) Non penetrative attack
(C) Anal assault than vaginal
(D) Less prevalence of HIV in the region

3. A non-pregnant woman presents with feeling of weakness without physical exertion, and blurred vision. The physical examination reveals pale conjunctiva and palmar pallor. The lab result shows 9 g/dl of hemoglobin.

What is the most likely diagnosis of this woman?

- (A) Moderate anemia
(B) Severe anemia

- (C) Mild anemia
(D) No anemia

4. A 32-year-old woman with suboptimal secretion of insulin reported to a clinician. The clinician assessed that the woman is not pregnant but has plan to get pregnant in a soonest possible time. The fasting plasma glucose test shows 136 mg/dl.

What is the most likely diagnosis of the woman?

- (A) Gestational diabetes
(B) Overt type 1 diabetes

- (C) Normal glucose level
(D) Overt type 2 diabetes

73. A Gravida II Para I woman was admitted in maternity ward for the diagnosis of active first stage of labor. On physical examination, uterine contraction 3/10/23-33, FHB 145 beats/minute, longitudinal lie, cephalic presentation and cervix 8 cm dilated.

What is the grade of uterine contraction?

- (A) Weak (C) Moderate
(B) Mild (D) Strong

74. A 37-weeks Gravida III Para II woman is admitted in obstetrics ward for the diagnosis of second stage of labor. On examination, uterine contraction 3/10/30-40", FHB 135 beats/minute, vertex presentation and bregma was palpated between right and posterior side of pelvic cavity.

What was the fetal position?

- (A) Right occiputo-posterior (C) Left occiputo-posterior
(B) Right occiputo-anterior (D) Left occiputo-anterior

75. A Pregnant woman was admitted to laboring room with complaint of pushing down pain for the last 3 hours at 39 weeks of gestation. On physical examination uterine contractions 3/10/45-50, FHB 140 beats/minute, cervical dilatation 4 cm, membrane intact and station 0.

What would be the diagnosis?

- (A) Latent 1st stage (C) Second stage
(B) Active 1st stage (D) Third stage

76. 25-year-old GII PI mother was admitted in active phase of labor at 7:00 PM, 08/09/2011 E.C. Spontaneous vaginal delivery occurred at 12:00 PM. The midwifery care giver evaluates the process of delivery using partograph. The partograph shows the following plotted information.

At 7:00 pm

- Descent: Fetal head was 1/5 palpable;
- Cervix dilated 6 cm;

At 11:00 pm

- Descent: Fetal head was 0/5 palpable;
- Cervix dilated 10 cm.

What was the estimated duration from starting of active 1st stage to delivery of baby?

- (A) 2 hour (C) 6 ours
(B) 5 hours (D) 7 hours

81. A 38-weeks Gravida II Para I woman was admitted in obstetrics ward for the diagnosis of active first stage of labor? On per vaginal examination, cervix 8 cm dilated, membrane ruptured, station -1, cord is easily palpable by fingers before the fetal presenting part. What is the most likely diagnosis?
- (A) Cord prolapse
(B) Occult prolapse
(C) Cord presentation
(D) Cord entanglement
82. A 37-weeks Gravida IV Para III woman was admitted in obstetrics ward for the diagnosis of active first stage of labor. On physical examination uterine contraction is 5/10'/40-55'', the cervix dilated fully, station is -2, position is left occiputo anterior, membrane ruptured, uterus is tender, and FHB 130beats/minute. Suddenly, the uterine contraction ceases, the woman bleeds profusely and fetal heart beat absent. What is the diagnosis?
- (A) Abruptio placenta
(B) Uterine rupture
(C) Placenta previa
(D) Vasa previa
83. A 38-weeks Gravida III Para II woman came with a complaint of profuse vaginal bleeding for 2 hours duration. On physical examination BP 80/60 mmhg, PR 120 beat/minute, there is no uterine contraction but severe abdominal tenderness, non-reassurance fetal heart beat, fetal part is easily palpable abdominally, cervix was 9 cm dilated and there is non-clotted vaginal bleeding. Based on the findings uterine rupture was diagnosed. What is the primary clinical finding used for the diagnosis?
- (A) Feto-maternal distress
(B) Profuse vaginal bleeding
(C) Diffuse abdominal tenderness
(D) Abdominal palpation of fetal parts
84. A 26year-old pregnant mother is presented with complaint of passage of fluid through the vagina at 36+5 weeks of gestation. On physical examination Temp: 38 Oc, RR: 30 breath/minute, Pulse 120 beat/minute and FHB 185 beat/minute. The uterus was tender, cervix admits tip of finger, amniotic membrane is ruptured and there is foul smelling vaginal discharge. What is the diagnosis?
- (A) Term PROM with chorio-amnionitis
(B) Term PROM without chorio-amnionitis
(C) Preterm PROM with chorio-amnionitis
(D) Preterm PROM without chorio-amnionitis

13. A 22-year-old lady is on her 1st day of admission after MVA procedure. She informed the midwife that she had mild vaginal bleeding. Physical examination showed a pulse rate of 75/min, blood pressure of 120/80mmHg, temperature of 37°C, and mild vaginal bleeding. Otherwise, no pertinent finding.

What is the most appropriate management for this woman?

- (A) Advise the woman that menstrual like bleeding is normal until two weeks
- (B) Administer oxytocin drop in 1000 ml normal saline to stop bleeding
- (C) Order complete blood count to rule out anemia
- (D) Shout for help and administer oxygen

14. A 26-year-old woman was presented to the nearby health center with complaint of excessive and prolonged vaginal bleeding. She was amenorrheic for eight weeks. The midwife took vital signs and recorded on midwives' note.

In what sequence should the vital signs be recorded?

- (A) Temperature - Pulse - Respiration and Blood Pressure
- (B) Pulse - Temperature - Blood pressure and Respiration
- (C) Respiration - Blood pressure - Pulse and Temperature
- (D) Blood pressure - Respiration - Temperature and Pulse

15. A Para one woman who delivered in a hospital was presented with a complaint of difficulty to pass urine due to fear of pain on genital laceration. On abdominal examination, her bladder is found at the level of umbilicus with distended bladder. The midwife planned to insert a tube to allow drain of urine.

What is the most appropriate catheterization technique for this woman?

- (A) Indwelling
- (B) Supra pubic
- (C) Straight
- (D) Interim

16. A 26-year-old Para-III woman had abdominal cramp after birth. The midwife assessed and ordered Ibuprofen 400 mg po PRN.

When is the most appropriate time to take this medication?

- (A) As needed
- (B) Once per day
- (C) Twice per day
- (D) Three times per day

17. A 32-year-old woman presented to a hospital with protrusion of fleshy matter from the wound site on her fourth post operative day. She had dry cough for the last three days. On examination, separation of abdominal fascia with protrusion of viscera were noted. The gynecologist.

What is the most appropriate next step in the management of this woman?

- (A) Consult to surgeon
- (B) Clean with antiseptic solution
- (C) Provide prophylactic antibiotics
- (D) Repair the separated fascia

59. A 35-year-old Gravida-II and Para-I mother came for 1st ANC visit with a complaint of headache. History revealed that she is easily fatigable and gestational age is 20 weeks by LMP. On physical examination-BP 100/70mmHg, PR-100beat per minute, RR-22breath per minute and pale conjunctiva were revealed. Laboratory investigation shows Hct level of 27% and Mean Corpuscular Volume- 60 μ m³.

What is the best management for this woman?

- (A) Provide iron-dextran 50 mg IM
- (B) Transfuse with cross matched blood
- (C) Provide iron folate tablet twice daily
- (D) Send home with advice to take iron rich foods

60. A 25-year-old primigravida lady came for her first ANC visit. The blood group and Rh investigation shows an A⁺ (A negative) result.

What is the most likely management that should come next for this client?

- (A) Detect fetal blood group
- (B) Provide anti D antibodies
- (C) Detect partner blood group
- (D) Perform indirect coomb's test

61. A 32-year-old Gravida-III and Para-II mother presented with severe headache at gestational age of 35 weeks. Physical examination revealed BP-180/110mmHg. The midwife provided hydralazine 5mg IV.

What is the most possible time of evaluation for this woman?

- (A) Every 20 minutes
- (B) Every 1 hour
- (C) Every 2 hours
- (D) Every 4 hours

62. A Gravida II Para I woman came to delivery ward with complaint of pushing down pain for the last 4 hours. Her last menstrual period was normal and stayed from June 20-25, 2010 E.C. The visiting date was 20/06/2011 E.C.

What was the gestational age at the time of visiting?

- (A) 33 weeks and 4 day
- (B) 34 weeks and 2 days
- (C) 35 weeks and 0 day
- (D) 38 weeks and 4 days

63. A Gravida II Para I woman came to delivery ward with complaint of pushing down pain for the last 4 hours. Her last menstrual period was normal and stayed from Pagume 1-4, 2010 E.C. The visiting date was 05/09/2011 E.C.

When was her expected date of delivery?

- (A) 06/10/2011 E.C.
- (B) 09/10/2011 E.C.
- (C) 11/10/2011 E.C.
- (D) 14/10/2011 E.C.

Handwritten calculations and notes at the bottom of the page, including dates like 2012, 2013, and 2014, and numbers like 21, 23, 25, 27, 28, 29, 30, 31, 32, 33, 34, 35, 36, 37, 38, 39, 40, 41, 42, 43, 44, 45, 46, 47, 48, 49, 50, 51, 52, 53, 54, 55, 56, 57, 58, 59, 60, 61, 62, 63, 64, 65, 66, 67, 68, 69, 70, 71, 72, 73, 74, 75, 76, 77, 78, 79, 80, 81, 82, 83, 84, 85, 86, 87, 88, 89, 90, 91, 92, 93, 94, 95, 96, 97, 98, 99, 100.

85. After assisting normal birth, a midwife wants to use this instrument for the next delivery and the instruments should be free from all microorganisms through instrumental processing.

What step the midwife will follow?

- (A) Decontamination -- cleaning -- sterilization
- (B) Cleaning -- high level disinfection -- sterilization
- (C) Cleaning -- decontamination -- high level disinfection
- (D) Decontamination -- high level disinfection -- sterilization

86. A 30-year-old gravid II Para I mother with GA of 40 weeks came to obstetrics ward at 8:00 am with the complaint of pushing down pain for 2 hours duration. On P/E -vital sign was stable, cervix 2cm dilated, membrane intact, station -2, contraction= 2c/10'/30-40" and FHB was 136BPM. At 12:00 am -cervix 3 cm dilated, membrane intact, station -2, contraction= 2c/10'/30-35", FHB= 144 BPM.

What will be the immediate management for the woman?

- (A) Admit and follow fetomaternal condition.
- (B) Ambulate and reevaluate after 4 hrs.
- (C) Discharge and advise to come back
- (D) Admit and start induction

87. A 25-year-old primi gravid with GA of 38 weeks came to OPD at 4:00 am with the complaint of pushing down pain for 3 hours duration. On P/E cervix =2 cm dilated, membrane intact, contraction= 2c/10'/30-35" and FHB was 144 BPM. At 12:00 am cervix 4 cm, membrane intact, contraction= 4c/10'/40 seconds and FHB= 152 BPM.

What will be the next management for the woman?

- (A) Admit and follow with partograph
- (B) Ambulate and re-evaluate after 4 hrs.
- (C) Rupture the amniotic membrane
- (D) Augment with Oxytocine

88. A 32-years-old Gravida IV Para III woman with GA of 39 weeks came to obstetrics ward at 2:00 am with the complaint of pushing down pain for 2 hours duration. On P/E, vital sign was stable, cervix 5cm dilated, membrane intact, station -1, contraction= 3c/10'/30-40 and FHB was 136 BPM. At 6:00 am physical examination was repeated and the finding was cervix 6 cm dilated, membrane intact, station 0, contraction= 2c/10'/30-35, FHB= 144 BPM.

What will be the next management of the woman?

- (A) Follow with partograph
- (B) Rapture the membrane
- (C) Augment with Oxytocine
- (D) Doing caesarean section

9. A 23-year-old primigravida lady came to a hospital with a complaint of vaginal bleeding of one day duration at a gestational age of 10 weeks and she also had an abdominal pain. On physical examination, the midwife finds uterus is not palpable abdominally, cervix admits 61 finger, and there is active bleeding.

What is the most appropriate management option?

- (A) Medical
(B) Dilatation and Curettage

- (C) Evacuation and Curettage
(D) Manual Vacuum Aspiration

10. A mother came to a hospital with a complaint of involuntary urine loss of two weeks duration. She also had fever, headache and urinary frequency. On physical examination, her pulse rate is 100/min, temperature is 38.7°C , and involuntary urine leakage. Urinalysis showed many WBC in urine.

What is the initial management for this patient?

- (A) Give antibiotic drugs
(B) Counsel about life style modification
(C) Surgical management of the incontinence
(D) Appoint the mother to return when the problem is worse

11. A mother with molar pregnancy is treated with suction curettage after being amenorrheic for the last 10 weeks. On the procedure, evacuation completeness was assured with ultrasound. Then the patient is discharged after her vital sign become in the normal range.

How often the serum β -hCG level should be assessed for this woman?

- (A) Every week until undetectable
(B) Every one month until undetectable
(C) Every three week until undetectable
(D) Every three months until undetectable

12. A 25-year-old, Para-II woman came to a hospital with a complaint of absence of menses for six months duration. On investigation, she is negative for serum β -hCG. Two weeks back she took oral progesterone but the menses did not start.

What is the most appropriate next management option for this client?

- (A) Give oral estrogen
(B) Repeat progesterone

- (C) Do chromosomal analysis
(D) Measure serum gonadotropins

SUBJECT CODE: 824

27. A Para-II woman came for post natal care after delivery of her baby 18 days ago. On assessment, she gave birth at this facility but had not received any antenatal care and she declined HIV testing during labor but then agreed to be tested after childbirth. Blood was taken for testing to detect HIV antibodies.

What is the most appropriate initial test for the woman?

- (A) ELISA test
(B) DNA-PCR test
(C) Rapid HIV test
(D) Western Blot test

28. A 25-year-old multigravida woman has presented to labor and delivery room with passing down pain. Her serologic test result was negative. The duty midwife has been following her condition using a Partograph for the last four hours. On examination, her vital signs were reassuring and the absence of leakage confirms that fetal membranes were still intact. She has a positive rapid HIV screening result.

What is the next plan that the midwife should consider?

- (A) Allow freedom of movement
(B) Do digital pelvic examination
(C) Do artificial rupture of membrane
(D) Continue following fetal/maternal condition

29. A 32-year-old pregnant woman came with a complaint of productive cough for 15 days duration. She was tested for HIV and found to be positive at her first ANC visit. She has associated night sweating and chest pain. Otherwise, she has no other abnormal clinical findings.

What is the best drug of choice for this patient?

- (A) Ethambutol
(B) Isoniazid
(C) Pyrazinamide
(D) Rifampicin

30. A 28-week pregnant woman came for her first ANC visit to the nearby health facility. She requested to have HIV testing and the result found to be positive. She has pulmonary tuberculosis and is taking rifampicin containing anti-TB drug regimen. Her hemoglobin is 12 gm/dl. After extensive discussion with the provider, she decided to start ART.

What is the most appropriate ART drug regimen for this client?

- (A) AZT + 3TC + EFV
(B) AZT + TDF + 3TC
(C) TDF + 3TC + EFV
(D) TDF + 3TC + NVP

BOOKLET CODE: 024

4. A 60-year-old mother came to a clinic with a complaint of protrusion of mass per vagina for one month duration. She had feeling of incomplete emptying. On physical examination, the midwife finds protrusion of soft mass on vaginal apex and bowel peristalsis behind the vaginal wall.

What is the most likely diagnosis for this client?

- (A) Cystocele
(B) Intestine
(C) Rectocele
(D) Uterine prolapse

5. A 32-year-old mother came to a clinic with a complaint of skin rash of two weeks duration. She had fatigue and fever. On physical examination, maculo-papular rash and a whitish, round, flat and raised lesions were seen in her genital area. VDRL was reactive.

What is the most likely diagnosis for this client?

- (A) Primary syphilis
(B) Secondary syphilis
(C) Tertiary syphilis
(D) Latent syphilis

6. A pregnant woman came to a hospital for a complaint of back pain for one week duration at term. On examination, vital signs are in the normal range, easily palpable fetal part and malposition identified. Trans-vaginal ultrasound showed empty uterus.

What is the best management option for this client?

- (A) Laparoscopy
(B) Hysterectomy
(C) Cesarean section
(D) Laparoscopic management

7. A 20-year-old multi-para lady came to a hospital with a complaint of abnormal vaginal bleeding of six months duration. The bleeding was irregular and occurs at any time in her cycle. On examination, her vital signs are in the normal range and no other pathological finding. Ultrasound examination revealed normal endometrial thickness.

What is the most likely management for this client?

- (A) Surgical
(B) Hormonal
(C) Counseling
(D) Radiation

8. A 36-year-old mother was presented to adult OPD with complaints of fever, fatigue, and skin rash for two months duration. She also has history of anemia, diarrhea and fever for six weeks. On physical examination, severe wasting, confused status, and the presence of ulcer in the lips. On laboratory investigation, her Rapid HIV test is reactive and urine HCG negative.

What is the next most appropriate management for this mother?

- (A) Initiate ART
(B) Give ARV prophylaxis
(C) Link to the ART clinic
(D) Give cotrimoxazole and appoint her

13. A 22-year-old lady is on her 1st day of admission after MVA procedure. She informed the midwife that she had mild vaginal bleeding. Physical examination showed a pulse rate of 75/min, blood pressure of 120/80mmHg, temperature of 37°C, and mild vaginal bleeding. Otherwise, no pertinent finding.

What is the most appropriate management for this woman?

- (A) Advise the woman that menstrual like bleeding is normal until two weeks
- (B) Administer oxytocin drop in 1000 ml normal saline to stop bleeding
- (C) Order complete blood count to rule out anemia
- (D) Shout for help and administer oxygen

14. A 26-year-old woman was presented to the nearby health center with complaint of excessive and prolonged vaginal bleeding. She was amenorrheic for eight weeks. The midwife took vital signs and recorded on midwives' note.

In what sequence should the vital signs be recorded?

- (A) Temperature - Pulse - Respiration and Blood Pressure
- (B) Pulse - Temperature - Blood pressure and Respiration
- (C) Respiration - Blood pressure - Pulse and Temperature
- (D) Blood pressure - Respiration - Temperature and Pulse

15. A Para one woman who delivered in a hospital was presented with a complaint of difficulty to pass urine due to fear of pain on genital laceration. On abdominal examination, her bladder is found at the level of umbilicus with distended bladder. The midwife planned to insert a tube to allow drain of urine.

What is the most appropriate catheterization technique for this woman?

- (A) Indwelling
- (B) Supra pubic
- (C) Straight
- (D) Interim

16. A 26-year-old Para-III woman had abdominal cramp after birth. The midwife assessed and ordered Ibuprofen 400 mg po PRN.

When is the most appropriate time to take this medication?

- (A) As needed
- (B) Once per day
- (C) Twice per day
- (D) Three times per day

17. A 32-year-old woman presented to a hospital with protrusion of fleshy matter from the wound site on her fourth post operative day. She had dry cough for the last three days. On examination, separation of abdominal fascia with protrusion of viscera were noted. The gynecologist.

What is the most appropriate next step in the management of this woman?

- (A) Consult to surgeon
- (B) Clean with antiseptic solution
- (C) Provide prophylactic antibiotics
- (D) Repair the separated fascia

Ethiopian Health Professionals' Licensure Examination

Midwifery Exam
Yekabti 2013/February 2021
PART TWO

BOOKLET CODE: 024
Number of Items: 100

SUBJECT CODE: 014
Time Allowed: 2:30 hours

DIRECTIONS: Each of the following questions is followed by four possible alternative answers. Read each question carefully and **blacken** the letter of your best choice on the separate answer sheet provided.

1. A 24-year-old lady came to a hospital with a complaint of vaginal bleeding for two days duration. She was amenorrheic for three months. She had no abdominal pain but had vaginal discharge. On physical examination, uterus is not palpable abdominally, fetal heart beat is not detected, and cervix is closed and no active vaginal bleeding.

What is the most likely diagnosis for this woman?

- (A) Complete abortion
(B) Inevitable abortion

- (C) Missed abortion
(D) Threatened abortion

2. A mother comes to a hospital for complaints of palpitation and light headedness of one day duration. She was amenorrheic for the last five weeks. On examination, her pulse rate is 120/min, blood pressure is 80/50mmHg, left lateral lower abdominal tenderness and cervical motion tenderness. Trans-vaginal ultrasound examination shows accumulation of blood in adnexal region.

What is the most likely classification for this pregnancy?

- (A) Abortifacient

- (B) Isthmus

- (C) Interstitial

- (D) Pimtrial

3. A 38-year-old Para-V woman came to a hospital with the complaint of inter menstrual bleeding for the last seven months. She witnessed that the bleeding starts in the middle of each cycle and lasts for two to three days. On examination, vital signs are within the normal range. Laboratory investigation showed, 11g/dl hemoglobin.

What is the most likely classification of menstrual disorder for this client?

- (A) Metrorrhagia
(B) Menorrhagia

- (C) Oligomenorrhea
(D) Menometrorrhagia

Ministry of Health
Ethiopian Health Professionals' Licensure Examination

MIDWIFERY
Ginbot 2015/May 2023

Full Name_____

PART: ONE

TIME ALLOWED: 2:30 HOURS

GENERAL DIRECTIONS

IN THIS EXAMINATION, THERE ARE A TOTAL OF **100** MULTIPLE CHOICE QUESTIONS. SELECT THE BEST ANSWER AND **BLACKEN** ONLY THE LETTER OF YOUR CHOICE ON THE SEPARATE ANSWER SHEET PROVIDED. FOLLOW THE INSTRUCTIONS IN THE ANSWER SHEET AND THE EXAMINATION PAPER CAREFULLY. USE ONLY PENCIL TO MARK YOUR ANSWERS. YOUR ANSWER MARK SHOULD BE **HEAVY** AND **DARK** COVERING THE ANSWER SPACE COMPLETELY. IF YOU CHANGE YOUR ANSWER, PLEASE ERASE IT COMPLETELY AND NEATLY.

YOU ARE ALLOWED TO WORK ON THE EXAMINATION FOR **2:30** HOURS. WHEN TIME IS CALLED, YOU MUST IMMEDIATELY STOP WORKING, PUT YOUR PENCIL DOWN, AND WAIT FOR FURTHER INSTRUCTIONS.

ANY FORM OF CHEATING OR AN ATTEMPT TO CHEAT IN THE EXAMINATION WILL RESULT IN AN AUTOMATIC DISMISSAL FROM THE EXAMINATION HALL AND CANCELLATION OF YOUR SCORE(S). PLEASE MAKE SURE THAT YOU HAVE WRITTEN ALL THE REQUIRED INFORMATION IN THE ANSWER SHEET BEFORE YOU START TO WORK ON THE EXAMINATION. YOUR ANSWER SHEET WILL NOT BE MARKED UNLESS YOUR QUESTION PAPER IS RETURNED.

DO NOT TURN THIS PAGE OVER UNTIL YOU ARE TOLD TO DO SO.

Ethiopian Health Professionals' Licensure Examination

Midwifery Exam Ginbot 2015/May 2023 PART ONE

Number of Items: 100

Time Allowed: 2:30 hours

DIRECTIONS: Each of the following questions is followed by four possible alternative answers. Read each question carefully and **blacken** the letter of your best choice on the separate answer sheet provided.

1. A 28-year-old woman who is planning to conceive came to a clinic for preconception care service. Previously she has lost a baby because of neural-tube defects and is worried about her future pregnancy outcome.

What is the most effective preconception intervention for this woman?

- | | |
|--------------------------------|----------------------------|
| (A) Folic acid supplementation | (C) Iodine supplementation |
| (B) Hepatitis B vaccination | (D) Iron supplementation |

2. A 29-year-old woman came to a family planning clinic before the date of appointment being upset. On assessment, she explained that, her partner is belittling, humiliating and undermining her in front of others. In addition, he has restricted her from socializing with her family and friends.

Based on her history, for what condition is the woman at greatest risk?

- | | |
|---------------------|----------------------------|
| (A) Sexual violence | (C) Physical violence |
| (B) Economic abuse | (D) Psychological violence |

3. A midwife is providing care for a 22-year-old woman who is planning to get pregnant. On assessment, she had history of heavy alcohol dependence since two years.

For what fetal complication is this woman at greatest risk?

- | | | | |
|-------------------|------------------|----------------|------------------|
| (A) Spinal bifida | (B) Microcephaly | (C) Macrosomia | (D) Cleft palate |
|-------------------|------------------|----------------|------------------|

4. A 30-year-old woman came to a clinic for counseling service. On assessment, she was diagnosed for severe anemia. She was managed accordingly and appointed after a month.

What is the most important laboratory investigation that would help to assess her progress during revisit?

- | | |
|----------------|--------------------------|
| (A) Urinalysis | (C) Blood group and RH |
| (B) Hemoglobin | (D) Random glucose level |

5. A 32-year-old woman came to an OPD complaining of lacerations and bleeding from vagina due to sexual abuse one hour back. On assessment, she stated that three men hanged her from behind while she was returning from work, and forced her into having sex. On physical examination, there are tears, bruises, scratches and abrasions to the posterior couchette. In addition, there are bite and punch marks on her face and marks of restraints on the wrists.

What is the main purpose of doing physical examination for this woman?

- (A) To understand the level of injury
- (B) To identify those involved in sexual abuse
- (C) To help the legal process forensic evidence
- (D) To determine what medical care should be provided

6. A 23-year-old commercial sex worker came to a family planning clinic in pursuit of getting advice on contraceptive methods.

What is the most recommended contraceptive method for this woman?

- (A) Dual method
- (B) Withdrawal method
- (C) Long term contraceptive method
- (D) Permanent contraceptive method

7. A 31-year-old woman who is planning to conceive came to a preconception service. On assessment, her HIV result was positive and her hemoglobin level was 10 g/dl.

What is the ideal time to start ART for this woman?

- (A) After her hemoglobin level reaches the normal range
- (B) She can start the treatment immediately
- (C) When she is WHO clinical Stage-IV
- (D) 14 weeks after she gets pregnant

8. A 30-year-old mother brought her 15-day-old neonate complaining purulent discharge from eye. After assessment, the midwife made a diagnosis of neonatal conjunctivitis.

What is the most appropriate management for this neonate?

- (A) Amoxicillin plus Doxycycline
- (B) Wiping eyes with warm water
- (C) Ceftriaxone plus Erythromycin
- (D) Applying 1 percent tetracycline eye ointment

9. A 26-year-old Para-III mother came for a family planning service after two weeks of delivery. After detailed counseling, she decided to use lactation amenorrhea method.

What appropriate maternal history should the provider ask to confirm eligibility?

- (A) Weight status
- (B) Nutritional status
- (C) Immunization status
- (D) Breast feeding status

10. A 28-year-old Para-VI mother came to a family planning service to space her pregnancy. She has regular vaginal bleeding. On examination, she is febrile. During counseling she stated that she does not want to use modern contraception because of the adverse effects she experienced. At the end, she decided to use natural family planning method.

Which natural family planning method is NOT recommended for this mother?

- (A) Basal body temperature (C) Standard day's method
(B) Cervical mucus method (D) Rhythm method

11. A 36-year-old Para-VI mother came to a family planning service to refill her contraceptive on appointment date. She is breastfeeding and has regular menstrual cycle. On assessment, she is HIV/AIDS sero-positive.

What should the health care provider do for this mother?

- (A) Changing the previous method (C) Couple dual protection
(B) Managing method side effects (D) Counseling her future fertility

12. A 39-year-old Para-VIII mother came to a family planning service to utilize contraception. During counseling, she decided to use progestin only pill.

What is the most appropriate physical examination the health provider needs to perform?

- (A) Cervical examination (C) Pelvic examination
(B) Abdominal examination (D) Breast examination

13. A 22-year-old woman came with her classmates to a reproductive adolescent and youth service room for consulting about condom breaks. On history, she has been using condom as a family planning method for two years; but before two days, she faced breakage of condom during sexual intercourse.

What is the most appropriate care needed for the client?

- (A) Providing emergency contraceptives
(B) Counseling about pregnancy terminations
(C) Counseling about the disadvantage of condom use
(D) Counseling her to avoid condom as a contraceptive

14. A 27-year-old Para-II mother came to a family planning service after three months of injection of depo provera. She stated that her bleeding is irregular and she wanted a method change. The midwife made an assessment and concluded that her complaint is normal in relation to the method she used.

What is the most important counseling component the provider needs to address?

- (A) Reproductive health issues
(B) Problem of narrow birth interval
(C) The need to change to other methods
(D) The side effects of injectable contraceptive

15. A 30-year-old Para-II mother came to a gynecology OPD with a complaint of inability to conceive for the last three months. On assessment, she reported that she was using injection of depo provera for five years. Her pregnancy test result was negative.

What is the most essential counseling component the provider needs to address?

- (A) Counsel her that her condition might be due to menopause
- (B) Reassure her since it is due to contraceptive side effect
- (C) Send her for further gynecologic investigation
- (D) Counsel her to repeat pregnancy test

16. A 26-year-old Para-III mother came to a gynecologic OPD for family planning service. She is exclusively breastfeeding her four-month-old baby and her monthly bleeding has not returned. She claimed that she does not want to use long acting methods.

What is the most appropriate method for the mother?

- (A) Progestin only pills
- (B) Combined oral contraceptive
- (C) Emergency contraceptive pill
- (D) Estrogen contained injectable

17. A 36-year-old woman came to a family planning unit for contraceptive use. She has mild headache but no other complaints. After counseling the woman about the available methods, she decided to use LNG-IUD. Finally, the provider wanted to check her vital signs to ensure safe use of LNG IUD.

Which vital sign is most important input for making wise decision?

- (A) Pulse rate
- (B) Temperature
- (C) Blood pressure
- (D) Respiratory rate

18. A 20-year-old woman came to a family planning unit with complaint of itchy skin around the area where implant was inserted a week before. She admitted that she removed the plaster and gauze applied immediately when she left the facility to hide the procedure from her families. On inspection, the insertion site is reddened and there is slight pus surrounding the area.

What is the most likely diagnosis?

- (A) Missing of implant
- (B) Expulsion of implant
- (C) Insertion site infection
- (D) Insertion site muscle injury

19. A 38-year-old woman came to a health center with a complaint of lower abdominal pain. She claimed that she has started to use IUCD since two weeks back, and started to feel pain after the fifth days of insertion. Currently, she has lower abdominal pain, nausea and feeling of pain during intercourse. She is febrile to touch. On laboratory investigation, her pregnancy test is negative.

What is the most likely complication this woman is suffering from?

- (A) Perforated uterus
- (B) Ectopic pregnancy
- (C) Local trauma of cervix
- (D) Pelvic inflammatory disease

20. A midwife is preparing for IUCD insertion to his client. Before insertion, the midwife intends to grasp the cervix with tenaculum.

What is the recommended site that the midwife has to use?

- (A) At the 5 and 7 o'clock positions (C) At the 2 and 10 o'clock positions
(B) At the 4 and 8 o'clock positions (D) At the 9 and 12 o'clock positions

21. A 37-year-old woman who has been using implant for five years came to a family planning unit for removal when it is due time for protection. She is satisfied with the contraception and wants to continue the method. The site where the rods located is clean and has no scar. The provider advised her about removal and re-insertion procedures, and selection of the recommended area for re-insertion.

What is the recommended area for re-insertion?

- (A) Insert the new rod at the same site where the rods are removed
(B) Change site of insertion at least 5 mm away from the previous
(C) Re-insert the rod opposite to the previous site at same arm
(D) Insert the new rod in the other arm

22. A 35-year-old Para-III woman came to a family planning unit to stop next chance of pregnancy. The provider advised her about the available options of contraception. After the counseling, she preferred to use tubal ligation. However, the provider told the woman that the time of her visit is not ideal for the procedure; hence, it has to be postponed.

What is the optimum time to get such service?

- (A) Within seven days of delivery (C) From 7 to 42 days of delivery
(B) Immediately after delivery (D) After 42 days of delivery

23. A 39-year-old woman is in a postnatal ward after she gave birth six hours back. A duty midwife is preparing to counsel the woman about postpartum family planning prior to discharge and identified important areas and components to be addressed during counseling.

Which information should NOT be included in the counseling?

- (A) Counsel her about methods, their effectiveness rates, and side effects
(B) Reassure her that all methods are safe and does not interfere with breast feeding
(C) Provide her with the information on contraceptive resupply and access to follow-up care
(D) Advise her on contraceptives taking into account reproductive goal and protection against STIs

24. A 30-year-old woman on her third postpartum day came to a family planning unit for contraceptive service. On history, she had prolonged labor and her baby died at birth. She planned to delay pregnancy for the next two years and wants to use implant.

What is the most appropriate response from the provider?

- (A) Allow to use implant and provide the service on same day
- (B) Tell her she is not candidate as she needs to delay for two years
- (C) Appoint her to come after six weeks of postpartum for insertion
- (D) Appoint her to come after six months of postpartum for insertion

25. A 27-year-old Gravid-II, Para-I mother came to a health center for routine ANC service utilization on day 05-07-2013E.C. During the course of the assessment, the midwife asked her LNMP. Her menstrual bleeding was regular for the last one year; and the last one started on 21-01-2013E.C and ended on 26-01- 2013E.C.

What is the gestational age of this pregnancy?

- (A) 23 weeks + 3 days
- (B) 23 weeks + 4 days
- (C) 24 weeks + 3 days
- (D) 24 weeks + 4 days

26. A 27-year-old Gravid-II, Para-I HIV positive mother came to an ANC unit on appointment. Based on her LNMP and early first trimester ultrasound evaluation, the gestational age is 37 weeks. Laboratory investigation showed that her hemoglobin level is 12 mg/dl, WBC count is 8000/mm³, platelet count is 150,000/mm³, viral load is 1500 copies/ml, and CD4 + count is 200 cells/mm³ (Normal value hemoglobin is 12-16mg/dl, WBC is 4000-10,000/mm³, platelet 100,000-450,000/mm³)

What measure should be taken to decrease the risk of MTCT of HIV for this mother?

- (A) Elective cesarean delivery at 40 weeks
- (B) Elective cesarean delivery at 39 weeks
- (C) Elective cesarean delivery at 38 weeks
- (D) Elective cesarean delivery at 41 weeks

27. A midwife screened a woman for HIV during ANC service. Prior to giving clear and accurate information about the screening, the midwife reported the result to the woman.

What principle of HIV testing and counseling did the provider breach?

- (A) Confidentiality
- (B) Connection
- (C) Comfort
- (D) Consent

28. A 26-year-old primigravida mother came to an ANC unit for antenatal care service. According to her LNMP, the gestational age is 38 weeks. On the third Leopold maneuver examination, the lower part of the abdomen was occupied by large, irregular and non-ballotable mass.

What is the most likely presentation of the fetus?

- (A) Face
- (B) Breech
- (C) Cephalic
- (D) Shoulder

29. A 27-year-old Gravid-II, Para-I mother at 14-week gestation came for ANC follow-up. She had no antenatal care during previous pregnancy and it ended up with intrauterine fetal death. On review of the obstetrics history from chart, the inspection of the dead body of her baby showed mummified, hydrocephalus and enlarged body organs. On physical examination, she is well looking. She and her husband were also investigated for VDRL and they were reactive.

What is the most appropriate interpretation for the above findings?

- (A) Poor suggestive for syphilis infection
 - (B) Highly suggestive for syphilis infection
 - (C) Poor suggestive for Gonorrheal infection
 - (D) Highly suggestive for Gonorrheal infection
30. An 18-year-old primigravida woman with nine weeks of gestation came to a hospital with complaints of nausea and vomiting for three days duration. Physical examination showed a dry tongue and inelastic skin. Laboratory findings showed no hemoparasite on blood films, Urine analysis is ketone + 3 and albumin trace, LFT and RFT results are in normal range.

What is the most likely diagnosis of this woman?

- (A) Cholecystitis
 - (B) Gastroenteritis
 - (C) Hyperemesis gravidum
 - (D) Gastro esophageal reflux disease
31. A 23-year-old multigravida woman at 32 weeks of gestation came to an ANC unit with complaint of difficulty of breathing. She was diagnosed for diabetes mellitus and she is on insulin. On assessment, inspection of abdomen showed tense and shiny skin with large striae, fundal height larger for date, girth of the abdomen is round and the fetal parts were difficult to clearly define. Moreover, on auscultation of fetal heart sound is difficult to hear.

What is the most likely diagnosis of this pregnancy?

- (A) IUGR
 - (B) IUFD
 - (C) Polyhydramnios
 - (D) Oligohydramnios
32. A 25-years-old Gravida-II Para-I woman was presented to an ANC clinic with complaint of cessation of fetal body movement for seven days duration. According to her LNMP and first trimester ultrasound result, the gestational age of the pregnancy was 32 weeks. On ultrasound evaluation the fetal cranial bones were overlapped.

What is the most likely diagnosis of this pregnancy?

- (A) IUFD
- (B) IUGR
- (C) Polyhydramnios
- (D) Oligohydramnios

33. A 30-year-old multigravida mother came to an ANC unit with complaint of passing of the due date for pregnancy. Based on LNMP and first trimester ultrasound, gestational age is 41 weeks and four days. Abdominal palpation findings showed that Fundal height is smaller for the gestational age and the fetal parts are easily palpable. Moreover, auscultation revealed normal fetal heart sound. On ultrasound evaluation, gestational age 40 weeks, AFI is 2 cm and placenta anterior and fundal.

What is the most appropriate management for this case?

- (A) Induction (C) Cesarean delivery
(B) Amnioinfusion (D) Wait until labor starts spontaneously

34. A 20-year-old primigravida woman at 34 weeks of gestation came to a hospital with referral paper from a health center for further management. Her history showed that fetal movement ceased before seven days. She has no known medical or surgical problems. On abdominal assessment, she has abdominal tenderness, fundal height is 32 weeks, cephalic presentation and FHB is absent. Her laboratory investigation result showed a hemoglobin level of 13 mg/dl, platelet count of 90,000/mm³, WBC count of 10,000/mm³, Blood group is B+, and urine analysis, liver and renal function tests are within normal range. The ultrasound evaluation shows absent FHB, gestational age 32 weeks, average fetal weight is 1.8 kg, AFI is 4 cm and placenta fundal. (Normal value Hemoglobin is 12 up to 16mg per dl, WBC is 4000-10,000 and Platelet 100,000-450,000).

What is the most appropriate management for this case?

- (A) Cesarean delivery (C) Augmentation of labor
(B) Induction of labor (D) Wait for spontaneous labor

35. A 32-year-old Gravida-III, Para-II woman at 33 weeks of gestation was presented to an ANC clinic with a complaint of leakage of fluid through vagina of six hours duration. However, she denied vaginal bleeding and abdominal pain. On examination, she is healthy looking and her blood pressure is 120/75mmHg, respiration rate is 16/min, pulse rate is 76/min and temperature is 36.6 degree centigrade. On abdominal examination, no tenderness, 30 weeks gravid uterus and normal heart rate tracing were noted. On speculum examination, there is accumulation of fluid in posterior fornix. When the woman is asked to cough, there is leakage of fluid through the cervix. Laboratory investigation showed normal findings. Ultrasound evaluation showed, GBM, FBM and FHB are seen, BPD is 30 weeks, AFI is 5 cm, and placenta is fundal and no gross congenital malformation seen.

What is the most appropriate management for this woman?

- (A) Admit to ward, steroid, antibiotic and induction of labor
(B) Admit to ward, antibiotic and follow the feto-maternal conditions
(C) Admit to ward, antibiotic, steroid and prepare for cesarean delivery
(D) Admit to ward, antibiotic, steroid and follow the feto-maternal conditions

36. A 25-year-old primigravida woman at 38 weeks of gestation (reliable for date) came to an ANC unit with complaint of vaginal bleeding. She is known hypertensive mother and on methyldopa 250 mg PO twice per day. On examination, she is healthy looking and her blood pressure is 135/85 mmHg, respiration rate is 16/min and pulse rate is 80/min. On abdominal examination, fundal height is 37 weeks, cephalic presentation and FHB is 140/min. Laboratory investigation showed normal findings. On ultrasound evaluation, GBM, FBM and FHB were seen, gestational age was 37 weeks, AFI was 10 cm and placenta was fundal and anterior.

What is the most appropriate management for this case?

- (A) Admit to ward and prepare for induction of labor
- (B) Admit to ward and follow fetomaternal conditions
- (C) Admit to ward, antibiotic and follow the fetomaternal conditions
- (D) Admit to ward, glucocorticoids and follow the fetomaternal conditions

37. A 32-year-old pregnant woman visited an ANC clinic for the first time. She had a regular menstrual cycle and gestational age is 12 weeks. Physical examination result showed that she is well and comfortable with no other abnormalities detected. All routine laboratory investigations were done and the results were within the normal range.

What is the appropriate intervention for this woman?

- (A) Provide iron and folic acid supplements
- (B) Instruct her to take caffeine just after meal
- (C) Avoid consuming fruits while taking iron and folic acid
- (D) Advise her to take tea at the same time when taking iron

38. A mother came to a health center for her third antenatal follow-up. She complains drowsiness and malaise of three days duration. On examination, her blood pressure is 110/60 mmHg and pulse rate is 95/min. Three weeks back she was given Iron-folate supplement. The midwife needs to evaluate her adherence to the supplement.

What is the most appropriate evidence for evaluating the adherence?

- (A) Slower refilling of the nail bed
- (B) Hemoglobin of less than 11 g/dl
- (C) Regular daily intake of Iron-folate
- (D) Pallor conjunctiva and mucus membrane

39. A pregnant mother at a gestational age of six weeks came for antenatal check-up for the first time. Her body mass index before pregnancy was 22 kg/m². All her histories, physical findings and investigations are normal. She needs an advice on well balanced diet to maintain a normal weight gain according to her BMI.

What is the appropriate advice regarding the daily intake of food for this mother?

- (A) Low fiber diet intake
- (B) Drink plenty of fluids
- (C) High saturated fatty foods
- (D) Low intake of protein rich foods

40. A pregnant university student came to a health facility for antenatal follow-up before her date of appointment. The pregnancy is wanted but unplanned. She has taken the first dose of tetanus toxoid (TT-1) one a week before.

When is the second dose of TT appointment?

- | | |
|-----------------------------|-------------------------------------|
| (A) Two weeks after today | (C) One week after the first dose |
| (B) Three weeks after today | (D) Five weeks after the first dose |

41. A 25-year-old Gravida-II, Para-I woman at 36 weeks of gestation confirmed by first trimester ultrasound was presented to an ANC clinic with complaint of leakage of fluid through vagina of 24 hours duration. However, she denied vaginal bleeding and abdominal pain. On speculum examination, there is accumulation of fluid in posterior fornix and leakage of fluid from the cervix. On abdominal examination, fetal parts are easily palpable, cephalic presentation and positive fetal heart beat. On ultrasound evaluation, expected fetal weight is 2.9 kg, positive fetal heart beat, AFI is 4 cm and placenta is fundal and anterior.

What is the most likely diagnosis of this woman?

- | | |
|-------------------------|----------------------------|
| (A) Term PROM | (C) Preterm PROM |
| (B) Prolonged term PROM | (D) Prolonged preterm PROM |

42. A reproductive age woman came to a clinic with a complaint of genital ulcer of two weeks duration. The ulcer is painful, vesicular and irritant. She also reported that she has fever, headache, malaise and muscular pain. On physical examination, a midwife observed a shallow and non-indurated tender ulcer on the perineum.

What is the most likely diagnosis of this woman?

- | | |
|--------------------------|------------------------------|
| (A) Neisseria gonorrhea | (C) Trichomonas vaginalis |
| (B) Herpes simplex virus | (D) Lymphogranuloma Venereum |

43. A pregnant woman came to an antenatal clinic with a complaint of thick, profuse, malodorous, yellow-green, frothy secretion from the vagina of two months duration. She has also pruritis, itching, vulval irritation, dyspareunia, dysuria, and frequent of urination. She also said that her husband has a penile discharge.

Which syndromic approach is appropriate to treat this woman?

- | | |
|-------------------|------------------------|
| (A) Genital ulcer | (C) Urethral discharge |
| (B) Genital bubo | (D) Vaginal discharge |

44. A 40-year-old mother came to an emergency OPD complaining of severe dizziness and drowsiness of week duration. She often drinks coffee or tea just after every meal. She dislikes eating fruits, vegetables, meat and grains. Her blood pressure is 90/60 mmHg, pulse rate is 100/min, temperature is 36.5 degree centigrade and respiratory rate is 24/min, and she is pale. On investigation, hemoglobin is 9.5 g/dl. However, laboratory results of CBC, stool exam and urine analysis are normal.

What is the diagnosis of this patient?

- | | |
|----------------------------|------------------------|
| (A) Iron deficiency anemia | (C) Physiologic anemia |
| (B) Megaloblastic Anemia | (D) Sickle cell anemia |

45. A 22-year-old primigravid woman came for an antenatal care follow-up for the first time. She lives in malaria endemic area but has no history of malarial attack. Her vital signs are within normal range and she has pink conjunctiva and mucous membranes. The hemoglobin level is 12g/dl.

What is the most appropriate measure to minimize the risk of malaria infection for this woman?

- (A) Intermittent preventive treatment
- (B) Prescribing Iron supplementation
- (C) Use of long lasting insecticidal bed net
- (D) Provision of personal protective equipments

46. A 38-year-old Gravida-VII Para-VI mother came for antenatal care follow-up at 32 weeks of gestation from known hookworm endemic area. Her vital signs are within the normal range, conjunctiva and mucus membranes are pink, and hemoglobin is 13 gm/dl.

What prophylactic drug should be provided to this mother?

- | | |
|------------------------------------|---|
| (A) Mebendazole 500 mg PO stat | (C) MgSo4 4 g IV for 24 hours after birth |
| (B) Ampicillin 2 g IV for two days | (D) Amoxicillin 500 mg BID for five days |

47. A pregnant woman at 34 weeks of gestation was presented to a hospital with a complaint of absence of fetal movement for 24 hours. She has no other danger signs of pregnancy. On physical examination the vital signs were in the normal range, but fetal heart beat was not heard and after counseling she was appointed to return back after 2 weeks.

What is the most appropriate issue that has to be addressed while counseling the woman?

- | | |
|-------------------------------|---------------------------|
| (A) Danger signs of pregnancy | (C) Neonatal danger signs |
| (B) Exclusive breast feeding | (D) Additional nutrition |

48. A 35-year-old Gravid-V, Para-II, Abortion-II mother at 20 weeks gestation as estimated by first trimester ultrasound came to a health center for ANC service utilization. She is known diabetic mother on insulin. Physical examination and laboratory investigation showed normal findings. Finally, the midwife sent her for ultrasound evaluation.

What is the importance of ultrasound evaluation for this mother?

- (A) To identify gross congenital anomaly (C) To calculate amniotic fluid volume
(B) To determine expected fetal weight (D) To estimate the gestational age

49. On date 25/05/2015 E.C a 28-year-old primigravida mother was presented to a labor triage with a complaint of pushing down sensation for the past three days. Upon history, her first day of LNMP was on 29/8/2014 E.C.

What is the calculated gestational age for this mother?

- (A) 37 weeks and 3 days (C) 39 weeks and 6 days
(B) 38 weeks and 5 days (D) 40 weeks and 4 days

50. A Gravida-IV, Para-II woman came to a hospital with a complaint of pushing down pain. Abdominal examination revealed, longitudinal lie, cephalic presentation, FHB 140/min, and descent 3/5 palpable above symphysis pubis.

What is the most appropriate physical examination to identify fetal lie?

- (A) Fundal palpation (C) Lateral palpation
(B) Fundal grip palpation (D) Deep pelvic palpation

51. A 25-year-old Gravida-II, Para-I labouring woman came to a labour ward with a complaint of pushing down pain. On abdominal examination, fetal lie is longitudinal, cephalic presentation, descent 0/5, FHR is 140/min, has three strong contractions. Pelvic examination revealed 9cm dilated cervix with ruptured membrane three hours back.

What is the stage of labour?

- (A) Active first stage labour (C) Second stage labour
(B) Latent first stage labour (D) Third stage labour

52. A midwife at a labour ward is following a woman in labour. As the woman had ante partum hemorrhage secondary to placental abruption, the fetal condition is thoroughly monitored with CTG. The midwife is ordered to trace fetal heart and notify the doctor if things are going abnormal.

What finding is most reassuring in tracing fetal heart rate?

- (A) Fetal heart rate of 140 with absent variability
(B) Fetal heart rate of 160 with marked variability
(C) Fetal heart rate of 120 with minimal variability
(D) Fetal heart rate of 150 with moderate variability

53. A midwife on duty follows the progress of a labor by plotting on the partograph. The day duty midwife handed over to the next night duty shift. When the new midwife evaluates the partograph, he noticed that the letter 'I' is plotted for the amniotic fluid assessment.

What interpretation of the partograph indicates the status of amniotic fluid?

- | | |
|-------------------------|----------------------------|
| (A) Blood-stained fluid | (C) Meconium-stained fluid |
| (B) Membranes ruptured | (D) Membranes not ruptured |

54. A 36-year-old Gravida-I, Para-0 pregnant woman came to a health facility complaining of pushing down pain for three hours duration. She also has headache and lower extremity swelling. On evaluation, her blood pressure is 140/90mmHg and she is in active first stage labor. For further investigation, she was sent for urine analysis to check the presence of protein, glucose, blood, and leukocyte esterase.

What is the most revealing laboratory finding for the above case?

- | | |
|-----------------|-------------------------------|
| (A) 4+ protein | (C) Negative glucose |
| (B) Trace blood | (D) Absent leukocyte esterase |

55. In a remote health center where there is no electricity supply, the refrigerator is not functioning. For that reason, the health center made injectable uterotonic drugs unavailable. A midwife tries to manage third stage by physiological approach.

What is the most likely course the midwife has to observe to deliver the placenta?

- | | |
|--------------------------|--------------------------------------|
| (A) Uterus becomes soft | (C) Umbilical cord protrudes further |
| (B) Until brisk bleeding | (D) Remove the placenta manually |

56. A laboring woman was admitted in labor ward with first stage labor. On examination, all findings are normal. A midwife told the woman to ambulate in the room. However, the woman prefers to stay on the bed.

What appropriate position is preferable for this woman?

- | | |
|------------------------------|----------------------------|
| (A) Dorsal supine position | (C) Right lateral position |
| (B) Any comfortable position | (D) Prone supine Position |

57. A 26-year-old laboring woman is in the second stage labour and transferred to delivery room. The midwife is to begin assisting birth of the fetal head.

What is the most effective management plan to deliver the head?

- (A) Wait for the movements of restitution and external rotation
- (B) Maintain flexion to prevent its early extension and to regulate its slow escape
- (C) Insert fore finger of each hand under the axillae and deliver gently by lateral flexion
- (D) Traction made on bisacromial diameter to place in anteroposterior diameter of the pelvis

58. A multigravid woman came to a labour ward with second stage labor. A midwife transferred her to a delivery room and started to conduct birth by preparing a uterotonic drug for active management of third stage labor.

What most important precaution should be applied before the drug administration?

- (A) Massage the uterus to facilitate the birth of placenta
- (B) Apply controlled cord traction to deliver the placenta
- (C) Palpate the abdomen to rule out presence of additional fetus
- (D) Instruct the woman to push down when she feels contraction

59. A primiparous woman is in active first stage of labour and cervix is 6 cm dilated. She failed to make progress on two subsequent examinations. The labour ward team has planned to evaluate the adequacy of contractions through direct measurement.

What is the most precise method to evaluate uterine activity in this woman?

- (A) Manual palpation
- (B) Serum oxytocin levels
- (C) External tocodynamometry
- (D) Intrauterine pressure catheter

60. A 28-year-old laboring woman is admitted to a labour ward with a diagnosis of active first stage of labour. Reassessment was done after four hours, and she had irregular and infrequent contractions, rate of cervical dilatation slower than 1 cm per hour and presenting part poorly applied to the cervix. The midwife noticed about the progress.

What is the most appropriate explanation of this labor progress?

- (A) Unsatisfactory progress
- (B) Satisfactory progress
- (C) Precipitated labor
- (D) Prolonged labor

61. A 32-year-old primigravida woman was admitted to a labor ward with the diagnosis of prolonged second stage labor due to poor uterine contraction. On abdominal examination, there is term size uterus with longitudinal lie and the fetal heartbeat is 110/min. During vaginal examination, there is vertex presentation lie on ischial spine.

What is the most important parameter that describes a prerequisite for vacuum delivery?

- (A) Station -1
- (B) Station 0
- (C) Bradycardic FHR
- (D) Poor uterine contraction

62. A 36-year-old Gravida-VII Para-VI mother came to a hospital after two days duration of pushing down pain. On history, she had frequent and strong uterine contraction. Upon abdominal examination, no contraction and negative fetal heart rate identified. Then a midwife diagnoses the case as uterine rupture.

What is the most likely sign of uterine rupture that describes this case?

- (A) Constriction ring
- (B) Ceased fetal movement
- (C) Titanic uterine contraction
- (D) Sudden cessation of contraction

63. A 36-year-old multipara mother came with referral for the diagnosis of shock after having a profuse vaginal bleeding following delivery of a 3800gm alive male neonate with spontaneous vaginal delivery.

What most likely sign and symptoms of shock would help the midwife to diagnose the case?

- (A) Urine output 40ml/hr
- (B) Slow respiration rate
- (C) Pulse rate greater than 110/min
- (D) Systolic blood pressure greater than 90 mmHg

64. A 38-year-old multi para mothers delivered a 4000 gm alive female neonate an hour ago. During her immediate post partum period, she bleeds profusely. On examination, her blood pressure is 90/30 mmHg, pulse rate is 112/min and temperature 35 degree centigrade. She had no history of medical disorder.

What is the most likely type of shock?

- (A) Septic shock
- (B) Cardiogenic shock
- (C) Anaphylactic shock
- (D) Hypovolemic shock

65. A 34-year-old Gravida-III Para-II mother came to a hospital with complaint of vaginal bleeding and abdominal pain. On history, she had no difficult delivery and her LNMP was not known .On ultrasound scan, gestational age was 32weeks, breech presentation and the placenta is in upper uterine segment.

What is the most likely diagnosis for this case?

- (A) Ruptured uterus
- (B) Antepartum hemorrhage
- (C) Local lesion of the cervix
- (D) Second trimester abortion

66. A 35-year-old Gravida-III Para-II mother came to a hospital on 26-05-2013E.C.On history, she did not remember her LMP. Her first trimester ultrasound scan on 12/10/2012E.C showed, single intrauterine viable fetus and gestational age from CRL is 9+3weeks. On abdominal examination, there is term sized uterus longitudinal lie, cephalic presentation and FHB is 124/min.

What is the most likely diagnosis?

- (A) Normal third trimester pregnancy
- (B) Intra uterine fetal death
- (C) Post term pregnancy
- (D) Twin pregnancy

67. A 35-year-old Gravida-V Para-IV woman visits the hospital complaining gush of fluid per vagina. She has no history of foul smelling vaginal discharge, dysurea and urgency. The gestational age makes 39weeks from LNMP. Her blood pressure is 110/60mmHg, pulse rate is 110/min and temperature is 38 degree centigrade. She has tenderness on abdominal examination. The FHR is 182/min. On inspection, there is leakage of fluid per vagina. There is no costo-vertebral angle tenderness.

What is the most likely diagnosis?

- | | |
|-----------------------|---------------|
| (A) Chorio-amnionitis | (C) Vaginitis |
| (B) Pylonephritis | (D) Cystitis |

68. A 34-year-old Gravida-II Para-I woman was admitted to a labor ward with the diagnosis of active first stage labor .Gestational age makes 39weeks from her LNMP. On abdominal examination, there is longitudinal lie breech presentation, FHB 112/min and Vaginal examination after two hours the cervix is fully dilated and station is 0, with spontaneous rupture of membrane cord will be visible though the vulva.

What is the most likely management plan for the case?

- | | |
|----------------------|----------------------------------|
| (A) Cesearan section | (C) Breech extraction delivery |
| (B) Vacuum delivery | (D) Spontaneous vaginal delivery |

69. A 35-year-old Gravida-III para-II mother came to a hospital with pushing down pain of labor. Up on assessment, her gestational age is 38 weeks .On abdominal examination, there is term sized uterus with longitudinal lie, breech presentation and FHR 124/min. On vaginal examination, the cervix is fully dilated and station is 0. Frank breech is diagnosed, with progressive pushing the lower extremities is delivered spontaneously but the upper extremities and the shoulders fail to be delivered spontaneously.

What maneuver is needed for this delivery?

- | | |
|---------------------|-------------------------------------|
| (A) Burns–Marshall | (C) Wigand Maneuver |
| (B) Lovset maneuver | (D) Mauriceau Smellie Veit maneuver |

70. A 35-year-old primigravida mother was referred from a health center for difficult labor. On physical examination, she is dehydrated, tachycardic, having low blood pressure and febrile. On abdominal examination, there is titanic uterine contraction, bandl ring, three tumor abdomen and FHB is negative .The cervix if fully dilated with excessive molding and high station.

What is the appropriate management plan?

- | | |
|--------------------------|----------------------------------|
| (A) Forceps delivery | (C) Cesarean section delivery |
| (B) Destructive delivery | (D) Spontaneous vaginal delivery |

71. A 36-year-old Gravida-VI Para-V mother came to a hospital with complaint of absent fetal movement. She has no history of medical disease. The calculated gestational age from LNMP is 38weeks. On physical examination, her vital sign is in the normal range. On abdominal examination, there is 34weeks sized uterus, longitudinal lie, cephalic presentation and FHB is negative. An ultrasound confirms absence of cardiac activity and all laboratory investigations are normal.

What is the most likely management plan?

- (A) Immediately ripe the cervix (C) Cesarean section
(B) Give time for spontaneous labor (D) Induction upon diagnosis

72. A 34-year-old Gravida-III, Para-II woman was admitted to a labor ward with a diagnosis of active first stage of labor. She has no history of medical disease and a midwife started to follow with a partograph. The midwife detected and reported the presence of NRFHR as a result emergency cesarean section was done. On the third postoperative day she developed pain and edema of the leg and thigh.

What is the most appropriate post operative care to be given to the patient?

- (A) Initiate Early ambulation
(B) Administration of antibiotics
(C) Initiate breast feeding within one hour
(D) Remove the catheter after eight hours of operation

73. A midwife working in labor ward is following a laboring woman in active first stage labor with partograph. Plotting started at 9:00am and all findings were non-revealing. After one hour, the membrane ruptured spontaneously and the midwife noticed that amniotic fluid is meconium stained and FHR became tachycardic.

What is the most appropriate next step to evaluate fetal condition?

- (A) Oxygen via tightly fitting face mask (6-8 liters /min)
(B) Correct maternal hypotension and dehydration
(C) Vaginal examination to exclude cord accident
(D) Decrease uterine activity by using tocolytics

74. A 28-year-old Gravida-II, Para-I laboring woman is admitted to a health center with a diagnosis of active first stage labour with 6cm cervical dilatation, intact membrane and zero station. When the midwife reassessed her progress after four hours, she has 3 moderate contractions in 10 minutes, rate of cervical dilatation slower than 1 cm per hour which is 8cm and bulged membrane. The midwife decided to take an action for the poor progress.

What is the most appropriate management plan based on the reassessment findings?

- (A) Rupture the membrane after excluding cord presentation
(B) Put the woman on IV fluid and augment with oxytocin
(C) Inform the family to arrange for referral to hospital
(D) Re-check the cervix to ensure cervical dilatation

75. A 34-year-old multiparous mother reported moderate fever associated with shortness of breath and chest pain. Her respiratory rate is 22/min and pulse rate revealed 110/min. At the time of examination she has hemoptysis and temperature of 38.5 degree centigrade. She has history of venous thrombosis.

What lab investigation is highly suggestive in excluding the diagnosis of this case?

- | | |
|--------------------------------|--------------------------------------|
| (A) Whole blood count | (C) Arterial blood gas investigation |
| (B) Determine clotting factors | (D) Investigate the level of D-Dimer |

76. A 27-year-old woman living with HIV came with a complaint of pushing down pain of 10 hours duration. She is on HAART. On physical examination, cervix is fully dilated and the presenting part is protruded through vaginal opening. The midwife is giving care for this high risk mother.

What is the most appropriate newborn care based on the above scenario?

- | | |
|---|---------------------------------------|
| (A) Slap back of the newborn | (C) Give care in separate room |
| (B) Cut cord under cover of light gauze | (D) Routine suction of nose and mouth |

77. A 22-year-old primipara woman came for a follow up after she gave birth through cesarean section under spinal anesthesia with an indication of obstructed labor secondary to cephalo-pelvic disproportion. She complains occasional headache located in the forehead and deep behind the eyes. Vital signs are in the normal range.

What is the most likely factors that gave rise to this condition?

- | | |
|-------------------------|-----------------------------|
| (A) Puerperal pyrexia | (C) Post puncture headache |
| (B) Puerperal psychosis | (D) Postpartum preeclampsia |

78. A 32-year-old laboring mother who has no history of antenatal care follow up is diagnosed for HIV during labor. She gave birth for a baby girl within three hours of ART initiation. The mother has decided to practice exclusive breastfeeding for the first six months.

What is the recommended ARV prophylaxis to be initiated for the baby girl?

- | | |
|----------------------------|---------------------------------|
| (A) NVP daily for 6 weeks | (C) NVP daily for 18 months |
| (B) NVP daily for 12 weeks | (D) NVP twice daily for 4 weeks |

79. A 39-year-old woman is admitted in a maternity ward after being treated for postpartum hemorrhage. The midwife wants to provide counseling on different components of postpartum care.

What most important component has to be emphasized depending on her case?

- | | |
|----------------------------|--|
| (A) Immunization | (C) Nutrition, including iron-rich foods |
| (B) Family planning method | (D) Danger signs of the mother and newborn |

80. A 35-year-old Para-I woman on the 12th day after delivery came to a clinic for postnatal follow up. She has yellow-white color vaginal discharge.

What is the most likely lochia type that can be identified from this case?

- (A) Lochia (B) Lochia alba (C) Lochia rubra (D) Lochia serosa

81. A 22-year-old primipara woman was presented by her family to a health facility on the third day of delivery. She had a sudden onset of fear, restlessness, mania and depression followed by hallucinations. Her family reported that she had suicidal attempt.

What is the most likely psychiatric disorder of the woman?

- (A) Postpartum blue (C) Postpartum psychosis
(B) Postpartum depression (D) Postpartum personality disorder

82. On the seventh postpartum day, a woman came to a postnatal clinic with a complaint of lower abdominal pain for the last three days. On physical examination, she has maternal tachycardia, uterine tenderness and foul smelling vaginal discharge. Additionally, the mother is febrile but all other vital signs are within the normal range.

What laboratory investigation is highly essential for this woman?

- (A) Blood group and cross-match (C) Stool microscopy
(B) Whole blood count (D) Urine protein

83. A 19 year-old Para-I woman came with complaints of urinary urgency, frequency and dysuria on the 4th week of postpartum period. She had vaginal discharge. Her blood pressure is 110/70 mmHg, pulse rate is 90/min, temperature is 37.8 degree centigrade, respiration rate is 22/min. On examination, there is costo-vertebral angle and supra pubic tenderness on palpation.

What diagnosis is most appropriate in this case?

- (A) Malaria (C) Urinary tract infection
(B) Uterine infection (D) Very severe febrile disease

84. A 37-year-old mother who gave birth at home came to a hospital immediately on postpartum period with a complaint of excessive bleeding for six hours. On physical examination, her pulse rate is 100/min, blood pressure is 80/60 mmHg and her skin feels cold. Furthermore, she is not oriented to time, place and person.

What is the most likely diagnosis for the above case?

- (A) Hemorrhagic shock (C) Neurogenic shock
(B) Cardiogenic shock (D) Septic shock

85. A 19-year-old primipara woman came to a clinic with a complaint of painful breast that lasted for seven days duration. She gave birth at home a week ago. On physical examination, her temperature was 40.5 degree centigrade, blood pressure was 110/90mmHg, pulse rate was 84/min, her breast is painful, tender, red, tense and hot, and axillary lymph nodes are enlarged.

What is the most likely complication that this woman has developed?

- (A) Galactoceles (C) Acute mastitis
(B) Breast abscess (D) Breast engorgement

86. A 19-year-old Para-II woman came to a hospital with complaints of high-grade fever and chills of one day duration. On history, she was on active labour for about 24 hours and she was catheterized because obstructed labor was anticipated. Meanwhile, the delivery was assisted with vacuum.

What is the most likely diagnosis of the above scenario?

- (A) Puerperal UTI (C) Normal puerperium
(B) Puerperal pyrexia (D) Puerperal psychosis

87. A 39-year-old woman came to a clinic at the 7th day of post partum period complaining of lower abdominal pain for the last three days. On physical examination, she is febrile with chills, rigor and has generalized abdominal tenderness. She has also abdominal distention and uterine tenderness. Finally, she was diagnosed with postpartum endometritis.

What was the most likely clinical feature that helped to reach the diagnosis?

- (A) Lower abdominal pain and fever (C) Abdominal guarding
(B) Abdominal distention (D) Chills and rigor

88. A laboring mother with a diagnosis of preeclampsia with severity signs is referred from a health center after she took loading dose of magnesium sulfate. The mother spontaneously delivered on her way. She arrived after eight hours of magnesium loading dose. Her current blood pressure is 130/90 mmHg and there was no severity sign.

What is the most appropriate management regarding the maintenance dose of MgSO₄?

- (A) Interrupt magnesium as she has already delivered
(B) Double dose of magnesium as she missed
(C) Continue for 24 hours after loading dose
(D) Continue until 24 hours after delivery

89. A 40-year-old Para-VI, Abortion-I mother came with referral from a nearby health center following her delivery. On physical examination, a blood pressure of 85/45mmHg, pulse rate of 115/min, pallor, cold clammy sweats, cyanosis of the fingers were observed. She is not oriented to person, place and time.

What is the most appropriate initial step in the management of this patient?

- (A) Establishing two or more intravenous ways for blood
- (B) Transfusion of blood of the same group if not available group O-ve
- (C) Establishing an airway and giving oxygen by mask or endotracheal tube
- (D) Administer Crystalloid solution as ringer lactate, normal saline or glucose 5 percent

90. A health center head invited a midwife to provide morning health education for clients and attendants regarding postpartum hemorrhage (PPH) prevention.

What is the most appropriate education component that would help to address the attendants?

- (A) Skilled birth delivery
- (B) Controlled cord traction
- (C) Deworming intestinal parasites
- (D) Uterotonic drug administration

91. On the seventh postpartum day a woman came with a complaint of involuntary leakage of urine for the last three days and was diagnosed with mild stress incontinence. She associated the leakage with sneezing and laughing. The mother also said that she has experienced the leakage recently after delivery. She has no history of chronic cough and constipation.

What is the most appropriate initial management for this woman?

- (A) Pessaries
- (B) Urethropexy
- (C) Kegel exercise
- (D) Midurethral sling

92. A midwife received a phone call from one of a female client who is on ART at PMTCT clinic. She disclosed that she missed taking the drug for more than 15 hours due to some family ceremony at her home.

What is the best measure the midwife shall respond for the adherence?

- (A) Skip the missed dose and continue the next dose as usual
- (B) Take the missed dose and continue the next dose as usual
- (C) Skip the missed dose and double up the next dose
- (D) Take the missed dose and the next dose together

93. A newborn baby immediately after birth showed the following signs and symptoms. Weak respiratory effort, well flexed and blue extremities, grimace, and pulse rate of 90/min.

What is the calculated APGAR score of the newborn?

- (A) 7
- (B) 6
- (C) 4
- (D) 5

94. A newborn baby at the fifth minute of birth was assessed for APGAR score. The findings showed good cry, coughing and sneezing, pulse rate of 105/min, and some flexed and blue extremities.

What is the most likely interpretation for the baby's APGAR score?

- (A) Normal (B) Moderate (C) Very low (D) Low

95. A postnatal mother is counseled on kangaroo mother care; but, she is not willing to carryout it. A provider kept on counseling the mother to convince her on the importance of kangaroo mother care. The provider mentioned that kangaroo mother care facilitates breastfeeding, keeps bonding, maintains temperature, and reduces frequency of apnea.

What is the primary importance of kangaroo mother care?

- (A) Reduces frequency of apnea (C) Keeps the baby warm
(B) Establishes breast feeding (D) Keeps bonding

96. A 25-year-old mother whose gestational age was 33 weeks was presented to a hospital with pushing down pain. On physical examination, her vital signs are in the normal range, 33 weeks sized abdomen, contraction 3/10'/45", and full cervical dilatation. The delivery team ordered an assistant to prepare vitamin K for the newborn care.

How much dose shall the assistant prepare for this case?

- (A) 0.25 mg (B) 0.5 mg (C) 1 mg (D) 2.5 mg

97. A midwife examined a term male newborn baby to identify the presence of primitive reflexes. After supporting the neonate in semi-erect position, the midwife allows the neonate's head to fall backwards onto her hand. She noticed a symmetric extension and abduction of the fingers and upper extremities followed by flexion of the upper extremities and an audible cry.

What is the primitive reflex demonstrated by the baby's response?

- (A) A normal moro reflex (C) A moderate tonic neck reflex
(B) An absence rooting reflex (D) Presence of palmar grasp reflex

98. A midwife evaluated whether or not a mother is practicing optimal breastfeeding for a one-week-old HIV exposed infant. The midwife found out that the baby is taking Nevirapin, feeding the breast exclusively in two to three hours interval day and night and about half of both breasts milk in one feeding. From this evaluation, the midwife realized that the feeding practice is not optimal.

Which breastfeeding practice makes it not optimal?

- (A) The feeding is less frequent
- (B) Breasts are not completely fed
- (C) The baby is taking medication
- (D) The feeding is optimal but the midwife evaluated it incorrectly

99. A five-day-old neonate who was born at home is presented with eye redness and discharge. The baby's mother was healthy throughout pregnancy and followed four antenatal visits. The midwife at neonatal clinic assessed the neonate and reached at the diagnosis of ophthalmia neonatorum and ordered Ceftriaxone 50mg/kg and Erythromycin 50mg/kg orally in four divided doses for 14 days.

What is the most likely history component that supports the diagnosis?

- (A) Unattended birth
- (B) Eye exposure to the sunlight
- (C) Possible maternal infection with STIs
- (D) Frequent eye touching nature of neonates

100. A term newborn baby fails to initiate and sustain breathing at birth. Five minutes after resuscitation, the neonate's re-evaluation result shows a weak respiratory effort, some flexed and blue extremities, no response to stimuli, and a pulse rate of 90/min.

What is the most likely diagnosis of the neonate?

- | | |
|---------------------|------------------------------|
| (A) Hypoglycemia | (C) Perinatal asphyxia |
| (B) Neonatal sepsis | (D) Hyaline membrane disease |

THE END

Ethiopian Midwifery COC Exam 2023

Ethiopian Midwifery COC Exam

1, A postpartum patient was in labour for 30 hours and had ruptured membranes for 24 hours .For which of the following would the nurse be alert?

A, Pelvic thrombophlebitis

B, Salpingitis

C, Endometritis

D, Endometriosis

Scenario 1: Darartu is 26 years old. She came to Jimma University specialized hospitals for delivery service. She is gravida III & para II. She is 38 weeks of gestation. The Nurse conducted PV the findings include palpating cord in front of fetal head, Membrane ruptured& Meconium stained amniotic fluid, Full cervical dilation, No suture overlapping and station zero. On abdominal Auscultation, Fetal heartbeat is decelerating.

2, Based on Scenario 1: What is the possible diagnosis?

A, Oveart UCP

B, Occult UCP

C, Cord presentation

D, Nuchal cord

3, What amount of oxytocin is required in induction and augmentation respectively?

A, 6 IU and 12 IU

B, 6 IU and 6 IU

C, 3 IU and 3 IU

D, 6 IU and 3 IU

4, What is adequate number of contraction to stop Induction and Augmentation?

A, 2 contraction/10 minutes lasting 20-40

C, 3 contraction/10 minutes lasting 40-60

C, 4 contraction/10 minutes lasting 60-90

D, 5 contraction/10 minutes lasting 60-90

5, The Presentation when fetus lies flexed attitude, the legs are flexed on the abdomen, and the presenting part is bulky and consists of buttocks external genitalia and both feet is:

A, Frank breech

B, Complete breech

C, Footling breech

D, Knee presentation

6, If bleeding occurs after 28 weeks or during labour but before delivery we can suspect:

A, Abortion

B, Ectopic

C, Molar pregnancy

D, Ruptured uteruses

Case Scennnario 1: W/ro Sinbon is 38 years old. She comes to a health centre where you are working. Her LMP is 5/12/2007 E.C. Her previous history of gravida include, 20 weeks, 27 weeks termination of pregnancy, 34 weeks dead

fetus were born and she gave birth alive three single female children's and two boys which are twin.

7, Based on Case scenario 1, what are W/ro Sinbon Gravida and Para respectively?

A, 7 and 3

B, 7 and 4

C, 7 and 5

D, 8 and 6

8, The dose of iron given during pregnancy as supplementation

A, 325 mg tid for 7 days

B, 325 mg daily for 3 months

C, 325 mg bid for 3 months

D, 325 mg tid for 3 months

9, Which drug Combination is currently used to PMTCT?

A, AZT + 3TC + NVR

B, TDF + 3TC + EFV

C, TDF + 3TC + NVR

D, AZT + 3TC + EFV

10, If the Female pelvis characterized as Heart shaped brim: Narrow fore-pelvis, The side wall is converged, The ischial spines are prominent and Pubic arch <90.

A, Gynacoid pelvis

B, Anthropoid

C, Platypelloid

D, Android pelvis

9, A patient with pregnancy-induced hypertension is likely to exhibit any of the following symptoms?

- A. .Proteinuria, headaches, double vision.
- B. Headaches,double vision,vaginal bleeding.
- C. Proteinuria,double vision,uterine contractions.
- D. Proteinuria,headaches,vaginal bleeding.

10, A pregnant customer is diagnosed with partial placenta previa. In explaining the diagnosis, the nurse informs the patient that the usual treatment for partial placenta previa is as follows?

- A. Immediate cesarrear delivery
- B. Platelet infusion
- C. Activity limited to bed rest
- D. Labour induction with oxytocin

Ethiopian Midwifery COC Exam

11, The effectiveness of Hormonal contraceptions may be reduced by the following drugs, except:

- A, Rifampicin
- B, Amoxaicillin
- C, Carbamazepine
- D, Phenytoin

12, The sequence to delivery of fetus is 1. Move up ward, 2. Clean the mouth, 3. Delivery the head, 4. Check the cord tie on the neck, 5. Move down ward, based on this

A, 2, 3, 5, 4,1

B, 3, 4,5,2,1

C, 3,2,4,5, 1

D, 1, 2,3,4,5

13, W/ro Aylech is 30 years old and she is gravid IV, come to Yabu Health Center for ANC follow up. On physical examination She is 32 weeks of pregnant and FHB is positive. Her BP 120/70mmhg, PR 88 bpm, laboratory investigation, on urine analysis; Protein is positive, hgb 9 g/dl and VDRL Positive, based on this what is her diagnosis?

A, Anemia and preeclampsia

B, Eclampsia and syphilis

C, Syphilis and preeclampsia

D, Anemia and syphilis

14, W/ro Ayelech is 25 years old and she is para III. She delivered at home and come to Adola Hospital with her mother with the history of vaginal bleeding while you are working at Oby/gyne ward. When you took history from her mother, the bleeding was started immediately after the delivery of the fetus, the placenta was complete, the uterus was well contracted, and then what is the cause of bleeding?

A, Tissue

B, Tone

C, Thrombin

D, Trauma

15 If one laboring mother comes to Shebe Health Center with history of pushing down pain while you are working at maternity ward, she is gravid II. When you perform physical examination FH is 34 weeks, FHB is positive, on vaginal

examination Cervix is fully dilated and station is zero. Which laboratory request is must at this time?

A, Hgb

B, VDRL

C, PITC

D, BG and Rh

16, When comparing medial episiotomy with medolateral episiotomy which on e false?

A, Medial episiotomy is less blood loss than medolateral episiotomy

B, Medial episiotomy heal in less time than medolateral episiotomy

C, Medial episiotomy is less pain full than med lateral episiotomy

D, Medial episiotomy is less extension to anal sphincter than medolateral episiotomy

17, W/ro Chaltu is 28 years old. She wants to use natural method of family planning. She adjusts her menstrual cycle for the first six months. She saw her menstrual cycle on the 1st month on December 28, 2015, on the 2nd month January 27, 2016, 3rd on February 29, 2016, 4th on March 25/2016, 5th month on April 30/2016 and on the last month May 26/2016.

Based on this when does she protect from sexual intercourse to avoid unplanned pregnancy?

A, 6–20

B, 7–20

C, 7–19

D, 6–19

18, A woman has undergone elective abortion one week ago. Now she comes to the clinic with the chief complaint of hemorrhage. In PE, the cervix is closed, the uterus is contracted without tenderness. Her temperature is normal. What is the best treatment?

- A, Doxy 100 mg bid for two weeks
- B, Again dilate and evacuate uterus
- C, Observation and check of Hg and Hct
- D, Hormone therapy

19, Case seniero W/ro Ayantu is 32 years old comes to Asedabo Health Health center while you are working at labour ward with history of pushing down pain. She is gravid V, Para IV (Two are alive and one still birth) She has delivered her last pregnancy after two days of labour. She has history of vesico vaginal fistula repairing two years back at JUSH. After you have done physical examination you have got the following findings: BP 100/70 mmhg, PR 96 bpm, FHB 146/min cervix is 4cm dilated,

- A, Based on this what is your plan for w/ro Ayantu?
- B, Follow up the labour by partograph
- C, Catheterize her and follow up the labour by pantograph
- D, Catheterize her and refer to the Hospital for immediate c/s to avoid recurrence fistula
- E, Catheterize her and refer to the Hospital

20, The third stage of labour:

- A, The routine use of oxytocics has been shown to improve hemoglobin level
- B, Ergometrine should be routinely used in all patients in the third stage

C, Ergometrine should be routinely used in all stage III patients

D, The incidence of retained placenta after vaginal delivery is 2 per cent

E, Waiting 60 min rather than 30 min will have the number of women requiring anesthetics for manual removal

21, The woman came with bright red vaginal bleeding; her gestational age is 30 weeks, on examination the lie is transverse, no pain during palpation thus the cause of bleeding might be:

A, Placenta abruption

B, Placenta previa

C, Local lesion.

D, Abortion

22, The four pronged National Strategies of prevention of MTCT includes all, except

A, Primary prevention of HIV infection.

B, Prevention of unintended pregnancies among HIV infected women

C, Prevention of unwanted pregnancies in HIV-infected women

D, Treatment, care and support of HIV infected women, infants and their families

23, Which one of the following intervention is important in reducing MTCT during labor and delivery?

A, Shorten second stage by using vacuum

B, Perform artificial rupture of membrane to fasten the first stage of labor

C, Follow labour using partograph and minimize vaginal examination

D, Assist delivery by performing episiotomy

24, A 25-year-old man could not father children. He and his wife are undergoing an infertility test. His wife's fertility is normal. On physical examination, he has descending testicles that appear to be of normal size. A testicular biopsy is performed and only Sertoli cells are present in the seminiferous tubules on microscopic examination. Which of the following laboratory test results is most likely?

- A, Decreased sperm count
- B, Elevated serum human chorionic gonadotrophin
- C, Decreased serum luteinizing hormone
- D, Increased serum testosterone
- E, Increased serum alpha-fetoprotein

25, A 21 year old woman comes in for first prenatal visit .On LMP she is 12 wks of she was certain. Upon examination you noted 20 wks uterus, therefore an US is performed and revealed bilaterally enlarged adnexa and a snowstorm pattern in the uterus . Based on these findings what is diagnosis of her?

- A, Ectopic pregnancy
- B, Twin pregnancy
- C, Molar pregnancy
- D, Polyhydroamniotic

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Time Allowed 1 hour and 50 Minutes

ID Number _____ **year** _____

General instruction

- ✓ The exam has parts only one part:
 - **Multiple Choice Questions # 120**
- ✓ Do not take Mobile phones into examination room. You must not keep it in your hand or put it in front of you even if it is **switched off** until the examination paper is handed over to the invigilator.
- ✓ Any form of cheating will automatically result **Disqualification**
- ✓ Don't start exam until you told to do so by **your invigilator**.
- ✓ **Don't encircle the questions, only fill on Answer sheet**

READ INSTRUCTIONS CAREFULLY BEFORE ANSWERING THE QUESTIONS.

GOOD LUCK

Multiple choice questions
Choose the best answer from given alternatives and encircles
the a fill on answer sheet

1. Steps for instrument possessing
 - A. Cleaning, decontamination, high level disinfection, proper Storage.
 - B. Decontamination, cleaning ,high level disinfection, , proper storage
 - C. Proper Storage, decontamination , cleaning, high level disinfection
 - D. Cleaning, proper Storage, decontamination, high level disinfection
2. A Midwife in maternity ward knows that the second step for the management of PPH is directed therapy one of the following is correct statement:
 - A. Manual removal, Curettage for thrombin
 - B. Correct inversion, repair laceration for tissue
 - C. Replace factors identify rupture for tissue
 - D. Give oxytocin , massage, compress for tone
3. A Midwife is caring for post partum client after third post partum day: Client history reveal that para III and has history of labour greater than 18 hour she give birth dead male neonate due to distress in labour, on physical examination pink conjunctiva moist tongue, breast firm tender, and warm and appear tense , the intervention may be:
 - A. Incising and draining the abscess.
 - B. Encouraging the mother to feed.
 - C. Restriction of breast stimulation
 - D. Administering intensive antibiotic
4. The patient hospitalized due to pelvic inflammatory disease is maintained on bed rest and is usually placed:
 - A. Semi-Fowler's position
 - B. Supine position
 - C. Prone position
 - D. Left lateral position

5. Several hours after delivery when assessing a client episiotomy the nurse find there is edema with severe ecchymosis, and the client is complaining of severe rectal pressure the fundus is firm. The vital signs are pulse 108, BP 105/60 this assessment most likely indicate
- A. Urinary tract infection
 - B. Uterine infection
 - C. Vaginal hematoma
 - D. Postpartum hemorrhage
6. Preventive measure for cervical cancer includes all of the following except:
- A. Education related to safe sex practice
 - B. Limiting child bearing two three years
 - C. Regular pelvic examination
 - D. Pap test
7. It helps the Midwife to differentiate Hydatidiform mole from other disorders
- A. Expulsion of grapelike vesicles
 - B. Bright red bleeding
 - C. Continuous passage of urine
 - D. Expulsion of fetus.
8. The Midwife is responsible to teach the client for correct procedure on:
- A. Pap test
 - B. Mammography
 - C. Breast self examination
 - D. Clinical breast examination
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 - B. 7--20
 - C. **7--19**
 - D. 6--19
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 - B. Again dilate and evacuate uterus
 - C. **Observation and check of Hg and Hct**
 - D. Hormone therapy

A. Tissue

C. Thrombin

B. Tone

D. Truama

25. If one laboring mother comes to Shebe Health Center with history of pushing down pain while you are working at maternity ward, she is gravid II. When you perform physical examination FH is 34 weeks, FHB is positive, on vaginal examination Cervix is fully dilated and station is zero. Which laboratory request is must at this time?

A. Hgb

C. PITC

B. VDRL

D. BG and Rh

26. When comparing medial episiotomy with medolateral episiotomy which one is false?

A. Medial episiotomy is less blood loss than medolateral episiotomy

B. Medial episiotomy heal in less time than medolateral episiotomy

C. Medial episiotomy is less pain full than med lateral episiotomy

D. Medial episiotomy is less extension to anal sphincter than medolateral episiotomy

27. W/ro Chaltu is 28 years old. She wants to use natural method of family planning. She adjusts her menstrual cycle for the first six months. She saw her menstrual cycle on the 1st month on December 28, 2015, on the 2nd month January 27, 2016, 3rd on February 29, 2016, 4th on March 25/2016, 5th month on April 30/2016 and on the last month May 26/2016. Based on this when does she protect from sexual intercourse to avoid unplanned pregnancy?

A. 6--20

C. 7--19

B. 7--20

D. 6--19

28. A woman has undergone elective abortion one week ago. Now she comes to the clinic with the chief complaint of hemorrhage. In PE cervix is closed, uterine is contracted with no tenderness. Her temperature is normal. What is the best treatment?

A. Doxy 100 mg bid for two weeks

B. Again dilate and evacuate uterus

C. Observation and check of Hg and Hct

D. Hormone therapy

29. Case seniero W/ro Ayantu is 32 years old comes to Asedabo Health Health center while you are working at labour ward with history of pushing down pain. She is gravid V, Para IV (Two are alive and one still birth) She has delivered her last pregnancy after two days of labour. She has history of vesico vaginal fistula repairing two years back at JUSH. After you have done physical examination you have got the following findings: BP 100/70 mmhg, PR 96 bpm, FHB 146/min cervix is 4cm dilated, Based on this what is your plan for w/ro Ayantu?
- A. Follow up the labour by partograph
 - B. Catheterize her and follow up the labour by pantograph
 - C. Catheterize her and refer to the Hospital for immediate c/s to avoid recurrence fistula
 - D. Catheterize her and refer to the Hospital**
30. The third stage of labour:
- A. The routine use of oxytocics has been shown to improve hemoglobin level**
 - B. Ergometrine should be routinely used in all patients in the third stage
 - C. Ergometrine may be used safely in management of the third stage of labour
 - D. The incidence if retained placenta after vaginal delivery is 2 per cent
 - E. Waiting 60 min rather than 30 min will have the number of women requiring anesthetics for manual removal
31. The women came with bright red vaginal bleeding; her gestational age is 30 weeks, on examination the lie is transverse, no pain during palpation thus the cause of bleeding might be:
- A. Placenta abruption
 - C. Local lesion.
 - B. Placenta previa**
 - D. Abortion
32. The four pronged National Strategies of prevention of MTCT includes all, except
- A. Primary prevention of HIV infection.
 - B. Prevention of unintended pregnancies among HIV infected women
 - C. Prevention of MTCT among HIV non-infected women**
 - D. Treatment, care and support of HIV infected women, infants and their families
33. Which one of the following intervention is important in reducing MTCT during labor and delivery?
- A. Shorten second stage by using vacuum
 - B. Perform artificial rupture of membrane to fasten the first stage of labor
 - C. Follow labour using partograph and minimize vaginal examination**
 - D. Assist delivery by performing episiotomy

34. A 25-year-old man has been unable to father children. He and his wife undergo an infertility workup. His wife's reproductive function is normal. On physical examination he has descended testes that appear normal in size. A testicular biopsy is performed and on microscopic examination only Sertoli cells are present in the seminiferous tubules. Which of the following laboratory test findings is the most likely to have?
- A. Decreased sperm count
 - B. Elevated serum human chorionic gonadotrophin
 - C. Decreased serum luteinizing hormone
 - D. Increased serum testosterone
 - E. Increased serum alpha-fetoprotein
35. A 21 year old woman comes in for first prenatal visit .On LMP she is 12 wks of she was certain. Upon examination you noted 20 wks uterus, therefore an US is performed and revealed bilaterally enlarged adnexa and a snowstorm pattern in the uterus . Based on these findings what is diagnosis of her?
- A. Ectopic pregnancy
 - B. Twin pregnancy
 - C. Molar pregnancy
 - D. Polyhydroamniotic
36. W/ ro Talile is 30 years old. She is 10 weeks pregnancy when she presents at Shenen Gibe Hospitals while you are working at maternity ward with history of light vaginal bleeding. This is Talile's second pregnancy. The pregnancy was planned and wanted. After you have completed your assessment of Talile's and your main finding including the following: Talile's BP 100/70mmhg, her PR 92bpm her temperature is 37.4She has no skin pallor. She has slight lower abdominal pain and light vaginal bleeding; her uterus is corresponding to the date; She has no uterine tenderness, no cervical motion tenderness. The cervix is closed; based on the above what is Talile's diagnosis?
- A. Threatened abortion
 - B. Inevitable abortion
 - C. Incomplete abortion
 - D. Missed abortion
37. Chaltu is 17 years old. She has normal menstrual flow for 6 months after her first menses. Now she starts with heavy bleeding which lasts many days. The pelvic findings by Ultrasonic scanning are normal. What will be the possible diagnosis of Chaltu?
- A. Puberty menorrhagia
 - B. Metropathia Haemorrhagica
 - C. Premenopausal Menorrhagia
38. All are true regarding post-menopausal bleeding except
- A. It can only be determined after 12 months of spontaneous amenorrhoea for a woman not on Hormonal replacement therapy(HRT)
 - B. Endometrial evaluation is needed for any postmenopausal woman on HRT less than 6 months with persistent uterine bleeding
 - C. The major cause of post-menopausal bleeding is endometrial cancer
 - D. Endometrial evaluation is needed for any woman previously amenorrheic on HRT who begins bleeding without apparent cause

39. Fenet is 21 years old. She has heavy menstrual flow, which lasts 11 days. She is married. She is Para I and gravid II. She is using Injection hormonal contraceptives. Which History is the most important to diagnosis the problem?
- A. Psychosocial History
 - B. Surgical History
 - C. Gynecologic History
 - D. Medical history
40. Which one of the following is not true concerning management of Vesico-vaginal fistula?
- A. It will heal if the bladder is drained continuously for 21 to 28 days
 - B. Surgery can be conducted as soon as it is diagnosed
 - C. Most of the cases is repaired vaginally
 - D. If the cause is cancer, biopsy should have been conducted before surgery is performed
41. All are true concerning Clinical Manifestations of Vaginal Prolapse except
- A. The patient may report a sense of pelvic pressure
 - B. Back pain and pelvic pain may occur
 - C. Rectocele resembles all symptoms cystocele
 - D. Prolapse can result in feelings of pressure and ulcerations and bleeding
42. W/ro Asiya is 28 years old. She is gravida II Para I. She gave birth one month ago. She complains loss of Urine during standing position. What is possible diagnosis of W/ro Asiya?
- A. Urge incontinence
 - B. Stress incontinence
 - C. Total Incontinence
 - D. Overflow incontinence
43. W/ro Bontu is 28 years old. She is gravid II and Para II. She complains pain of the lower out quadrant of her breast on left breast. Breast examination reveals, there is bloody discharge from nipple and orange peel skin of breast. Mammographic examination shows the tumor size of 2cm. The X-ray shows enlargement in supra clavicular area and pectorals muscle involvement. She is diagnosed with breast cancer. What is the Stage of the cancer?
- A. Stage III
 - B. Stage II
 - C. Stage I
 - D. Stage IV
44. Which one of the following is not true about management of Urinary incontinency?
- A. Behavioral intervention can be the only management
 - B. Behavioral intervention can be integrated to Medical management
 - C. Behavioral intervention can be integrated with surgical management
 - D. None

45. Which one of the following urinary incontinency is occur due to a problem related to postoperative period?
- A. Overflow incontinency
B. Stress incontinency
C. Urge incontinency
D. Total incontinency
46. If the presenting part of the fetus is 1 cm above the ischial spines, the station will be recorded as:
- A. +2
B. -1
C. +1
D. -2
47. In obstetrics ward A 28 year's old Primigravida mother was gave birth two days ago by cesarean section due to cephalo pelvic disproportion secondary to contracted pelvis. You are the one who is giving post partum care for the mother. After all you want to council her postpartum care both for her child and herself before discharge. What is the optimal birth spacing you counsel the mother to minimize adverse health effect of the child?
- A. Three years and older
B. Two years and older
C. Four years and older
D. One year and older
48. Which one of the following is incorrect regarding the GDM during post Partum period?
- A. Insulin is not required in the first 24 hour or only one forth of previous dose.
B. Maternal insulin requirement fall because fall in HPL after Placenta separation.
C. If the mother has elevated blood glucose and she is lactating provide Oral hypoglycemic
D. The baby born from mother with GDM may develop Hypoglycemia.
49. WHO recommendations for prevention and control of malaria during Pregnancy in sub-Saharan Africa includes all of the following except
- A. Intermittent preventive treatment.
B. Febrile malaria case management.
C. Insecticide treated bed net
D. Continues preventive treatment.
50. Among the following one considered as commonest causes of Mastitis
- A. Candida albicanus.
B. Staphylococcus aureus.
C. Escherichia coli.
D. Clostridium welchi

51. A midwife receive 5 client in postnatal ward from a duty midwife after one hour of delivery from those client in postnatal unit one women develop PPH: the client history reveled she is para six and elevated BGL after 28 weeks of gestation the whole phase of labour completed within 2 hour on physical examination that uterus soft and relaxed based of the history the midwife suspect that the risk for developing this complication might be
- A. Functional/anatomic distortion and infection
 - B. Uterine muscle exhaustion and over distention**
 - C. Uterine anomalies fibroid and multiple gestations
 - D. Intra amniotic infection due to prolonged PROM
52. A client at 36 weeks of gestation with history of one spontaneous delivery being induced the Midwife recognize that possible indication for induction might be
- A. PROM**
 - B. Prolonged pregnancy
 - C. Multiple pregnancy
 - D. Multiparity
53. Midwife understand “retained placenta”
- A. Related to time after full dilation
 - B. Related to time after birth of baby**
 - C. Abnormal throphoblast invasion
 - D. Related to time after active phase
54. A Midwife identify cord prolapsed with absent FHB cervical dilatation 8cm station +1 the appropriate intervention might be
- A. Elevate the foot of the bed
 - B. Pushing back presenting part
 - C. Wait for SVD**
 - D. Immediate cesarean section
55. Primary PPH is bleeding which occur
- A. From 24 hour to 6 week.
 - B. The first 24 month
 - C. The first 24 hour**
 - D. After 24 hour
56. Among the following, one is true about forceps and Vacuum delivery.
- A. If forceps fails vacuum can be used and Vise versa
 - B. Vacuum helps to rotate the head of the fetus
 - C. If the station is above zero Forceps can be used
 - D. Forceps can be used for pre-term labour**

57. The loading dose of magnesium sulfate is given via
- A. IV over 5 minutes, followed by deep IM injection into each buttock
 - B. IV over 5 minutes, followed by deep IM injection into one buttock
 - C. Simultaneous IV and IM injections
 - D. IV bolus, followed by deep IM injection into each buttock
58. Midwife is caring for post partum client after forceps birth with local anesthesia what the midwifery action is appropriate in the immediate post partal period?
- A. Assess the client perineum for hematoma
 - B. Check the client dressing every 30 minute
 - C. Asses bishop score for oxytocin administration
 - D. Teach the client to cough and deep breathe
59. Painful menstruation commonly associated with to pathology
- A. Secondary dysmenorrhea
 - B. Primary dysmenorrhea
 - C. Primary amenorrhea
 - D. Secondary amenorrhea
60. Which one of the following is false about post partum psychosis?
- A. The cause can be infections, drug intoxication, toxemia & blood loss
 - B. The symptoms includes hallucination, delusion, feelings of not wanting to care for the baby , suspiciousness & confusion
 - C. Suicide and infanticide are very rare
 - D. Subsequent pregnancies are associated with an increased risk to develop the disorder
61. A 28wks pregnant mother with the c/m of intractable vomiting and nausea and decreased BP and increased pulse with weight loss and dry tongue is diagnosed as
- A. Morning sickness
 - B. Mulitply pregnancy
 - C. Hydated formole
 - D. Hyperemesis gravidrum
62. Chronic hypertension with new protein urea is defined as
- A. Preeclampsia superimposed on chronic hypertension
 - B. Eclampsia
 - C. Chronic hypertension
 - D. Pre-eclampsia
63. The diagnostic criteria for mild preeclampsia is
- A. Diastolic BP $\geq$ 90-110 and Proteinuria 3+
 - B. Diastolic BP Between 100 and 110 mm Hg and Proteinuria 2+
 - C. Diastolic BP $\geq$ 90-110 and Proteinuria 2+
 - D. Diastolic BP $\geq$ 110 mm Hg or more and Proteinuria 3+

64. The treatment of metritis should include
- A. IV ampicillin or IV gentamicin or IV metronidazole
 - B. Iv ampicillin, plus iv gentamicin and iv metronidazole**
 - C. A combination of oral antibiotics
 - D. A broad spectrum oral antibiotic
65. The correct rate for ventilating a newborn is:
- A. 20 breaths per minute
 - C. 40 breaths per minute**
 - B. 30 breaths per minute
 - D. 80 breaths per minute
66. To help prevent heat loss, the newborn should be
- A. Dried thoroughly immediately after birth
 - B. Dried thoroughly after the cord has been cut
 - C. Dried thoroughly and covered with a clean cloth immediately after birth**
 - D. Covered with a clean, dry cloth after the cord has been cut
67. Newborn cord care involves
- A. Applying a dry dressing to the cord stump
 - B. Swabbing the cord stump with alcohol and applying a dry dressing
 - C. Keep the cord stump dry without putting any substance on it**
 - D. Covered with antiseptic soaked wet gauze
68. Prostaglandins should not be given
- A. Per vaginum
 - C. By IV**
 - B. By IM injection
 - D. By mouth
69. An antihypertensive drug should be given for hypertension in severe pre-eclampsia or eclampsia if diastolic blood pressure is
- A. Between 100 and 110 mm Hg
 - B. 110 Hg or more**
 - C. 115 mm Hg or more
 - D. 120 mm Hg or more
70. Eclamptic fits may occur in the
- A. Antepartum period only
 - B. Intrapartum period only
 - C. Postpartum period only
 - D. Antepartum, intrapartum or postpartum periods**
71. In a breech presentation, the fetal heart beat
- A. Can usually be heard at a location higher than expected for a vertex presentation**
 - B. Can usually be heard at a location lower than expected for vertex presentation
 - C. Can usually be heard in the same location as for a vertex presentation
 - D. Is not able to be heard

72. External rotation of the fetal head indicates that the shoulders are:
- A. In the anterior diameter of the pelvic outlet
 - B. In the posterior diameter of the pelvic outlet
 - C. In the antero-posterior diameter of the pelvic outlet
 - D. Stuck behind the symphysis pubis
73. Cervical dilation plotted to the right of the alert line on the partograph indicates
- A. Satisfactory progress of labor
 - B. Unsatisfactory progress of labor
 - C. The end of the latent phase
 - D. The end of the active phase
74. Before applying controlled cord traction during active management of the third stage of labor
- A. Oxytocin is administered intramuscularly and the attendant waits for the uterus to contract
 - B. The woman's bladder is catheterized
 - C. Pressure is applied to the fundus
 - D. All of the above
75. Plotting on the partograph should begin
- A. In the active phase of the first stage of labor
 - B. In the latent phase
 - C. When the cervix reaches full dilatation
 - D. When the woman is admitted to the labor ward
76. A woman who suffers shock as a result of an obstetric emergency may have
- A. A weak, fast pulse
 - B. High blood pressure
 - C. Normal breathing
 - D. A good urine output
77. Infection can be transmitted from clients to health care workers through
- A. Contaminated needles or other sharps instruments that pierce the health worker's skin
 - B. Splashes in the health care worker's eye of contaminated blood or body fluids
 - C. Broken skin that is exposed to contaminated blood and body fluids
 - D. All of the above
78. The single-most practical procedure for preventing the spread of infection is
- A. Wearing gloves
 - B. Wearing a mask
 - C. Hand washing
 - D. Cleaning skin with alcohol before injections

79. Following insertion of an IUCD, you should recommend that the client, even if she has no problems, have it checked after:
- A. Three days
 - B. **Three to six weeks**
 - C. One week
 - D. Three to six months
80. Iucds can be inserted during:-
- A. Any time during the menstrual cycle provided that the client is not pregnant.
 - B. Immediately after delivery
 - C. Immediately after non-complicated abortion.
 - D. **All of the above**
81. Which one of the following is incorrect about counseling?
- A. REDI framework for counseling helps in assessing and addressing the needs of the client
 - B. Clients have the right and responsibility to make decisions and carry them out.
 - C. **The steps of REDI counseling framework have to be followed strictly in sequential order.**
 - D. An important role for family planning (FP) counselor is to assist clients in anticipating and strategizing about how to overcome the barriers that might prevent them from implementing their decisions.
82. For a client who is late for her DMPA injection, the grace period is:
- A. 1 week
 - B. 2 weeks
 - C. 3 weeks
 - D. **4 weeks**
83. The IUCD may not be the best method for women who have:
- A. Gallbladder disease
 - B. No child
 - C. **Acute/sub acute pelvic infections**
 - D. Hypertension
84. All except one of the following are warning signs that you should teach to an IUCD client, which indicate that she may be having a problem with her IUCD and should seek medical attention
- A. Amenorrhea
 - B. Change in menstrual bleeding pattern in the first 3 months of use.
 - C. Sexual partner has abnormal penile discharge
 - D. **String is longer than usual**
 - E. Pain with intercourse

85. The recommended regimen for medical abortion up to 9 completed weeks in Ethiopia is
- A. Mifepristone 600mg orally followed 48 hrs later by misoprostol 400 microgram vaginally.
 - B. Mifepristone 600mg orally followed 48 hrs later by misoprostol 600 microgram vaginally
 - C. Mifepristone 200mg orally followed 48 hrs later by misoprostol 800microgram vaginally
 - D. Mifepristone 200mg orally followed 48 hrs later by misoprostol 600microgram vaginally
86. All are long acting family planning implants, except
- A. IUCD
 - B. Transdermal patches
 - C. Jaddle
 - D. Sino-implants
87. Which of the following is correct about the importance of male involvement in reproductive health?
- A. Men's role in preventing unwanted pregnancy is often limited to encouraging their wife's to use family planning
 - B. Male involvement slow the transmission of sexually transmitted infections
 - C. They make motherhood safer
 - D. All of the above
88. Which of the following is correct about the importance of male involvement in reproductive health?
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89. Which of the following Abortion care is not given at community level?
- A. Distribution of emergency contraceptives
 - B. Referral to post-abortion and safe abortion services
 - C. Training of Community level workers and junior health professionals
 - D. None of the above

90. During maternal and child health care in the community, which is the best intervention to reduce maternal and child mortality and morbidity.
- A. Promotive care
 - B. Curative care
 - C. Preventive care
 - D. A and C
91. The one included in the basic safe abortion care services is:
- A. Blood transfusion
 - B. Removal of retained products greater than 12 week
 - C. Administer uterotonics
 - D. Perform laparotomy
92. Concerning adolescent health, when we do say adolescents are healthy?
- A. When they feel guilty of antisocial behaviors
 - B. When they are able to cope with environmental stressors life
 - C. When they are unable to engage in the developmental tasks of adolescence
 - D. When they experience a sense of illness and worry
93. Early sexual debut is one of adolescent health problems. One is not true about the reason of this problem?
- A. Hormonal change leading to early sexual maturation and early menarche
 - B. Socio-economic problems
 - C. Permissive attitude of society towards premarital sexual relations for girls
 - D. Conflict and forced migration
94. All are correct reasons about the limited contraceptive use among adolescents except one.
- A. Lack of awareness to anticipate the consequences
 - B. Feeling on not being vulnerable
 - C. Lack of life skill life skill to negotiate sexual indulgence
 - D. User friendliness of reproductive health services
95. All are the problems of adolescents but which one exposes them to morbidity and mortality in developing countries?
- A. Malnutrition
 - B. Addictive Behavior
 - C. Unsafe sex
 - D. Gender based violence
96. All are true about the importance of school health services but one is not.
- A. It has an influence of health on the ability to learn
 - B. Most problems arises as a result of attending school
 - C. Address large group at one place
 - D. It focuses on preventive health care

97. Which type of the female genital mutilation could lead to higher risk of obstetric complications?
- A. Clitordectomy
B. Excision
C. Infibulation
D. Unclassified
98. Which of the following is/are not true about the concepts of reproductive health?
- A. It should primarily focus on women in child bearing age
B. It is integral to human development
C. It is the Rights of every Human being to be have access to RH
D. It addresses health from womb to the tomb
99. What is the importance of serial measurement on child growth monitoring except one?
- A. To detect the position of the child on the growth curve
B. To detects the direction of growth and explores hidden problems
C. To makes right decision regarding the interventions required
D. To assess of normal growth of infants and young children
100. Women's policy state different issues about women, among these issues which is not correct?
- A. Women shall have equal to men in the enjoyment of human rights
B. Women have the right to be free from the influence of HTP
C. Women have the right to maternity leave without remuneration
D. Women have equal rights as men with regard to marriage
101. Following are the target diseases of EPI except
- A. Tb
B. Mumps
C. Measles
D. Tetanus
102. Vaccine can be stored at health facility level for,
- A. 1 month
B. 2 months
C. 3 months
D. 6 months
103. Status epilepticus is a condition where the seizure last for ,
- A. 5 minutes
B. 30 minutes
C. 45 minutes
D. 60 minutes
104. Following are the causes of Acute Rheumatic Fever except
- A. Previous infection with group B streptococcus
B. Substandard health practices
C. Overcrowded living condition
D. Poor socioeconomic status and living condition

105. Which of the following would the nurse need to keep in mind as a predisposing factor when formulating a teaching plan for child with UTI?
- A. A shorter urethra in females
 - B. Frequent emptying of the bladder
 - C. Increased fluid intake
 - D. Ingestion of acidic juice
106. Which of the following nursing diagnosis would be inappropriate for the infant with gastro esophageal reflux disorder
- A. Fluid volume deficit
 - B. Risk for aspiration
 - C. Altered nutrition less than body requirement
 - D. Altered oral mucous membrane
107. Following are the medical complications of the child who is referred for in-patient therapeutic management of SAM except
- A. Bilateral pitting edema
 - B. Severe vomiting
 - C. Extensive skin lesion
 - D. Fever 38 o c
108. Recommended duration of out-patient antibiotic therapy for child with SAM,
- A. 7 days
 - B. 10 days
 - C. 14 days
 - D. Until therapy finishes
109. The definitive diagnosis for dehydration in marasmic patient is from,
- A. A skin pinch
 - B. Sunken eyes
 - C. History
 - D. Mental status

110. For treatment SAM in infant less than 6 months the nurse must prepare diluted F100 by following technique,
- A. Use 100 ml F100 already prepared and add 35 ml of water
 - B. Use 200 ml F100 already prepared and add 35 ml of water
 - C. Use 100 ml F100 already prepared and add 70 ml of water
 - D. Use 300 ml F100 already prepared and add 35 ml of water
111. Which of the following functions or activities requires recruiting and placing qualified personnel needed for the organization so that it may achieve its objectives and goals?
- A. Planning
 - B. Organizing
 - C. Controlling
 - D. Staffing
 - E. Leading
112. All are the primary responsibility of the first level manager except
- A. Directing the day-to-day activity of the organization
 - B. Establishing organizational goals and strategic plan
 - C. Supervising the work of non-managerial personnel
 - D. None of the above
113. Which of the following is a potential environmental obstacle to delegation?
- A. Standards
 - B. Insecurity
 - C. Inexperience
 - D. All of the above
114. Which of the following characteristics is NOT related to Laissez-faire leadership?
- A. Very little guidance from leaders
 - B. Complete freedom for followers to make decisions
 - C. Leaders provide the tools and resources needed
 - D. There is little time for group decision-making
 - E. Group members are expected to solve problems on their own
115. The Midwife is studying the difference between a manager and a leader. Which of the following is the major difference between managers and leaders?
- A. Leaders do not make good managers
 - B. Managers seldom are leaders
 - C. Managers always have legitimate power
 - D. Leaders always have legitimate power

116. He recalls that the management process involves five functions which follow the following sequence.
- A. Planning, staffing, organizing, controlling, directing
 - B. Planning, organizing, directing, staffing, controlling
 - C. Organizing, planning, directing, staffing, controlling
 - D. Planning, organizing, staffing, directing, controlling
117. Midwife Edris is aware that there are three levels of skills, which he should possess as manager. These are:
- A. Conceptual skills, literary skills, technical skills
 - B. Political skills, conceptual skills, human skills
 - C. Interpersonal skills, technical skills, human skills
 - D. Conceptual skills, interpersonal skills, technical skills
118. A type of interpersonal power that is based on the possession of unique skills, knowledge and competency of the manager is
- A. Reward power
 - B. Referent power
 - C. expert power
 - D. Coercive power
119. Which management function requires managers to make decisions on the four fundamental elements of objectives, actions, resources, and implementation?
- A. Controlling
 - B. Planning
 - C. Organizing
 - D. Leading
120. Organizing involves the following:
- A. Motivating employees
 - B. Defining goals
 - C. Monitoring employees' activities
 - D. Grouping tasks into departments
 - E. Determining appropriate tasks and resources



College of Health Science

Department of Midwifery

COMPREHNSIVE EXAM FOR MIDWIFE STUDENT

DIRECTION

- Total number of questions= 134
PART I- Obstetrics= 1-72
PART II- Gynecology , FP and RH= 73-104
PART III- Neonatology and pediatrics= 105-120
PART IV- Public health= 121-134
- Total number of pages-
- All questions are multiple choice
- Select the best answer and write on the answer sheet

Time allowed

PART I- OBSTETRICS

1. Which one of the following is **false** about Gynecoid type of female pelvis?
 - A. It is the normal female type
 - B. Inlet is slightly transverse oval
 - C. Sacrum is wide with average convexity and inclination
 - D. Side walls are straight with blunt ischial spines
2. Which one of the following is **true** about posterior fontanel?
 - A. Formed from the sagittal, coronal and frontal sutures are meet and diamond in shape
 - B. Formed at the junction of the lambdoidal and sagittal sutures and triangular in shape
 - C. Formed from the sagittal, coronal and frontal sutures are meet and triangular in shape
 - D. Formed at the junction of the lambdoidal and sagittal sutures and diamond in shape
3. Which of the following statement(s) is/are correctly describes menstruation?
 - A. In a normal menstrual cycle you expect menstruation to last approximately 21-35 days
 - B. During menstruation only the functional layer of the endometrium is shed, with the basal layer remaining intact
 - C. During menstruation only the basal layer of the endometrium is shed, with the functional layer remaining intact
 - D. During menstruation the entire endometrium is shed
4. Physiologic anemia is one of change occurring during pregnancy. The cause is__?
 - A. Due to reduction of RBC production
 - B. Unbalanced increment of RBC production and total blood volume
 - C. Increment of total blood volume keeping RBC production constant
 - D. None
5. The accommoA 23-year-old woman gravida 2, para 2 calls her midwife 7 days postpartum because she is concerned that she is still bleeding from the vagina. It would be appropriate to tell this woman that it is normal for bloody lochia to last up to:
 - A. 5 days
 - B. 8 days
 - C. 11 days
 - D. 14 day
6. A midwifery instructor is conducting a lecture and is reviewing the functions of the female reproductive system. He asks student X to describe the follicle-stimulating hormone (FSH) and the luteinizing hormone (LH), The student accurately responds by stating that:

- A. FSH and LH stimulate the formation of milk during pregnancy
 - B. FSH and LH are secreted by the corpus luteum of the ovary
 - C. FSH and LH are secreted by the adrenal glands
 - D. FSH and LH are released from the anterior pituitary gland.
7. A woman is 22 years old and has missed two of her regular menstrual periods. Her midwife confirms early intrauterine pregnancy and this is her first pregnancy. She is married and works as a salesperson in local department store. To determine woman's due date, which of the following statements more important?
- A. Date of first menstrual period
 - B. Date of last intercourse
 - C. Date of last normal menstrual period
 - D. Age at menarche
8. A woman missed her menstrual period 1 week ago and has come to your clinic for a pregnancy test. Which placental hormone is measured in pregnancy tests
- A. Progesterone
 - B. Estrogen
 - C. Human placental lactogen
 - D. Human chorionic gonadotropin
9. A 16 weeks age of pregnant woman come to you clinic for antenatal check up. In history you assured that the client had two previous pregnancies. The first pregnancy was terminated at 21 wks of gestation, and the second was born dead at 38 weeks of gestation. From this history which option describes the respective Gravidity, parity, abortion and stillbirth of the mother?
- A. 2, 0, 1, 0
 - B. 3, 1, 1, 1
 - C. 2, 0, 1, 1
 - D. 3, 2, 1, 1
10. Focused antenatal care (FANC) is a new model of antenatal care which replaced the traditional type. Which statement best describes this model of antenatal care?
- A. It has maximum of four visit
 - B. It works for normal and abnormal pregnancy
 - C. It emphasis on risk classification
 - D. It is individualized and client centered
11. When a grand multiparas mother comes to your health institution and you take history and physical examination, then your assessment is active 1st stage of labor. While you are

following this laboring mother with a partograph which component of a partograph do you interpret in relation to alert and action lines?

- A. Descent
- B. FHB
- C. Cervical dilation
- D. Molding

12. Which is false about partograph?

- A. It is diagrammatic presentation of fetal and maternal condition and progress of labor
- B. Plotting will not be started at latent first stage of labor
- C. Following with partograph is not recommended in hospitals
- D. It helps for early diagnosis and referral of prolonged or obstructed labor

13. A 20 yrs old gravida II, para I mother on her 39+2wks of GA presented with complaint of pushing down pain 8 hrs duration. On P/E the FHB =142 BPM, cervix 8 cm dilated, with vertex presentation, frequent uterine contraction, then the woman is on__

- A. Active first stage of labour
- B. Latent first stage of labour
- C. Second stage of labour
- D. Third stage of labor

14. While you are managing third stage of labor actively which one is **not** included as component ?
- A. CCT
 - B. Uterine massage
 - C. Fundal pressure
 - D. Give uterotonic drug
15. While you are examining placenta after delivery the correct combination of umbilical vessels is_____
- A. Two arteries and two veins
 - B. Two arteries and one vein
 - C. One artery and two veins
 - D. One artery and one vein
16. A 22-year-woman, gravida 1, para 0, at 40 weeks of gestation, presents to labor and delivery reporting regular contractions for the last 2 hours. She denies loss of fluid from the vagina and reports good fetal movement. Her cervix is dilated 2 cm and 50% effaced, and fetal vertex is at 0 station. There is a regular uterine contraction every 2 to 3 minutes and fetal heart rate is 154 bpm without decelerations. What is the next step in management?
- A. Cesarean section
 - B. Oxytocin administration
 - C. Fundal massage
 - D. Walk for 1 to 2 hours then return to check her cervix
 - E. Admit and follow with partograph
17. A mother comes to your hospital with pushing down pain; you assess her and put your diagnosis in relation with fetal condition, maternal condition, and progress of labour. Fetal condition highly determined using?
- A. Color of amniotic fluid
 - B. Descent
 - C. Cervical dilation
 - D. Uterine contraction
18. Monitoring contractions is very important during labor. To monitor uterine contractions, what should the midwife do?
- A. Observe for the client's facial expression to know that the contraction has started or stopped.
 - B. Instruct the client take note of the duration of her contractions.
 - C. Offer ice chips to the woman.
 - D. Spread the fingers lightly over the fundus to monitor the contraction

19. A 22 years old unmarried lady came to your clinic with the complaint of loss of appetite, nausea and vomiting for a week. She has history of irregular menses but amenhoric for the last 8 wks. Her BP was 90/50mmHg and PR of 102bpm. Then you intend to rule in hyper emesis gravidarum. What will help you to rule in hyper emesis gravidarum?
- A. Urine HCG positive
 - B. Ova or parasite seen
 - C. Urine ketone negative
 - D. History of watery diarrhea
20. Based on the above case scenario after you rule in sever hyperemesis gravidarum, what will you do?
- A. Give PO anti emetics and send her home
 - B. Keep nothing per os(PO)
 - C. Give vitamin B-complex tablet
 - D. Rehydrate with soft drinks
21. You are at a primary hospital and someone from nearby health center called you for consultation. He told you that a pregnant woman with gestational age of 30wks visited the health center for the complaint of three episodes tonic clonic types of body movement at home. At arrival she was confused and not oriented to time, person and place Her BP was 160/105mmHg. The diploma midwife from the health center was writing the referral paper to your hospital when he called you. What will you recommend to do upon referral?
- A. Secure an IV line and resuscitate aggressively
 - B. Position the mother on supine position
 - C. Suck secretions from mouth
 - D. Don't give any antihypertensive or anticonvulsant
22. The mother in the above scenario arrived to your hospital with BP of 150/100, PR of 100 and RR of 24. She was conscious and oriented to TPP and quiescent uterus. Which of the following could be your plan management?
- A. Assess biophysical profile twice per week
 - B. Start induction of labor with oxytocin
 - C. Discontinue anticonvulsant
 - D. Appoint after a week and send her home with methyldopa
23. From the above case description for which complication do you prescribe prophylactic medication for this particular mother?
- A. Acute renal failure
 - B. Disseminated intravascular coagulation(DIC)
 - C. Aspiration pneumonia
 - D. Hemi paralysis
24. For management of preeclampsia which will **not** go into consideration.
- A. Gestational age
 - B. Parity
 - C. Severity of the disease
 - D. Fetal maturity

25. Correct about maintenance dose of magnesium sulfate?
- A. Given every 6 hours
 - B. Given at each buttock alternately
 - C. Continued for 24 hours postpartum
 - D. Give irrespective of vital signs
26. Of the following which one **cannot** be differential diagnosis of vaginal bleeding in late pregnancy?
- A. Abortion
 - B. Vasa previa
 - C. Cervical polyp
 - D. Uterine rupture
27. On your day duty, a woman visited obstetrics ward with vaginal bleeding for two hours duration. At arrival she was in labor (2 moderate uterine contractions per ten minutes) and the gestational age was 39 weeks. What could be the next management after you put her on intravenous fluid?
- A. Digital vaginal examination
 - B. Cesarean section
 - C. Obstetrics ultrasound
 - D. Augmentation
28. A gravida 2 mother by her 28 weeks of gestation came to MCH clinic for routine ANC follow up and upon scanning the placenta was posterior marginalis. You appointed her to be scanned at term and the result was the same. So what will you do?
- A. Elective cesarean section
 - B. Induction of labor
 - C. Wait for spontaneous labor
 - D. Emergency cesarean section
29. Which one of the following is the possible cause of polyhydramnios?
- A. Renal agenesis
 - B. Esophageal atresia
 - C. Premature rupture of membrane
 - D. Prolonged pregnancy
30. Which could be intrapartum complication of polyhydramnios?
- A. Unstable lie and malpresentation
 - B. Premature labor
 - C. Cord presentation
 - D. Cord prolapsed
31. If you faced a mother with premature rupture of membrane in your clinic by the gestational age of 32 weeks and no sign of labor, how will you manage?
- A. Try to shorten latency period
 - B. Assess cervical dilatation
 - C. Start corticosteroids
 - D. Encourage ambulation
32. Production of natural surfactant is vital event for lung maturity. Which of the following is not correct about surfactant?
- A. Help to reduce surface tension in the lung
 - B. Its production increase with gestational age
 - C. Corticosteroid drugs are important to enhance its production
 - D. It usually causes birth asphyxia

33. Which one is **not** important in determining biophysical profile?
- A. Fetal movement
 - B. Amniotic fluid volume
 - C. Fetal breathing movement
 - D. None
34. Which one is correct about asymmetric intrauterine growth restriction?
- A. HC/AC ratio elevated
 - B. Early insult
 - C. Rare than symmetric
 - D. Both cell size and number are restricted
35. If you diagnosed intrauterine fetal death and the mother didn't agree to have induction of labor, for how long the dead fetus can be kept in utero for spontaneous onset of labor to the maximum?
- A. One week
 - B. Two weeks
 - C. Three weeks
 - D. Four weeks
36. For the above case (IUFD), Which of the following problem is a feared complication?
- A. Obstructed labor
 - B. Oligohydramnios
 - C. Disseminated intravascular coagulation
 - D. Polyhydramnios
37. Which of the following is **not** complication of post term pregnancy?
- A. Macrosomia
 - B. Fetal Asphyxia
 - C. Meconium Aspiration
 - D. Prematurity Syndrome
38. In principles of multiple gestation which **cannot** occur?
- A. Dichorionic diamniotic
 - B. Monochorionic diamniotic
 - C. Dichorionic monoamniotic
 - D. Monochorionic monoamniotic
39. Which one of the following is allowed to vaginal delivery in case of multiple gestations?
- A. Twin A breech twin B cephalic
 - B. Twin A cephalic twin B breech
 - C. Both breech
 - D. Twin A Breech Twin B unknown presentation
40. For occurrence of Rh isoimmunization, which one of the following is the prerequisite?
- A. Rh positive mother carrying Rh negative fetus
 - B. Entry of fetal compatible blood cells into maternal circulation??
 - C. Development of Rh antibodies for rh by the mother
 - D. Irrespective with paternal blood type
41. A multigravid mother at ANC clinic was having pale conjunctiva, light headedness, she was barefooted and from non malaria endemic rural area. Her Hgb was 8.1gm/dl, blood exam negative. How do you manage this patient?
- A. Give iron sulfate with folic acid 325mg po
 - B. Transfuse at least one unit cross matched blood
 - C. No need of antihelmentic drug
 - D. Appoint with regular visit.
42. Prevention of mother to child transmission by administering (3TC+ EFV+NVP) is:
- A. Prong-1
 - B. Prong 2
 - C. Prong 3
 - D. Prong 4

43. You are following labor in labor ward and you get 2cont/10mins lasting 25-35secs, cervix persists 4cm for the last four hours and the position of presenting part was ROA, she gave birth 3 years back vaginally. What could be your plan of management?
- A. Prepare for c/s
 - B. Augment
 - C. Follow for 4 more hours
 - D. Instruct the mother to ambulate
44. A gynecologist diagnoses a pregnant woman & he decides the pregnancy should be terminated with induction. Which one of the following can be his possible indication of induction?
- A. Placenta previa
 - B. Previous c/s scar
 - C. Abruptio placenta
 - D. Twin pregnancy
45. What will be the outcome of properly unmanaged obstructed labor
- A. End up with rupture of uterus
 - B. Fetal death
 - C. Maternal sepsis
 - D. All
46. Which one the following is **wrong** combination of causes of prolonged labor with its example?
- A. Maternal factor - contracted pelvis
 - B. Fetal factor – macrosomia
 - C. Power related- hypertonic uterus
 - D. None
47. As a professional midwife what will you do in second stage of labor
- A. Assist the labor accordingly
 - B. Follow labor with partograph
 - C. Administer 10 IU oxytocin IM
 - D. Monitor fetal heart beat every 30 mins
48. In case of face presentation which one of the following positions **does not** allowed for vaginal birth?
- A. Direct mento anterior
 - B. Right mento transverse
 - C. Left mento transverse
 - D. Direct mento posterior
49. Hip flexed and knee extended describes:
- A. Frank breech presentation
 - B. Complete breech presentation
 - C. Footling breech presentation
 - D. Knee breech presentation
50. As a Midwife you are delivering a fetus with breech presentation. When the arm fails to deliver spontaneously you tried to deliver with certain maneuver. Which maneuver will help you to deliver the baby's arm?
- A. Lovset's maneuver
 - B. MauriceauSmellieVeit maneuver
 - C. Pinard maneuver
 - D. Modified Prague maneuver
51. When you examine the mother for Bishop Score the membrane ruptured and you felt anterior cord prolapse. Then you consulted the senior clinical midwife and he told you to prepare for emergency c/s. Which position you prefer for this mother?
- A. Left lateral position
 - B. Knee chest position
 - C. Supine position & buttock up
 - D. In the position she preferred
52. Of the following which **can not** be manifestation of ruptured uterus?

- A. Tetanic uterine contraction
B. Easily palpable fetal part
B. Absent fetal heart beat
D. Tender abdomen
53. Which is **correct** about secondary postpartum hemorrhage?
- A. Mostly caused by uterine atony
B. Keeping the mother for at least 6 hours postpartum to prevent it
C. Give 10 IU oxytocin IM to prevent it
D. It is the rare and associated with sepsis
54. For a mother referred to you with a diagnosis of retained placenta, all activities are recommended immediately at admission. But which activity is recommended later?
- A. Catheterization of the mother
B. Securing IV line and resuscitate her with fluids
C. Administer uterotonic drug
D. Manual removal of placenta
55. While you are working in MCH clinic, a 34 weeks pregnant mother came to you with complaints of frontal headache, and epigastric pain. You measured all her vital signs and she was having elevated blood pressure. What will you do next?
- A. Give potent anti pain and send her home with appointment card
B. Workup for urinalysis and organ function test
C. Secure line and put her on maintenance fluid
D. Give antacids
56. Which one of the following is **not correct** about management of premature rupture of membrane?
- A. Give antibiotics
B. Administer corticosteroids
C. Encourage ambulation
D. Counsel the mother to record kick count
57. Three delays are killing mothers in developing countries like Ethiopia. If you heard a scenario as a mother died after she arrived to the primary hospital due to no light for emergency c/s. which category of delay is it?
- A. First
B. Second
C. Third
58. Which statement is **wrong** about episiotomy?
- A. Appropriate time to make incision is when the head is crowning.
B. Routine administration of prophylactic antibiotics before incision is not required.
C. Lidoicaine is infiltrated to one of single layer of vaginal layer before incision
D. Suturing of episiotomy involves the mucus membrane, muscle and skin.
59. Which one of the following is the advantage of mediolateral episiotomy over median type?
- A. Less blood loss
B. Ease for incision and repair
C. Less extension of the incision
D. Less dyspareunia
60. When you are following a labor in health center, the labor was precipitated and the mother gave birth to macrosomic baby. In perianal examination you have seen a tear which extend

the whole layer of vaginal wall and internal and external anal sphincters. Which of the following is **correct** statement based on the case?

- A. It is fourth degree tear, and need to refer for specialized management in hospital
- B. It is third degree tear, and need to refer for specialized management in hospital
- C. It is second degree tear and can be repaired at your health facility
- D. It is second degree tear and need to refer for specialized management

61. Which of the following statement is **correct** about instrumental delivery?

- A. In assisting delivery of preterm fetus, vacuum delivery is more preferred than forceps
- B. In case of failed vacuum application, forceps application is recommended procedure
- C. Forceps delivery usually not helps to achieve rotation and traction to the fetus
- D. Instrumental delivery is not recommended in unengaged presenting part

62. The correct application of vacuum extraction is achieved when the centre of the cup is superimposed on the flexion point. The correct application site is__

- A. 3 cm in front of the posterior fontanelle at the sagittal suture.
- B. 3 cm posterior of the posterior fontanelle at sagittal suture
- C. 3 cm in front of anterior fontanel at sagittal suture
- D. 3 cm posterior to anterior fontanel at sagittal suture

63. When applying vacuum delivery, in which condition it is said failed?

- A. The head does not advance with each pull
- B. The fetus is not delivered with a single pull
- C. The fetus is not delivered within 10 minutes
- D. With correct application, the vacuum slips off the head once

64. A 25 year old multigravida woman is diagnosed for prolonged 2nd stage of labor. Her physical examination findings were; contraction 2/ 10min lasting 45-50 seconds, FHB 100 BPM, Cx = 10 cm, presentation= vertex with excessive caput, position= ROA, and station +2 and poor maternal effort, which type of forceps is appropriate to assist this mother for delivery?

- | | |
|-------------------|-----------------|
| A. Low forceps | C. Mid forceps |
| B. Outlet forceps | D. High forceps |

65. In which of the following condition forceps delivery is contraindicated?

- A. Prolonged second stage
- B. To shorten second stage in cases of maternal distress
- C. Diagnosed cephalopelvic disproportion
- D. After-coming head in breech presentation

66. A 34 years old gravid 2 mother with history of classical type of CS come to your ANC clinic at GA of 35 weeks of gestation. You have identified that the presentation is breech, FHB is positive. Then best advice about plan of delivery will be ____
- A. You are candidate for vaginal birth after cesarean delivery (VBAC) and you can attempt the delivery here
 - B. You are candidate for VBAC and you may attempt the spontaneous delivery at area where CS service is available
 - C. You are candidate for VBAC and you may attempt with induction of labor at area where CS service is available
 - D. You are not candidate for VBAC and you need to follow at area where CS service is available
67. Which one of the following practice is not recommended in preoperative preparation of women for cesarean section?
- A. Administration of prophylactic antibiotics
 - B. Shaving of surgical site
 - C. Administration of antiemetic
 - D. Catheterization of the women
68. Which one of the following is **wrong** statement about post operative care after uncomplicated CS delivery?
- A. Triple antibiotics is recommended for 5 to 7 days post operatively
 - B. Breast feeding is started immediately after the mother awake from anesthesia
 - C. Sips are commonly recommended after appreciation of bowel sound
 - D. Woman with lower uterine segment transverse CS stay shorter than classical type
69. Destructive deliveries are procedures performed on dead fetus to reduce the bulky size so as to facilitate vaginal delivery. In which condition the procedure is safe to perform?
- A. When cervix is partially dilated
 - B. When the descent is 2/5 or lower
 - C. When the membrane is intact
 - D. In women with rupture of uterus
70. If a practitioner assessed sever hydrocephalous with absence of majority of brain tissue, and you decided to perform destructive delivery after discussing with the mother. The best fitting procedure for this case is__
- A. Craniotomy
 - B. Decaptation
 - C. Evisceration
 - D. Cephalocentesis
71. Which of the following statement is **wrong** statement about amniotomy?
- A. It is a procedure performed to induce or augment labor
 - B. Unengaged presenting part is relative contraindication for the procedure
 - C. Controlled amniotomy is recommended in women's with polyhydramnious
 - D. Cord prolapse is unlikely following the procedure

72. Post partum hemorrhage (PPH) is one of identified causes of maternal morbidity and mortality in our country. The majority of PPH is attributed to which cause?
- A. Infection
 - B. Retained placenta and other conceptus tissue
 - C. Atonia of uterus after delivery
 - D. Trauma of genital tract

PART II. GYNECOLOGY, FAMILY PLANNING AND REPRODUCTIVE HEALTH

73. A 22 year's old girl comes with complaint of irregular menstrual cycle. Which phase of menstrual cycle is responsible for irregularity of her menses?
- A. Proliferative phase
 - B. Follicular Phase
 - C. Secretory phase
 - D. Luteal phase
74. A 24 years old woman comes with complaint of vaginal bleeding in early pregnancy, physical examination revealed that fundal height is equivalent to calculated GA, with PV examination the cervix is open. What is the most likely diagnosis?
- A. Incomplete abortion
 - B. Complete abortion
 - C. Inevitable abortion
 - D. Threatened abortion
75. Which statement is **wrong** during utilization of MVA cannula?
- A. Gestational age has no importance to prefer cannula size
 - B. If you are using cannula with single aperture rotate your hand 360 degree
 - C. If you are using cannula with double aperture , rotate your hand 360 degree
 - D. If you are using cannula with single aperture rotate your hand 180 degree
76. A 26 years old woman diagnosed with incomplete abortion and based on her LMP GA is 9 weeks which one is preferable method to evacuate
- A. Medical abortion
 - B. MVA
 - C. D&E
 - D. EVA
77. A primigravida woman with gestational age of eight weeks come with a complaint of lower abdominal pain with vaginal bleeding. In physical examination cervix is closed. In laboratory investigation progesterone level is 3IU/L and HCG level is 8000IU. Trans abdominal ultrasound not able to visualize Gestational sac. What is the most likely diagnosis?
- A. Incomplete Abortion
 - B. Complete abortion
 - C. Molar pregnancy
 - D. Ectopic Pregnancy

78. A 23 year's old gravid I para 0 woman come with a complaint of vaginal bleeding, her gestational age is 11 weeks. Ultrasound revealed snowball vesicles and fundal height is more than calculated gestational age. what will be the most likely DX
- A. Ovarian torsion
 - B. Ectopic pregnancy
 - C. GTD
 - D. Inevitable abortion
79. Which one of the following could differentiate ruptured ectopic pregnancy from un ruptured
- A. Culdocentesis
 - B. Amenorrhea
 - C. Vx bleeding in early pregnancy
 - D. Nausea & vomiting
80. Nineteen years old women come with complain of excessive vaginal discharge with fever. In physical examination there is lower abdominal tenderness, cervical and adnexal motion tenderness. In laboratory HCG negative. In ultrasound there is accumulation of fluid in posterior fornix of the Uterus. What is the most likely diagnosis?
- A. Ovarian cyst
 - B. Ovarian torsion
 - C. PID
 - D. peritonitis
81. A 53 years old Gravida VII, Para V women with complaint of pelvic pressure and feeling of sitting on the ball. No history of prolonged labour and heavy lifting .what is the most likely risk factors?
- A. Premature bearing down
 - B. Fundal pressure
 - C. Multiparity
 - D. Pelvic tumors
82. A 50 years old woman with complaint of unable to conceive for 3 years and Pelvic pressure. In examination myoma reaching the size of more than 12 weeks sized gravid uterus is diagnosed. What is the most likely management for this woman?
- A. Myomectomy
 - B. Chemotherapy
 - C. Radiation
 - D. NSAID
83. A 46 years- old women come with complain of vaginal bleeding during her douching and sexual intercourse. She had a history of multiple sexual partner and history of smoker five years ago. Which of the following is most likely causative agent?
- A. HPV 9,11
 - B. HPV 16,18
 - C. HPV 31,40
 - D. HPV 44,52
84. A woman is come to your clinic with a complain of vaginal bleeding which is irregular, and pain during intercourse. HCG was negative, and you feel offensive ordure when she undress

her underwear. In vaginal examination you felt irregular cervix with different texture. Then you decided to refer this woman to higher facility with high degree suspicion of ____

- A. Myoma
- B. STIs
- C. Cervical cancer
- D. Cervical incompetence

85. A mother “X” is 39 years old gravida V para IV woman, has no any history of oral contraceptive utilization and no history of smoking. A mother “Y” is 35 years old gravida II para II woman, has history of oral contraceptive utilization for 2 consecutive years with history of smoking 5 years ago. A mother “Y” has Diagnosed with endometrial cancer so what will be the most likely risk factor?

- A. Age
- B. Oral contraceptive
- C. Smoking
- D. Lowerparity

86. Which method of family planning that is **not** recommended for the couples who have no children is:

- A. IUCD
- B. COC
- C. Voluntary sterilization
- D. Implanon

87. A 19 years old girl come with objection of unsafe sex with her boyfriend 2 days ago and needs post pills. in store there is only low dose combined oral contraceptive, so how could manage her with available drug?

- A. Take 2pills as soon as possible and 2 pills after 12 hours
- B. Take 4 pills as soon as possible and 4 pills after 12 hours
- C. Take 20 pills as soon as possible and 20 pills after 12 hours
- D. Take 1pill as soon as possible and 1 pill after 12 hours

88. A 25 years -old women come in to your health center for complain of missing three pills she took the missed one pill as soon as she remembered and continued her daily pill taking as usual and when you count the remaining active pills, eight pills are left. Which one of the following is the most likely intervention for such women?

- A. Abstain for next 7 days, finish active pills, avoid inactive pills &start new pack immediately.
- B. No need of Abstain for 7 days finish active pills, avoid inactive pills and start new pack immediately.
- C. No need of Abstain for 7 days, finish both active and inactive pills
- D. Abstain for next 7 days, finish both active and inactive pills

89. Which one of the following is **correct** about Implanon

- A. Could insert Implanon for a women switching from Depoprovera after a week of reinjection time.
 - B. Could use as an emergency contraceptive method when there is no emergency contraceptive pills
 - C. Could use for lactating postpartum mother less than six week
 - D. If she switch from non-hormonal contraceptive and ≥ 7 days of LNMP no need of back up method
90. A 32 years old woman with history of breast cancer but not current cancer, seek jadlle family planning. To decide on administration of this, first you need to categorize based on WHO medical eligibility criteria. So in which category you put it?
- A. Category I
 - B. Category 2
 - C. Category 3
 - D. Category 4
91. Which one of the following is correct complication management of implants
- A. Supplement of COC pills in a women with heavy prolonged bleeding
 - B. Immediately remove implants if she complain darkness of incision area
 - C. Remove all roads if she complain spontaneous expulsion of one road
 - D. Remove the implants if she complain acne
92. Who is eligible for IUCD insertion from those three?
- A. After septic abortion
 - B. Postpartum 48hrs-4wks
 - C. Postpartum 4wks or longer
 - D. Puerperal sepsis
93. Which type of contraceptive is preferable for hypertensive mothers with BP of 160/100
- A. Depoprovera
 - B. Oral contraceptives
 - C. Implants
 - D. Copper bearing IUD
94. A woman need to space her child with calendar method but she complain irregularity of her menses with a range of 27-32 days and you need to suggest her on **Rhythmic method** of family planning and she tells you dates of her menstrual cycle for six month was 27,29,30,28,32 and 27 .so tell her unsafe period

- A. 16-24
 - B. 8-19
 - C. 25-27
 - D. 9-21
95. What will be your advice for a women who got pregnant despite presence of IUCD and come in second trimester to you for advice?
- A. Tell her it should be removed now
 - B. Tell her to leave it there and continue pregnancy
 - C. Encourage her for safe abortion
 - D. None
96. A woman with pelvic infection after a month of IUD insertion. What is immediate intervention for her?
- A. Remove IUD
 - C. Administer antibiotics
 - B. Leave IUD there
 - D. Need ultrasound evaluation
97. Assume you are assigned in a family planning room and you are the assigned midwife to give a reproductive health care in different aspects. Your target population of a service includes for whom the service is primarily or solely intended?
- A. All women of child-bearing age
 - C. All Children
 - B. Both sexes of adolescent group
 - D. All adults
98. All are components of Reproductive Health **except**
- A. Family planning services
 - C. Prevention & treatment of infertility
 - B. Promoting safe motherhood
 - D. Treatment of ovarian tumor
99. Which one of the following is an **indirect** cause of maternal Mortality?
- A. PPH
 - C. Unsafe abortion
 - B. preeclampsia
 - D. Anemia
100. Unlawful kidnapping or carrying away of girls for marriage is____
- A. Rape
 - C. Early marriage
 - B. Abduction
 - D. FGM
101. A pregnant woman comes to your clinic seeking safe abortion. For this service which activitiy is not legal for you?
- A. Comparing and checking whether the reason for abortion is permitted by abortion law
 - B. Confirming the reason she gave for abortion is correct and real
 - C. Checking gestational age is permitted for ones professional level
 - D. Checking gestational age is permitted for the facility we are working

102. Which is **false** statement about syndromic management of STIs.
- A. Classification of the main causal pathogens by the clinical syndromes they produce
 - B. Use of flow charts to manage a particular syndrome
 - C. Education and counseling of the patient on how to prevent re-infection
 - D. Notification and treatment of sex partners
 - E. Expensive laboratory procedure required
103. When you take history and perform physical examination for the sake of applying syndromic management in your health institution you find that vulvar itching and vaginal discharge. Then for this case the most likely syndromic diagnosis is____
- A. Vaginal discharge syndrome
 - B. Genital ulcer syndrome
 - C. Lower abdominal pain syndrome
 - D. Urethral discharge syndrome
104. Basic essential cares are obstetric procedures which are essential and practiced at health center level. Which one is not component of basic obstetric care service ?
- A. Administration of parenteral antibiotics
 - B. Administration of antihypertensive drug
 - C. Transfusion of blood
 - D. Removal of retained products of conceptus

III– NEONATOLOGY AND PEDIATRICS

105. A child presents with nausea, vomiting, tenderness and pain in right lower quadrant of the abdomen and the diagnosis is appendicitis. Which one is true about appendicitis?
- A. psoas sign is right quadrant pain on medial rotation of the right thigh
 - B. Obturator sign is there is right quadrant pain on flexion of the right thigh
 - C. Rovsing's sign is pain in left lower quadrant when right lower quadrant palpated
 - D. Rebound tenderness is production of pain when pressure is released
106. A Midwife is working in delivery room. Immediately after birth, she found newborn's heart beat is 80 beats /minute, crying, actively moving, sneezing and completely pink in color. What nursing care is required for the newborn?
- A. Refer immediately
 - B. Start resuscitation
 - C. Call for help
 - D. Continue routine care as normal
107. What is the chance of transmission of HIV virus from mother to child without any prevention of mother to child transmission (PMTCT) intervention?
- A. About half of them
 - B. About one third
 - C. About one fourth
 - D. About one fifth
108. A new born delivered with absence of scalp, skull and the upper part of the brain. What will be this congenital problem?
- A. Hydrocephalus
 - B. Macrocephaly
 - C. Microcephaly
 - D. Anencephaly
109. For the above case, what will be your advise in planning for future pregnancy
- A. Pre pregnancy and early pregnancy folic acid supplementation
 - B. Avoidance of pregnancy in the future
 - C. Better to start folic acid supplementation after confirming pregnancy
 - D. Tell her the case is un prevented and no need of any treatment
110. Which one of the following is component in correct positioning of breast feeding?
- A. Mouth opens wide
 - B. Mother supports whole body's of infant
 - C. Lower lip is turned outside
 - D. Chin touching the breast

111. A 2 month old child comes to a clinic with a breathing problem and a health worker counts his breathing. A 2 month old infant is said to have fast breathing if:
- A. Breathing is 40 breath per minute or more
 - B. Breathing is 50 breath per minute or more
 - C. Breathing is 60 breath per minute or more
 - D. Breathing is 70 breath per minute or more
112. Which statement is true about newborn resuscitation?
- A. Neck should be in a neutral to slightly extended position
 - B. Nose will be suctioned first
 - C. The first two minute is called golden minute
 - D. Size zero face mask is used for term newborn
113. A child was on efavirenz, lamivudine and nevirapine ART regimen and rifampicin, isonized, pyrazinamide and ethambutol anti TB regimen. Which of the following first line anti TB drug forms drug interaction with ART drug?
- A. Isoniazid (H)
 - B. Pyrazinamide (Z)
 - C. Rifampicin(R)
 - D. Ethambutol (E)
114. A 5- year old child on ART medication for the last one week came today to you because he had CNS problems and night mares. Which drug is the cause of the above problems?
- A. Tenofovir
 - B. Ziwdovudin
 - C. Efavorinze
 - D. Neverapine
115. A four year old child comes with a chief complain of fever. When the health worker assesses the child, he gets generalized skin rash, cough and red eyes. What will be the classification of this disease?
- A. Measle
 - B. No measle
 - C. Measle with mouth complication
 - D. Severe complicated measle
116. A “10 years-old” child is admitted to hospital. He is diagnosed with heart failure. The physician orders IV digoxin. What things most importantly be monitored after digoxin is given?
- A. Weight
 - B. Level of consciousness
 - C. Vital signs
 - D. Input and output

117. A mother gave birth at 34-weeks of gestation. An hour after birth, you found the newborn's respiration 65 breaths/minute. On examination, he had bluish lips and mucous membrane and inspiratory subcostal retractions. Chest X-ray shows "low lung volume". What is the most likely diagnosis of the newborn?
- A. Meconium aspiration syndrome(MAS) C. Transient tachypnea of the newborn
B. Respiratory distress syndrome(RDS) D. Severe asphyxia
118. A "14 months-old" child came to you. The mother reported the child had poor appetite and intermittent diarrhea since two months. History on the card reveals the child had doubled birth weight at twelve months. Which indices of nutritional status most likely used for the child?
- A. Weight for height C. Weight for age
B. Height for age D. Abdominal girth
119. A mother came with her "10-week infant" for immunization as per her schedule. The mother reported the infant has diarrhea and he is under treatment for intestinal malformation. Which vaccine should temporarily be on hold at this visit?
- A. PCV-2 C. Penta-2
B. Polio-2 D. Rota-2
120. A "12 years- old" child came to hospital with history of shortness of breath and intermittent dry coughing. History reveals symptoms worse at night. On physical examination, the child had expiratory wheeze. It is observed that the clinical condition improves and child gets better after salbutamol puff. What is the most likely problem of the child?
- A. Bronchial asthma C. Croup
B. Pneumonia D. Bronchitis

PART V- PUBLIC HEALTH

121. In what order do managers typically perform their managerial functions?
- A. Organizing, planning, controlling, leading
 - B. Organizing, leading, planning, controlling,
 - C. Planning ,organizing, leading, controlling
 - D. Planning, organizing, controlling, leading
122. The degree to which organizational tasks are subdivided into separate work jobs is called
- A. Decentralization
 - B. Division of labor
 - C. Span of control
 - D. Centralization
123. “ To reduce Infant Mortality Rate from 70/1000 Live Births to 50/1000 Live Births by 2018” What does it indicate?
- A. Strategy
 - B. Objective
 - C. Mission
 - D. Vision
124. Which of the following is a correct statement about communication in an organization?
- A. One way communication considers feedback from receiver
 - B. Two way communication is open to feedback
 - C. One way communication allows more audience participation
 - D. Two way communication takes less time
125. One of the following is not a principle of primary health care?
- A.Intersectoral collaboration
 - B.Community participation
 - C.Equality
 - D. Decentralization
126. The test score of a class of 10 students was recorded as 4, 5, 5, 8, 6, 7, 3, 6, 6, and 10 with a mean of 6. Which score has greatest effect on the variance of this data?
- A. 3
 - B. 5
 - C. 6
 - D. 10
127. Which stages of demographic transition is characterized by high mortality and high fertility, with low (moderate) population growth (young population)?
- A. Pre-transitional
 - B. Transitional
 - C. Post –transitional
 - D. Super-transitional
128. Suppose the population of country X is in 2016 is 50 million with a net growth rate of 2.5% per year. What will be the population doubling time for the country?
- A. 32 years
 - B. 38 years
 - C. 28 years
 - D. 22 years

129. One of the following is **true** about type I (α) error during a hypothesis testing.
- A. It is the probability of rejecting the true null hypothesis
 - B. It is the probability of failing to reject true null hypothesis
 - C. It is the probability of rejecting false null hypothesis
 - D. It is the probability of failing to reject false null hypothesis
130. One of the following is cause for increasing of prevalence of cases in a community
- A. High case-fatality rate from disease
 - B. Longer duration of the disease
 - C. Decrease in new cases
 - D. In-migration of healthy people
131. One of the following results of odds ratio (OR) indicates that there is a preventive association between exposure and outcome.
- A. $OR > 1$
 - B. $OR > 2$
 - C. $OR = 1$
 - D. $OR < 1$
132. Which one of the following is not a characteristic of a confounding variable?
- A. The variable should be associated with the exposure under study
 - B. The variable should be independently associated with the outcome
 - C. The variable should lie on the causal pathway between exposure and disease
 - D. The variable should not lie on the causal pathway between exposure and outcome
133. Suppose patients admitted in Debre Markos Hospital with a diagnosis of coronary heart disease and without a diagnosis of coronary heart disease are interviewed about their past experience on smoking to assess the possible association of diabetes mellitus and diabetes mellitus. Which study design do you think more appropriate to study these patients?
- A. Case - control study
 - B. Experimental study
 - C. Cohort study
 - D. Cross-sectional study
134. Which section of a research report helps to interpret and compare your study results with the other previous study results?
- A. Result
 - B. Conclusion
 - C. Discussion
 - D. Recommendation

14

D

14. A 40-year-old Para VII woman presented family planning clinic with her husband for delaying her next pregnancy. After health care provider counsel them her husband agreed for vasectomy.

What is the most important information needed to perform the procedure?

- (A) Diabetes mellitus (C) History of previous surgery
(B) HIV status (D) Their plan to have other baby

15

B

15. A 21-year-old lady comes to family planning clinic to use contraceptive. After counseling, she decides to use a family planning method which has importance to prevent pregnancy as well as HIV/AIDS.

What would be the best method for this woman?

- (A) Cervical cap (C) Diaphragm
(B) Condom (D) Spermicidal

16

A

16. A 32-year-old woman comes to your health center to use family planning method. She had no history of diabetes and hypertension. After counseling, she decides to use a regular method of contraceptive method which has low estrogen and progesterone hormone.

What would be most likely side effect of this method?

- (A) Bleeding patterns changes (C) Cause of ovarian cancer
(B) Risk of Endometrial cancer (D) Risk of symptomatic PID

- A 32-year-old Para V woman presented to a family planning clinic after six days of IUCD insertion. At the time of insertion the procedure was very painful. Currently she has abdominal pain and negative urine HCG.

What is the best diagnosis for this client?

- (A) Partial expulsion of the strings (C) Missing of Strings
(B) Uterine perforation (D) Uterine mass

- A 44 year-old woman presented at family planning clinic for delaying her future pregnancy. She had seven children; hence she wants to delay her pregnancy. On examination her blood pressure was 140/100mmHg and has no abnormal vaginal discharge. The health provider counseled the woman about the available contraceptive methods.

What is the best method for this woman?

- (A) Implants (C) Monthly injectables
(B) Intrauterine devices (D) Tubal ligation

17

19 B

A 27-year-old woman comes to family planning clinic to delay the next pregnancy. She has irregular menstrual cycle from September up to February which was 21, 28, 23, 26, 27 and 33 respectively. She was counseled about each method and she prefers to use natural methods of family planning.

What would be the best method for this woman?

- (A) Lactation amenorrhea method
- (B) Calendar rhythm method
- (C) Standard day method
- (D) Withdrawal method

20 D

A 26-year-old woman comes with complaint of nausea, irregular bleeding, breast tenderness, headache and weight change to your hospital. The woman has started to use combined oral contraceptive pills three weeks ago.

What would be the appropriate management for this woman?

- (A) Stop using combined oral contraceptive pills and change by another method
- (B) Take each pill at any time every day to help reduce irregular bleeding
- (C) Stop using COCs because side effects are the sign of illness
- (D) Take pills with food or at bedtime to help avoid nausea.

21 B

A 21-year-old woman came to family planning clinic to prevent pregnancy after she got sexually active six days ago.

What would be the best emergency contraceptive method for this woman?

- (A) Ulipristal acetate
- (B) Cooper bearing IUD
- (C) One dose regimen of COC
- (D) Two dose regimen of POP

22 D

A 23-year-old woman came to family planning clinic to use emergency contraceptive pills after having unprotected sexual contact before three days. A health care provider counseled her and she decided to use copper bearing Intrauterine device.

What would be the best time to start this method for this woman?

- (A) After 14 days with backup
- (B) After seven days with back up
- (C) Within 14 days without back up
- (D) Within seven days without back up

23 B

A 40-year-old Para VI woman presented at family planning clinic after four years of LNG-IUCD insertion. On examination she was infected by HIV, currently develops advanced clinical disease and no diabetes mellitus.

What is the best management for this client?

- (A) Leave the LNG-IUCD and monitor for pelvic inflammatory disease
- (B) Leave the LNG-IUCD and monitor for sexually transmitted disease
- (C) Remove the LNG-IUCD and monitor for pelvic inflammatory disease
- (D) Remove the LNG-IUCD and monitor for sexually transmitted disease

33

B

33. A 36-weeks Gravida III Para II woman came to nearest hospital with complaint of vaginal bleeding for three days duration. The bleeding was painless, bright red, sudden and scanty in amount. She has no history of falling accident but had one cesarean scar on her last pregnancy.

Which obstetric palpation should be avoided until the cause of bleeding is ruled out?

- (A) Deep palpation (C) Fundal palpation
(B) Pelvic palpation (D) Lateral palpation

34. A 34-year-old pregnant woman came for 1st ANC visit. She is counseled and tested for HIV and found to be reactive.

What additional laboratory investigation should be ordered for this woman?

- (A) Complete blood count (C) VDRL
(B) CD4 count (D) HCT

34

B

35. A 36-week Gravida II and Para I client came for second visit with a complaint of discomfort under the ribs for one week duration. Abdominal examination revealed presence of round hard and smooth mass occupying the fundus and soft, broad, indefinite and non ballotable mass occupying the lower pole of the uterus.

What is the most likely fetal presentation?

- (A) Breech (C) Cephalic
(B) Brow (D) Face

35

A

36. A midwife wanted to conduct a research to detect nutritional deficiency by taking blood samples from pregnant woman in the community. During analysis of the blood samples showed microcytic hypo-chromic red blood cells in around 10% of the samples.

What nutritional deficiency is suspected for this finding?

- (A) Iron deficiency (C) Folic acid deficiency
(B) Calcium deficiency (D) Vitamin B₁₂ deficiency

37. A pregnant woman on her 25 weeks of gestation comes to the health center for her ANC visit. History revealed that her husband was recently diagnosed and starts treatment for Hepatitis B infection. Laboratory result showed that she is free from Hepatitis B infection.

What is the most possible management for this client? When should she get the first vaccine?

- (A) Re-assess Hepatitis B status after three month
(B) Initiate Hepatitis B vaccination immediately
(C) Initiate Hepatitis B treatment immediately
(D) Initiate Hepatitis B treatment after delivery

36

A

HPLC – Midwifery

Hepatitis B is transmitted by infected blood, blood products, and in saliva, vaginal secretions, and semen. It is a sexually transmitted disease.

Acute viral hepatitis....

Management

Supportive medical measures as for the non-pregnant patient.

rest, adequate nutrition, vitamins, sufficient protein, carbohydrate, low fatty diet.

Obstetric management

Early stage of pregnancy: active treatment of the disease, then artificial abortion should be performed.

During midpregnancy and late pregnancy, vitamin K and C should be administered, and actively prevent pregnancy-induced hypertension

During labor and delivery: vitamin K is administered, preparing fresh blood; avoid operative obstetric intervention; shortening the second stage of labor (by a vacuum); preventing the laceration of the birth canal; preventing retained placental fragments or membranes; using of oxytocin.

Postpartum period: Antibiotic drugs, artificial feeding, infant isolation

Prevented by the administration of hepatitis B immune globulin after birth, followed promptly by hepatitis B vaccine in newborn infant.

Treatment of severe hepatitis proper!

Preconception care

Once a patient is diagnosed to be HIV positive the following should be done:

- Counseling on the diagnosis and linkage to trained personnel for further counseling
- Baseline investigations including CD4 and viral load
- Advise on contraception use with focus on avoiding unintended pregnancy; the preference is to give them dual contraception with one of them being condoms.
- Advise on general health including good nutrition

* Adequate caloric intake; consumption of iron rich foods (beans lentils, meat, liver); iron and folate for three months; iodized salt

- Prevention of malaria: use of ITN for women living in malaria endemic areas.

Management protocol on selected obstetrics topics for hospitals. MOH, Ethiopia **182 182**

- Screening & treatment for opportunistic infections & STIs

34.41

Strategy for Rapid Human Immunodeficiency Virus (HIV) Testing of Pregnant Women in Labor

If the rapid HIV test result in labor and delivery is positive, the obstetrical provider should take the following steps:

1. Tell the woman she may have HIV infection and that her neonate also may be exposed
2. Explain that the rapid test result is preliminary and that false-positive results are possible
3. Assure the woman that a second test is being performed to confirm the positive rapid test result

4. To reduce the risk of transmission to the infant, immediate initiation of antiretroviral prophylaxis should be recommended without waiting for the results of the confirmatory test
5. Once the woman gives birth, discontinue maternal antiretroviral therapy pending receipt of confirmatory test results
6. Tell the women that she should postpone breast feeding until the confirmatory result is available because she should not breast feed if she is infected with HIV
7. Inform pediatric care providers (depending on state requirements) of positive maternal test results so that they may institute the appropriate neonatal prophylaxis

38

A

38. A pregnant woman on her 20 weeks of gestation came for ANC visit. Her record showed that she took 4 dose of TT vaccine during her previous pregnancy.

How many dose of TT vaccine should be given to her in this pregnancy?

- (A) One dose (C) Three dose
(B) Two dose (D) Four dose

39

A

39. A pregnant woman on her 10 weeks of gestation came to health facility to start her ANC visit. The midwife finds she has no history of TT vaccination and provide her first dose of TT vaccine.

How many doses of TT vaccine could she receive until her EDD?

- (A) Two (C) Four
(B) Three (D) Five

40

A

40. A 28-year-old gravid woman came to Health center for ANC visit. The physical examination result shows normal finding. The serum investigation in Rapid Plasma Reagin (RPR) described as reactive.

What is the possible diagnosis for this client?

- (A) Malarial infection (C) HIV/AIDS infection
(B) Gonorrhea infection (D) Syphilis infection

41. A pregnant woman with gestational age of 20 weeks came to ANC clinic for first visit. Physical examination showed that: BP 120/70 mmHg, PR 90/min, RR 20/min and afebrile to touch. Lab result indicated that: Hgb 10.5g/dl, HIV reactive VDRL, non-reactive and Platelet count 250×10^3

What is the most likely diagnosis for this woman?

- (A) AIDS (C) HIV infected
(B) HIV exposed (D) HIV negative

- A 34-weeks pregnant woman presented with chief complaint of vaginal bleeding for three days duration. History revealed the bleeding started suddenly and it was painless. Physical examination showed soft and relaxed abdomen, positive fetal heart beat and leakage of bleeding per vagina.

What is the most likely diagnosis?

- (A) Ante partum hemorrhage (C) Molar pregnancy
(B) Ectopic pregnancy (D) Uterine rupture

43
B

43. A 36-weeks Gravida III Para II woman admitted to a hospital with a diagnosis of ante partum hemorrhage secondary to placenta previa. Ultrasound scanning result revealed that the placenta located posteriorly and reaches the margin of the internal cervical os but does not cover it.

What is the degree of placenta previa based on ultrasound finding?

- (A) Type-I (C) Type-III
(B) Type-II (D) Type-IV

44. A 10-week Primigravida woman is admitted in a hospital with a diagnosis of hyperemesis gravidarum. She had history of nausea and villous vomiting with an episode of three times per day. And laboratory investigation is sent for urinalysis.

What is the most likely laboratory finding for this problem?

- (A) High chloride level (C) Presence of ketone
(B) Low specific gravity (D) Presence of protein

44
C

5. A 42-year-old, Gravida II Para I with gestational age 22-week by LNMP came to the hospital with chief complaint of blurring of vision and epigastric pain for two days duration. She had associated frontal headache which is unable to relieve with regular analgesics. The urine output measures 70 ml /hour. The diastolic blood pressure also 110 mm Hg on two occasions. In addition, urinalyses showed trace Proteinuria.

What is the most likely diagnosis?

- (A) Eclampsia (C) Severe pre-eclampsia
(B) Mild pre-eclampsia (D) Gestational hypertension

45
C

- A 30-week primigravida woman presented with abdominal distention started three weeks back. Abdomen is markedly enlarged and looks globular with fullness at the flanks. The fundal height is 38 weeks with fluid thrill. The ultrasound result showed that the amniotic fluid volume is 11 cm by the largest vertical pocket and absence of fetal skull.

What is the most likely cause of abdominal distension?

- (A) Anencephaly (C) Esophageal atresia
(B) Hydropsfetalis (D) Open spinal bifida

- A 32-week Gravida IV Para III woman came in the nearest hospital for her 2nd ANC visit. She has flank pain and shortness of breathing for the last two weeks. Physical examination revealed a 40 weeks sized gravid uterus with barrel shaped abdomen. Many fetal parts are palpable. Ultrasound result showed that amniotic fluid with the largest vertical pocket measures 6cm.

What will be the most likely diagnosis?

- (A) Fibroid (C) Polyhydraminous
(B) Ovarian cyst (D) Multiple pregnancy

Polyhydramnios...

Causes:

Diabetes mellitus

Placental tumor

RH isoimmunization

Multiple pregnancy, common with mono zygotic twins

Fetal anomalies

Esophageal atresia

Spinal bifida

Anencephaly

Clinical Manifestations:

Breathlessness and discomfort

Pressure pain in the back, abdomen and thighs.

Exacerbation of minor disorders of pregnancy.

Edema and varicosities of the vulva and lower limbs may be present.

Unstable lie

Abdominal examination

On inspection

The uterus is larger for expected GA

Stretched and shiny with marked striae gravidarum and

Obvious superficial blood vessels.

Auscultation

Fetal heart is difficult to hear

Ultrasonic scan may be used to confirm the diagnosis

On palpation

The uterus feels tense

difficult to feel the fetal parts

Fetus may be balloted between the two hands

A fluid thrill may be elicited

Management

Mild asymptomatic-Managed expectantly

The cause should be determined

The women is not admitted

Advise to get adequate rest

Subsequent care will depend on:

The mother's condition

The Fetal condition

Severe-Symptomatic Polyhydramnios

The mother should be admitted to hospital

An upright position helps to relive dyspnea

She may be given antacids to relive heart burn and nausea.

Up to 500 ml Amniotic fluid can be removed

Labor will be induced if the symptoms become worse or gross abnormality is diagnosed.

48. A 37-year-old Primigravida lady came to gynecology OPD at GA of 12-weeks with a complaint of repetitive vomiting. History revealed that she is unable to perform regular activities. Physical examination findings include BP-90/50 mmHg and PR-100 beat per minute and dry coated tongue. Laboratory investigation shows Urine ketone +3.

What is the most appropriate initial management in this case?

- (A) Rehydrate with IV fluid
- (B) Start nasogastric feedings
- (C) Provide IV antiemetic drugs
- (D) Assess serum level of electrolyte

49. A 28-year-old Gravida-IV and Para-III mother presented with absence of fetal movements for one day duration at gestational age of 32 weeks. Physical examination revealed that fundal height of 30 cm and fetal heart beat not heard on fetoscope.

What is the most likely next management for this client?

- (A) Conduct laboratory test
- (B) Perform ultrasound scan
- (C) Tell her that her fetus is dead
- (D) Admit for observation

50. A 32-year-old Gravida-2, Para-1 woman on her 32 weeks of gestation presents with a complaint of fast growing abdomen associated with fatigue and dyspnea. Abdominal examination shows fundal height of -36cm and Fetal heart beat is not audible with fetoscope. Ultrasound shows singleton live fetus with cranium absent and Pocket -10cm.

What is the most possible management?

- (A) Terminate the pregnancy
- (B) Advise to take bed rest

Labor will be induced if the symptoms become worse or gross abnormality is diagnosed.

- (C) Perform amnioreduction
- (D) Provide indomethacin

- B A 27-year-old Gravida-III and Para-II mother from remote area is presented with one episode of vaginal bleeding at GA of 34 weeks. The bleeding is dark red and soaks her underwear only. Physical examination showed BP-110/70mmHg, PR- 88 beat per minute, and RR-20 breath per minute, T^o-36°C, no uterine contraction and Fetal heart rate - 136beat per minute. Ultrasound examination revealed placenta implanted on uterine fundus.

What is the most likely management that should come next for this client?

- (A) Admit for observation
- (B) Reassure and send home
- (C) Advise and appoint her after a week
- (D) Send her home after 4 hours evaluation

CLASSIFICATION

Table 7. Classification of abruptio placenta

Clinical presentation	Mild (Grade1)	Moderate (Grade 2)	Severe (Grade 3)
Amount of blood	<400 mL	400-1000mL	>1000mL
Uterine irritability (pain/tenderness)	Normal / slightly increased	Increased	Tetanic, reactive,board like, tender on palpation
Fetal condition	No fetal heart abnormality	Abnormal / rarely death	Fetal distress or death common
Shock	Absent	Mild, posturalhypotension	Severe and always present
Fibrinogen level	Normal	Slightlydecreased	Decreased

DIAGNOSIS

The clinical presentation of abruptio placenta mainly depends on the extent of placental separation, rate of separation and flow of blood through the cervix (concealed/ revealed).

- *Vaginal bleeding: menstrual-like (dark), totally concealed or the amount is less than the degree of the shock*
- *Abdominal pain/ (uterine) tenderness:*
- *NRFHRP or absent fetal heart beat*
- *Coagulation defect: frank bleeding (epistaxis, ecchymosis, petechiae)*

INVESTIGATIONS:

- *HCT*
- *Blood group and Rh,*
- *Coagulation profile: Platelet count, PT, PTT, fibrinogen or bedside clotting and bleeding test*
- *Ultrasound: Fetal assessment, retroplacental clot and for exclusion of placenta previa.*

TREATMENT:

- *Resuscitate and stabilize on arrival, and admit the patient*
- *Assess maternal and fetal wellbeing*
- *Prepare cross matched blood (at least 2 units)*

Expectant management: <37 weeks, patient in stable condition and reassuring fetal condition

- *Give dexamethasone 6 mg IM BID or Betamethasone 12 mg IM every 24 hours for 48 hours if GA < 37 weeks*
- *Anti D 300µg IM if Rh negative and not sensitized*
- *Closely monitor maternal and fetal conditions*
- *Prevent and treat anemia*

Immediate delivery: Gestational age is >37 weeks/estimated fetal weight >2.5 Kg, deranged vital signs, heavy bleeding, NRFHRP, IUFD, malformed fetus, established labor

- *Mode of delivery: Vaginal delivery is preferred.*
- *Cervical ripening and induction of labor, amniotomy*
- *Emergency cesarean section: For severe bleeding endangering maternal life, NRFHR or other Obstetrics indications*

COMPLICATIONS

- *Hemorrhagic shock (acute kidney injury, congestive heart failure), • DIC • Utero-placental insufficiency that may lead to IUGR, fetal distress or IUFD. • PPH*

52

B

52. A 38-year-old primigravida lady presented with severe headache at gestational age of 38-week. She has blurring of vision and epigastric pain. Physical examination revealed BP-150/100mmHg, PR- 88beat per minute, RR-20breath per minute and no uterine contraction. Laboratory investigation indicated +2urine protein.

What would be the most appropriate initial management for this woman?

- (A) Induce labor (C) Administer Hydralazine
(B) Administer MgSo4 (D) Admit her to high risk ward

53

BPP
=2+1
+2+1
+2=8
(IUG
R)

B

53. A 37-year-old Gravida-V and Para-IV came to ANC clinic for 4th visit at GA of 37 weeks. Physical examination revealed:

- stable vital sign
- same weight measurement as previous visit
- Fundal height-31cm
- FHB -136/min
- No uterine contraction
- Cephalic presentation.

Vaginal examination indicated the cervix is soft, 1cm dilated, anterior position and 50% effaced. Fetal head is at station 0.

What is the most appropriate management for this woman?

- (A) Prepare her for c/s
(B) Admit her to high risk ward
(C) Perform rupture of membrane
(D) Send home with advise to come when labor starts

- A 27-year-old primigravida lady comes to a clinic for 1st ANC visit at gestational age of 22weeks. Physical examination revealed stable vital signs, fundal height-28cm and two fetal heart rate (FHR) heard (FHR1=132 and FHR 2=144beat per minute).

What is the most possible management for this woman?

- (A) Admit for observation
(B) Appoint her after 8 weeks
(C) Advice on additional calorie intake
(D) Perform ante partum fetal surveillance

Diagnosis of twin pregnancy

family history of twin

Ultrasound (two heads at 15 weeks)

X-ray after 12 weeks

Abdominal examination

Inspection

The size of the uterus may be larger than expected period of gestation after 20 week

Palpation

The fundal height may be greater than expected for the period of gestation

The presence of two fetal poles (head or breech) multiple limbs

Lateral palpation may reveal two backs or limbs on both sides

Pelvic palpation of fetus may lie behind the other and make palpation difficult

Auscultation

Hearing two fetal hearts difference of at least 10 beats per minute

Effect of twin pregnancy

Exacerbation of minors disorders nausea morning sickness and heart burn may be more persist

Anemia iron deficiency

Pregnancy induced hypertension (greater in monozygotic)

poly hydro amnions common in monozygotic and fetal abnormalities

Dyspnea and indigestion back ache are common

Pressure symptoms tendency of edema of the ankle and varicose veins is increased

Effect on laboure

Occurs before term because of over stretching of the uterus or

May be induced because of complication

Preterm babies may be light for gestation and mal-presentation

Management of delivery

First stage of labour

Preparation for the reception of two immature babies

If fetal distress occur delivery will need expedited often by C/S

If the uterine activity is poor I.V oxytocine may be required

if preterm pregnancy neonatal care unit should be informed(two incubators)

Second stage of labour

obstetrician ,anesthetist and pediatrician should be present because of risk of complication

Resuscitation equipment should be prepared

the delivery trolley should include

equipment for episiotomy and aminotomy

forceps and extra cord clamps

elective episiotomy may be considered for preterm and fetal distress

The second stage is conducted up to the birth of the first baby

After delivery of the first twin abdominal examination is made to ascertain the lie ,presentation and position of the second fetus and to auscultate the fetal heart beat

If the lie is not longitudinal an attempt is made to correct it by external cephalic version

If the presenting part is not engaged it should be pushed in to the pelvis by fundal pressure before the membrane rupture

Stimulate the contraction with I.V syntocine

When the presenting part is visible encourage the mother to push during contraction

With three or four good contraction and effective pushing the 2nd baby has to be delivered with in 15 minutes

label the twins

55

C

55. A 30-year-old gravid HIV positive mother with gestational age of 33 weeks came to ANC clinic with complain of head ache for two days duration. History revealed that the head ache was continuous and associated with fever and sweating for two days duration. She had also nausea and vomiting including loss of appetite. Lab investigation was reactive for PF. Physical examination showed that: BP 90/60 mmHg, PR 90 B/min, RR 24, T° 38°C and MUAC 18 cm.

What nutritional care plan should be applied for this patient?

- (A) Nutritional care plan A
- (B) Nutritional care plan B
- (C) Nutritional care plan C
- (D) Nutritional care plan D

A 35-year-old HIV positive gravid woman came to ANC unit for follow up based on her appointment. She started ART drugs one month back. During the assessment of her adherence level it is found that she missed two doses ART drugs because of illness.

What is the ART level of adherence?

- (A) Excellent
- (B) Good
- (C) Fair
- (D) Poor

56

B

A gravid woman who is on ART for two years came for ANC visit on her 28 weeks of gestation. On history, it is found that her husband who is living with her had pulmonary TB and is currently on anti TB drugs. She had no fever, cough and night sweating. The Physical examination shows appropriate weight increment according to gestational age.

What is the best drug for this client?

- (A) Isoniazid
- (B) Erythromycin
- (C) Co-trimoxazole
- (D) Anti-TB drug regimen

A gravid mother who is linked to PMTCT clinic one month back complains of having a problem taking the ART drugs. During revision of history it is revealed that her husband is not supportive of her taking the drugs.

What is the best management option for this problem?

- (A) Litigation
- (B) Stop the drug until he got agreed
- (C) Provide scientific information
- (D) Take the drug without his knowledge

57

C

	%	Missed of 30 doses	Missed of 60 doses
Good (G)	≥ 95	≤ 2 doses	≤ 3 doses
Fair (F)	85-94	3-5 doses	4-9 doses
Poor (P)	< 85	≥ 6 doses	> 9 doses

Good level (rate of 95% or above) of drug adherence is required for optimal virologic, immunologic, and clinical outcomes.

Good adherence ensures maximum durability of the first-line ARV regimen.

The success of any adherence strategy depends on:

education of patients before the initiation of ART (assist)

an assessment of their understanding of the therapy (Assess)

their readiness for treatment (Agreement).

59

C

A 35-year-old Gravida-II and Para-I mother came for 1st ANC visit with a complaint of headache. History revealed that she is easily fatigable and gestational age is 20 weeks by LNMP. On physical examination-BP 100/70mmHg, PR-100beat per minute, RR-22breath per minute and pale conjunctiva were revealed. Laboratory investigation shows Hct level of 27% and Mean Corpuscular Volume- 60m³.

What is the best management for this woman?

- (A) Provide Iron-dextran 50 mg IM
- (B) Transfuse with cross matched blood
- (C) Provide Iron folate tablet twice daily
- (D) Send home with advice to take Iron rich foods

A 25-year-old primigravida lady came for her first ANC visit. The blood group and Rh investigation shows an A⁻ (A negative) result.

What is the most likely management that should come next for this client?

60

C

- (A) Detect fetal blood group
- (B) Provide anti D antibodies
- (C) Detect partner blood group
- (D) Perform indirect coomb's test

A 32-year-old Gravida-III and Para-II mother presented with severe headache at gestational age of 35 weeks. Physical examination revealed BP-180/110mmHg. The midwife provided hydralazine 5mg IV.

What is the most possible time of evaluation for this woman?

- (A) Every 20 minutes
- (B) Every 1 hour
- (C) Every 2 hours
- (D) Every 4 hours

A Gravida II Para I woman came to delivery ward with complaint of pushing down pain for the last 4 hours. Her last menstrual period was normal and stayed from June 20-25, 2010 E.C. The visiting date was 20/06/2011 E.C.

What was the gestational age at the time of visiting?

- (A) 33 weeks and 4 day
- (B) 34 weeks and 2 days
- (C) 35 weeks and 0 day
- (D) 38 weeks and 4 days

A Gravida II Para I woman came to delivery ward with complaint of pushing down pain for the last 4 hours. Her last menstrual period was normal and stayed from Pagume 1-4, 2010 E.C. The visiting date was 05/09/2011 E.C.

61

A

When was her expected date of delivery?

- (A) 06/10/2011 E.C.
- (B) 09/10/2011 E.C.
- (C) 11/10/2011 E.C.
- (D) 14/10/2011 E.C.

20+25
AS
23

20/25 to
S 9 23/20
S 9

28/15

22/20

2006

3+4x6

24+3

27

4x7 23/20

Anticonvulsant therapy (seizure prophylaxis)

- Give Magnesium sulfate as shown in the box below.
- In all severe preeclamptic mothers during admission & continued during period of evaluation & observation for 24 hours.
- Diazepam: may be used as alternative, if MgSO₄ is not available (as shown below).

Control hypertension

Administration of antihypertensives should be started if the systolic blood pressure is 160 mmHg or higher and/or the diastolic blood pressure is 110 mmHg or higher. Hydralazine or labetalol is the drug of choice for acute control.

Note: An important principle is to maintain blood pressures above the lower limits of normal.

Hydralazine

- Give 5 mg IV slowly every 20 minutes until blood pressure is lowered (to diastolic blood pressure <110 mmHg). The maximum dose is 20 mg per 24 hours.

Labetalol

- *Oral treatment:* Administer 200 mg; repeat dose after one hour until the treatment goal is achieved. The maximum dose is 1200 mg in 24 hours.
- *Intravenous treatment:* Administer 10 mg IV. If response is inadequate after 10 minutes, administer 20 mg IV. The dose can be doubled to 40 mg and then 80 mg with 10-minute intervals until blood pressure is lowered below threshold. The maximum total dose is 300 mg; then switch to oral treatment.

Nifedipine

- As alternative for acute therapy, administer 10 mg orally. Repeat dose after 30 minutes if response is inadequate until optimal blood pressure is reached. The maximum total dose is 30 mg in the acute treatment setting. For maintenance therapy 10-20 mg PO bid is given.

Alpha methyl dopa

- Administer 250-750 mg every six to eight hours. The maximum dose is 3000 mg per 24 hours.

64

D

64. A 37-weeks Gravida II Para I woman was admitted in obstetrics ward for the diagnosis of active first stage of labor. She had 4 antenatal care visits and all recommended laboratory investigation has been done.

What laboratory investigation is most relevant at admission?

- (A) Blood group & Rh (C) Hepatitis B
(B) VDRL (D) Hematocrit

65. A pregnant woman gave birth with spontaneous vaginal delivery. After examination of placenta, the health care provider evaluates the perineum and extended vaginal tear to the rectal mucosa, muscle and anal sphincter was identified.

What is the degree of tear?

- (A) 1st (C) 3rd
(B) 2nd (D) 4th

65

D

66. A 25-year-old primigravida woman presented with complaint of pushing down pain. On pelvic examination one foot inferior to the buttock was identified..

Which operative delivery is best indicated for this woman?

- (A) Cesarean (C) Forceps
(B) Vacuum (D) Version

67. A 35-year-old G-III-P-II woman presented with true labor at 38+5 weeks of gestation at 2 o'clock local time. On physical examination cervical dilation was 6 cm, fetal position was left occiputo-anterior and amniotic membrane was ruptured. After re-evaluating her at 6 o'clock you found full cervical dilatation. She has not been given any regional anesthesia. But at 7:30 o'clock she is exhausted to push adequately and still in labor with station of +1 and FHB 160 beats/minute.

66

A

What is the most likely operative delivery indicated for this woman?

- (A) Cesarean (C) Forceps
(B) Vacuum (D) Version

68. A primigravida woman is in 2nd stage of labor at 33+2 weeks of gestation. On P/E uterine contraction-3/10'/40-50'', station- 0, FHB-120 beats/minute, BP 120/110 and protein urea of 3+. Shortening 2nd stage of labor was mandatory.

67

B

What is the most likely operative delivery indicated for this woman?

- (A) Cesarean (C) Forceps
(B) Vacuum (D) Version

FORCEPS DELIVERY

DEFINITION

Forceps delivery is an assisted vaginal delivery effected using obstetric forceps.

CLASSIFICATION:

- • **Low forceps:** applied when the station is +2 or below.
- • **Outlet forceps:** applied when the fetal head is at station +3 (at the pelvic floor).

INDICATIONS:

- • The same as indications for vacuum delivery. In addition, it can be applied for after-

Management protocol on selected obstetrics topics for hospitals. MOH, Ethiopia **154 154**

- coming head in breech presentation (Piper's forceps) and mentoanterior face presentation.

PREREQUISITES:

69. A 26-year-old Gravida-II Para-I woman is admitted with the diagnosis of true labor at 3 o'clock local time. The woman has no bad obstetric history. At admission cervical dilatation was 4cm, amniotic membrane was ruptured, station -2, pelvic was adequate, uterine contraction 2/10'/20-25", and FHB 140beat/minute. During re-evaluation at 7 o'clock cervical dilatation was 5cm, station -1, uterine contraction 2/10'/30-35" and FHB 130bpm.

What next step is expected to be done?

- (A) Induction (C) Cesarean delivery
(B) Augmentation (D) Vacuum delivery

70. A Gravida II Para I woman came with a complaint of pushing down pain for 10 hours duration. Physical examination revealed 8 cm dilated edematous cervix, amniotic membrane is ruptured, prolapsed cord. U/S finding showed amniotic fluid index of 4cm and low placental attachment. Footling breech was diagnosed.

What clinical feature associated with the diagnosis?

- (A) Oligohydramnios (C) Prolapsed cord
(B) Edematous cervix (D) Placenta previa

- B? A Gravida I Para O woman with gestational age of 38 weeks was admitted in obstetrics ward for the diagnosis of second stage of labor. On physical examination BP 110/70mmhg, PR 82 b/m, RR 23 b/m, adequate uterine contraction and FHB 140 b/m. After 1 hour of admission she gave birth to a 3.8 kg female neonate. Following expulsion of the placenta, profused vaginal bleeding has occurred. While evaluating her, BP-85/40mmhg, Pulse-130(weak), RR-35breath/minute, pale conjunctiva, and scanty urine output. Shock was diagnosed.

What is the most likely clinical finding that helped to diagnose shock?

- (A) Rapid breathing (C) Low blood pressure
(B) Pale conjunctiva (D) Scanty urine output

71. A pregnant woman came to delivery ward with complaint of pushing down pain for the last 4 hours on 21/06/2011 E.C. Her last normal menstrual period was from June 1 - 6, 2010 E.C. On physical examination uterine contraction 3/10'/25-35", cervical dilation 3 cm.

C What would be the most likely diagnosis?

- (A) Post-term labor (C) Preterm labor
(B) Post date labor (D) Term labor

81
A

81. A 38-weeks Gravida II Para I woman was admitted in obstetrics ward for the diagnosis of active first stage of labor? On per vaginal examination, cervix 8 cm dilated, membrane ruptured, station -1, cord is easily palpable by fingers before the fetal presenting part.

What is the most likely diagnosis?

- (A) Cord prolapse
- (B) Occult prolapse
- (C) Cord presentation
- (D) Cord entanglement

82
B

82. A 37-weeks Gravida IV Para III woman was admitted in obstetrics ward for the diagnosis of active first stage of labor. On physical examination uterine contraction is 5/10'/40-55'', the cervix dilated fully, station is -2, position is left occiputo anterior, membrane ruptured, uterus is tender, and FHB 130beats/minute. Suddenly, the uterine contraction ceases, the woman bleeds profusely and fetal heart beat absent.

What is the diagnosis?

- (A) Abruptio placenta
- (B) Uterine rupture
- (C) Placenta previa
- (D) Vasa previa

83

C

83. A 38-weeks Gravida III Para II woman came with a complaint of profuse vaginal bleeding for 2 hours duration. On physical examination BP 80/60 mmhg, PR 120 beat/minute, there is no uterine contraction but severe abdominal tenderness, non-reassurance fetal heart beat, fetal part is easily palpable abdominally, cervix was 9 cm dilated and there is non-clotted vaginal bleeding. Based on the findings uterine rupture was diagnosed.

What is the primary clinical finding used for the diagnosis?

- (A) Feto-maternal distress
- (B) Profuse vaginal bleeding
- (C) Diffuse abdominal tenderness
- (D) Abdominal palpation of fetal parts

A 26year-old pregnant mother is presented with complaint of passage of fluid through the vagina at 36+5 weeks of gestation. On physical examination Temp: 38 Oc, RR: 30 breath/minute, Pulse 120 beat/minute and FHB 185 beat/minute, The uterus was tender, cervix admits tip of finger, amniotic membrane is ruptured and there is foul smelling vaginal discharge.

What is the diagnosis?

- (A) Term PROM with chorio-amnionitis
- (B) Term PROM without chorio-amnionitis
- (C) Preterm PROM with chorio-amnionitis
- (D) Preterm PROM without chorio-amnionitis

A

81. A 38-weeks Gravida II Para I woman was admitted in obstetrics ward for the diagnosis of active first stage of labor? On per vaginal examination, cervix 8 cm dilated, membrane ruptured, station -1, cord is easily palpable by fingers before the fetal presenting part.

What is the most likely diagnosis?

- (A) Cord prolapse
- (B) Occult prolapse
- (C) Cord presentation
- (D) Cord entanglement

B

82. A 37-weeks Gravida IV Para III woman was admitted in obstetrics ward for the diagnosis of active first stage of labor. On physical examination uterine contraction is 5/10'/40-55'', the cervix dilated fully, station is -2, position is left occiputo anterior, membrane ruptured, uterus is tender, and FHB 130beats/minute. Suddenly, the uterine contraction ceases, the woman bleeds profusely and fetal heart beat absent.

What is the diagnosis?

- (A) Abruptio placenta
- (B) Uterine rupture
- (C) Placenta previa
- (D) Vasa previa

C

83. A 38-weeks Gravida III Para II woman came with a complaint of profuse vaginal bleeding for 2 hours duration. On physical examination BP 80/60 mmhg, PR 120 beat/minute, there is no uterine contraction but severe abdominal tenderness, non-reassurance fetal heart beat, fetal part is easily palpable abdominally, cervix was 9 cm dilated and there is non-clotted vaginal bleeding. Based on the findings uterine rupture was diagnosed.

What is the primary clinical finding used for the diagnosis?

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What is the diagnosis?

- (A) Term PROM with chorio-amnionitis
- (B) Term PROM without chorio-amnionitis
- (C) Preterm PROM with chorio-amnionitis
- (D) Preterm PROM without chorio-amnionitis

77

A

77. A 38-year Gravida I Para 0 women was admitted in obstetrics ward for the diagnosis of active first stage of labor at 4:00 AM. She is exhausted for pushing long period of time. A midwife follows the labor with partograph and the fetal heart beat pattern line indicates that the last plotted point is 90 b/m in the absence of uterine contraction.

What is the most likely diagnosis?

(A) Fetal distress

(B) Normal heart beat

(C) Tachycardic heart beat

(D) Reassuring fetal heart beat

78. A 21-year-old primi-gravid woman with gestational age of 38 weeks came with a complaint of pushing down pain for 25 hours duration. On physical examination uterine contraction 3/10'/50-55'', descent 4/5, FHB 135 beats/minute. On vaginal examination the cervix dilates fully, station is -3, position is right occiputo anterior and the parietal bone overhangs the symphysis pubis displacing the thumb and you feel prominent ischial spine. Based on this finding prolonged labor was diagnosed.

78

B

What is the most likely cause of prolonged labor?

(A) Mal position and Mal presentation

(B) Cephalo-pelvic disproportion

(C) Uterine dysfunction

(D) Poor maternal effort

A 30-year-old G-II-P-I woman is admitted in maternity ward for the diagnosis of active first stage of labor. On pelvic examination cervix is 5 cm dilated, fetal ischial tuberosities, sacrum and anus was appreciated but no other part of the fetus is palpable.

What type of breech presentation is it?

(A) Incomplete

(B) Complete breech

(C) Footling breech

(D) Frank breech

A 28-year-old Gravida I Para 0 woman presented unconsciously to health facility at 37+3 weeks of gestation. On physical examination, BP 80/50 mmhg, PR 100 BM but week, RR 30 breaths/minute and cold clammy skin. Cervix was 5 cm dilated and there was profuse non clotted vaginal bleeding. After thorough evaluation shock was diagnosed.

What type of shock is it?

(A) Septic

(B) Cardiogenic

(C) Anaphylactic

(D) Hypovolemic

79

D

85

A

85. After assisting normal birth, a midwife wants to use this instrument for the next delivery and the instruments should be free from all microorganisms through instrumental processing.

What step the midwife will follow?

- (A) Decontamination – cleaning -- sterilization
- (B) Cleaning -- high level disinfection -- sterilization
- (C) Cleaning -- decontamination -- high level disinfection
- (D) Decontamination -- high level disinfection -- sterilization

86

B

86. A 30-year-old gravid II Para I mother with GA of 40 weeks came to obstetrics ward at 8:00 am with the complaint of pushing down pain for 2 hours duration. On P/E -vital sign was stable, cervix 2cm dilated, membrane intact, station -2, contraction= 2c/10'/30-40" and FHB was 136BPM. At 12:00 am -cervix 3 cm dilated, membrane intact, station -2, contraction= 2c/10'/30-35", FHB= 144 BPM.

What will be the immediate management for the woman?

- (A) Admit and follow fetomaternal condition.
- (B) Ambulate and reevaluate after 4 hrs.
- (C) Discharge and advise to come back
- (D) Admit and start induction

A 25-year-old primi gravid with GA of 38 weeks came to OPD at 4:00 am with the complaint of pushing down pain for 3 hours duration. On P/E cervix =2 cm dilated, membrane intact, contraction= 2c/10'/30-35" and FHB was 144 BPM. At 12:00 am cervix 4 cm, membrane intact, contraction= 4c/10'/40 seconds and FHB= 152 BPM.

What will be the next management for the woman?

- (A) Admit and follow with partograph
- (B) Ambulate and re-evaluate after 4 hrs.
- (C) Rupture the amniotic membrane
- (D) Augment with Oxytocine

87

A

87. A 32-years-old Gravida IV Para III woman with GA of 39 weeks came to obstetrics ward at 2:00 am with the complaint of pushing down pain for 2 hours duration. On P/E, vital sign was stable, cervix 5cm dilated, membrane intact, station -1, contraction= 3c/10'/30-40 and FHB was 136 BPM. At 6:00 am physical examination was repeated and the finding was cervix 6 cm dilated, membrane intact, station 0, contraction= 2c/10'/30-35, FHB= 144 BPM.

What will be the next management of the woman?

- (A) Follow with partograph
- (B) Rapture the membrane
- (C) Augment with Oxytocine
- (D) Doing caesarean section

89

B

89. A multi Gravida woman with GA of 40 weeks admitted at 2:00 pm with the diagnosis of active first stage of labor. On P/E cervix 4 cm dilated, membrane intact, contraction= 3c/10'/45" and FHB was 144 BPM. At 6: 00 pm the woman was re-evaluated and there was no fetal and maternal abnormality.

What will be the minimum expected cervical dilation at this time?

(A) 5cm

(B) 6cm

(C) 7cm

(D) 8cm

A- Gravida II Para I woman came with a complaint of pushing down sensation for 8 hours duration. After assessing her and fetal condition, she was admitted with a diagnosis of second stage of labor. The midwife was attending the delivery. After delivery of the fetal head there was tightly attached umbilical cord around the neck.

What will be the immediate management?

90

B

(A) Doing episiotomy to deliver easily

(B) Double clamp and cut the cord

(C) Facilitate delivery with vacuum

(D) Slip it over the fetus's head

A 34-year-old woman gave birth to a 2.8 kg male neonate at health institution. 10 IU Oxytocine was given intramuscularly and controlled cord traction was tried. But the placenta was not delivered within 15 minutes.

What will be the next management?

(A) Observe for additional 15 minutes

(B) Repeat another dose of Oxytocine

(C) Manual removal of placenta

(D) Massage the uterus

A 32-year-old woman gave birth to a 3.4 kg male neonate at health center. 10 IU Oxytocine was given intramuscularly and controlled cord traction was tried. But the placenta was not delivered within 30 minutes. There is no sign of PPH.

What will be the next management?

91

A

(A) Manual removal of placenta

(B) Refer for further management

(C) Give additional 10 IU Oxytocine IM

(D) Continuing controlled cord traction for 30 minutes

93
A

93. A multi Gravida woman gave birth to a 3.5 kg male neonate at health institution. After placing the newborn on mother's abdomen the health care provider intended to do cord care.

How will the health care provider cut the cord?

- (A) Tie at 3 and 5 cm from the umbilicus and cut between the ties
- (B) Tie at 3 and 5 cm from the umbilicus and cut beyond the ties
- (C) Tie at 3 cm from the umbilicus and cut the cord
- (D) Tie at 5cm from the umbilicus and cut the cord

94
C

94. A 37-weeks Gravida II Para I woman came to hospital with a complaint of vaginal bleeding for 3 hours duration. On physical examination, she is disoriented, BP 85/40 mmhg, PR 115 beat/minute, Temperature 34.0°C, no uterine contraction, FHB 110 beat/minute and cervix admit tip of finger.

What will be the immediate management?

- (A) Resuscitation
- (B) Catheterization
- (C) Shout for help
- (D) Collecting blood sample

95
C

95. A primi gravid lady with GA of 41 weeks admitted with the diagnosis of second stage of labor. Her perineum was too tight to deliver the fetal head and episiotomy was indicated.

What is the appropriate time for doing episiotomy for the woman?

- (A) When the fetal head is at station+2
- (B) When the perineum starts spontaneous tearing
- (C) When the fetal head distended the vulva 2-3 cm
- (D) When cervix fully dilated since the woman is primigravida

A 35-year-old gravid III Para II woman with GA of 49 weeks admitted with diagnosis of second stage of labor. The fetal head is crowning and the perineum is tight. To deliver the fetus easily and to avoid recto-vaginal fistula, episiotomy is indicated.

What type of episiotomy should be avoided?

- (A) Medial
- (B) Medio-lateral
- (C) Lateral
- (D) Bilateral

A Gravida II Para I woman with gestational age of 38 weeks was admitted in obstetric ward with the diagnosis of prolonged second stage of labor. After evaluation instrumental delivery was decided with labor pain management to assist vaginal delivery.

What nerve should be blocked?

- (A) Vulval
- (B) Pudendal
- (C) Mesenteric
- (D) Gluteal nerve

98

A

23

98. A 28-year-old Gravida III Para II woman with GA of 40 weeks admitted at 2:00 pm with the diagnosis of active first stage of labor. At admission cervix was 5 cm dilated, amniotic membrane was intact. She had 3 uterine contractions in 10 minute lasting 40 seconds each. Partograph was started. At 6:00 pm she was re-evaluated and cervical dilatation was 9 cm.

Where will be cervical dilation plotted on the partograph?

- (A) At alert line
- (B) At the action line
- (C) At the left of the alert line
- (D) At the right of the alert line

99. A 38-weeks Gravida I Para 0 women was admitted in obstetrics ward for the diagnosis of active first stage of labor. After 4 hours of admission, she has 3 strong contractions, decent $1/5^{\text{th}}$; fetal heart beat (FHB) 140 beats/minute, cervix 10 cm dilated, vertex presentation, occiputo-anterior position and station +1. After two hours of full cervical dilatation, FHB fluctuates between 160 and 180/minute and station 2+.

What is the most appropriate decision that a health care provider should take?

- (A) Perform episiotomy and use fundal pressure
- (B) Position the mother left lateral to improve fetal heart beat
- (C) Secure IV line and refer the mother to hospital
- (D) Use vacuum extractor to assist vaginal delivery

99

D

A 37-year-old Gravida V Para IV woman came to the hospital at 37 weeks of gestation with a complaint of pushing down pain for 12 hours duration. on physical examination BP 80/10 mmhg, pulse rate 120/ minute, and she was disoriented, abdomen was distended, uterine contraction is not detectable and fetal part is easily palpable abdominally. Laparotomy was decided to manage.

What makes laparotomy to be preferable?

- (A) Disorientation
- (B) Uterine rupture
- (C) Obstructed labor
- (D) Poor uterine contraction

100

THE END

Ethiopian Health Professionals' Licensure Examination
Midwifery Exam

Hidar 2012/December 2019
PART TWO

BOOKLET CODE: 024
Number of Items: 100

SUBJECT CODE: 14
Time Allowed: 2:30 hours

DIRECTIONS: Each of the following questions is followed by four possible alternative answers. Read each question carefully and **blacken** the letter of your best choice on the separate answer sheet provided.

1. A 27-year-old Gravida I Para 0 women with GA of 39 weeks was admitted to labor ward at 8 am. On P/E: Vital sign was stable, uterine contraction=3/10'/45", Cervix was fully dilated and station was 0.

When is the most appropriate time to evaluate the degree of decent?

- (A) 8:15 am (C) 8:45am
(B) 8:30am (D) 9:00 am

2. A 34 weeks Gravida II Para I woman came to hospital with a complaint of vaginal bleeding for 8 hours duration. On P/E: BP=70/30mmHg, PR=110 beat/minute, RR=28 breath/minute, no uterine contraction and FHB was positive. Shock management was initiated. After 30 minutes, re-evaluation was done and pulse rate is 90/minute, blood pressure is 100/30 mmHg and urine output is 30ml.

What further management is expected?

- (A) Blood transfusion
(B) The mother is not yet stable and need extra fast running fluid
(C) The mother is responding to treatment and needs maintenance fluid
(D) Maternal condition is not improving and need to add,2000 ml fluid/30 minute

3. A midwife has started examining the genitalia of a woman who had a normal child birth. While the woman was examined, she started to hold and relax her perineal muscle as if she was ceasing urination.

What kind of exercise was she doing?

- (A) Keagel's exercise (C) Pelvic exercise
(B) Rectal exercise (D) Psoas exercise

20. A 30-year-old woman comes with complaint of heavy and prolonged bleeding, breast tenderness and headache with blurring of vision at a certain hospital. The woman started monthly injectables three months ago.

What would be the most likely management for this woman?

- (A) Evaluate her because prolonged bleeding is not caused by monthly injectables
(B) She should stop and change to a method without estrogen
(C) Recommend her to avoid from using a supportive bra
(D) Continue monthly injectables with backup methods
21. A 30-year-old woman comes to a family planning clinic with complaint of expulsion of IUD with her menstrual cycle six days before. She has history of unprotected sexual contact.

What would be the best emergency contraceptive method for this woman?

- (A) Ulipristal acetate
(B) Cooper bearing IUD
(C) One dose regimen of COC
(D) Two dose regimen of POP
22. A 34-year-old woman who uses injectable contraceptive to delay her next pregnancy came to a hospital with a fear of pregnancy as a result of missing her injection.

What would be the appropriate time to administer emergency contraceptive method for this woman?

- (A) More than 4 weeks late for her repeat injection of DMPA
(B) More than 1 weeks late for her repeat injection of NET-EN
(C) More than 3 days late for her repeat monthly injection
(D) More than 1 week for all injectables contraceptive
23. A 38-year-old woman presented at family planning clinic for complaints of bleeding which lasted for 9 days after implant insertion. She didn't start any medication for this problem. On examination her blood pressure was 120/80mmHg.

What is the most appropriate management for this client?

- (A) Combined oral contraceptives with 50 µg of ethinyl estradiol
(B) Provide Combined oral contraceptives for two weeks
(C) Provide Ibuprofen 400mg daily after meals for 5 days
(D) Provide 50 µg ethinyl estradiol daily for one week

8. A 36-year-old married and sexually active woman who came to gynecology (GYN) with the complaint of vaginal discharge of 1 month duration. STI was diagnosed and managed accordingly with her partner. After initial treatment the woman didn't respond.

What is the next management for this woman?

- ☒ (A) Assess for HIV infection
- (B) Change the regimen of the treatment
- (C) Continuing the initial management
- (D) Doing culture and sensitivity test

9. A 34-year-old woman comes to your hospital to delay her next pregnancy. She has no history of irregular vaginal bleeding and smoking. After counseling, she decides to use progesterone only pills.

What would be an appropriate medical history required before providing the above method?

- (A) Iron deficiency anemia
- (B) Mild hypertension
- (C) Thyroid disorder
- (D) Liver cirrhosis

10. A 34-year-old Para I clinical stage four HIV positive woman, came to family planning clinic to use contraceptive. She is unmarried and diagnosed for cervical pre-epithelial lesion. She has no history of allergy for latex.

What would be the best barrier method for this woman?

- (A) Cervical cap
- (B) Diaphragm
- ☒ (C) Condom
- (D) Spermicidal

11. A 29-year-old woman comes to family planning clinic for use of contraceptive and she complains vaginal discharge associated with lower abdominal pain for five weeks duration. She has no history of breast cancer. On physical examination, adnexal tenderness and cervical motion tenderness were noted. On laboratory examination, gram negative diplococcal were reported.

What contraceptive method would be contraindicated for this woman?

- (A) IUCD
- (B) Implant
- (C) Injectables
- (D) COCs

20. A 30-year-old woman comes with complaint of heavy and prolonged bleeding, breast tenderness and headache with blurring of vision at a certain hospital. The woman started monthly injectables three months ago.

What would be the most likely management for this woman?

- (A) Evaluate her because prolonged bleeding is not caused by monthly injectables
- (B) She should stop and change to a method without estrogen
- (C) Recommend her to avoid from using a supportive bra
- (D) Continue monthly injectables with backup methods

21. A 30-year-old woman comes to a family planning clinic with complaint of expulsion of IUD with her menstrual cycle six days before. She has history of unprotected sexual contact.

What would be the best emergency contraceptive method for this woman?

- (A) Ulipristal acetate
- (B) Cooper bearing IUD
- (C) One dose regimen of COC
- (D) Two dose regimen of POP

22. A 34-year-old woman who uses injectable contraceptive to delay her next pregnancy came to a hospital with a fear of pregnancy as a result of missing her injection.

What would be the appropriate time to administer emergency contraceptive method for this woman?

- (A) More than 4 weeks late for her repeat injection of DMPA
- (B) More than 1 weeks late for her repeat injection of NET-EN
- (C) More than 3 days late for her repeat monthly injection
- (D) More than 1 week for all injectables contraceptive

23. A 38-year-old woman presented at family planning clinic for complaints of bleeding which lasted for 9 days after implant insertion. She didn't start any medication for this problem. On examination her blood pressure was 120/80mmHg.

What is the most appropriate management for this client?

- (A) Combined oral contraceptives with 50 µg of ethinyl estradiol
- (B) Provide Combined oral contraceptives for two weeks
- (C) Provide ibuprofen 400mg daily after meals for 5 days
- (D) Provide 50 µg ethinyl estradiol daily for one week

53. A 38-year-old Gravida-II and Para-I woman was brought by ambulance to an emergency room. Her relatives witnessed one episode of convulsion. On physical examination, she was unconscious, BP=130/90mmHg, PR=110beats per minute and RR=11 breath per minute.

What would be the best initial management for this woman?

- (A) Shout for help
- (B) Provide nasal oxygen
- (C) Send laboratory investigation
- (D) Start loading dose of $MgSO_4$

54. A 32-year-old Gravida-III Para-II mother was presented with leakage of fluid from vagina for two hours duration at gestational age of 38 weeks. On physical examination: BP=100/70mmHg, PR=88beats per minute, RR=20 breaths per minute, $T^{0}=36^{\circ}C$, she has no uterine contraction and cephalic presentation. Speculum examination indicated fluid pooling in posterior fornix and cervix was closed with posterior position.

What is the most likely management for this woman?

- (A) Admit to labor ward
- (B) Admit to high risk ward
- (C) Administer Ampicillin 2gm IV
- (D) Ripen cervix using sublingual Misoprostol

55. A 38-year-old primigravida lady was presented with pushing down perineal pain of 6 hours duration at gestational age of 37 weeks. She was diagnosed with multiple pregnancies on ANC follow up. Physical examination revealed: fundal height 42cm, Single fetal cephalic, uterine heart rate 144beats per minute, Engagement 0/5, Presentation both cephalic, contraction 3 over 10 minute lasting 35-40 seconds and cervical dilatation of 3cm. Plain film of abdomen revealed that the head of both twin is at the same level.

What is the most likely management for this woman?

- (A) Prepare for cesarean section
- (B) Advice to ambulate
- (C) Administer tocolytic drugs
- (D) Admit and Follow progress of labor

Ethiopian Health Professionals' Licensure Examination
Midwifery Exam

Hidar 2012/December 2019
PART ONE

BOOKLET CODE: 023
Number of Items: 100

SUBJECT CODE: 13
Time Allowed: 2:30 hours

DIRECTIONS: Each of the following questions is followed by four possible alternative answers. Read each question carefully and **blacken** the letter of your best choice on the separate answer sheet provided.

1. A woman who is planning to get pregnant comes for diabetes mellitus checkup after suspecting symptoms of diabetes.

What diagnostic criterion satisfies the diagnosis of diabetes mellitus?

- (A) Two -hour post load glucose level of 210 mg/dl
- (B) Fasting plasma glucose of 110 mg/dl.
- (C) Decreased urination
- (D) Loss of appetite

2. A couple who has planned to conceive consulted a midwife for adequacy of her nutritional status for pregnancy. The woman weighs 40kg and measures 150cm height.

What is true about the nutritional status of this woman?

- (A) The BMI measures 16.8 kg/m^2
- (B) Maintain the current weight gain
- (C) Higher weight gain is required
- (D) She should avoid intake of high protein foods

3. A 28-year-old married affluent engineer got pregnant despite her plan of conceiving a baby after completing her third degree education.

What type of pregnancy is this couple facing?

- (A) Unwanted but supported pregnancy
- (B) Unwanted and unsupported pregnancy
- (C) Wanted and supported pregnancy
- (D) Wanted and unsupported pregnancy

Time Allowed 1 hour and 50 Minutes

ID Number _____ **year** _____

General instruction

- ✓ The exam has parts only one part:
 - **Multiple Choice Questions # 120**
- ✓ Do not take Mobile phones into examination room. You must not keep it in your hand or put it in front of you even if it is **switched off** until the examination paper is handed over to the invigilator.
- ✓ Any form of cheating will automatically result **Disqualification**
- ✓ Don't start exam until you told to do so by **your invigilator**.
- ✓ **Don't encircle the questions, only fill on Answer sheet**

READ INSTRUCTIONS CAREFULLY BEFORE ANSWERING THE QUESTIONS.

GOOD LUCK

Multiple choice questions

Choose the best answer from given alternatives and encircles the a fill on answer sheet

1. Steps for instrument possessing
 - A. Cleaning, decontamination, high level disinfection, proper Storage.
 - B. Decontamination, cleaning ,high level disinfection, , proper storage
 - C. Proper Storage, decontamination , cleaning, high level disinfection
 - D. Cleaning, proper Storage, decontamination, high level disinfection
2. A Midwife in maternity ward knows that the second step for the management of PPH is directed therapy one of the following is correct statement:
 - A. Manual removal, Curettage for thrombin
 - B. Correct inversion, repair laceration for tissue
 - C. Replace factors identify rupture for tissue
 - D. Give oxytocin , massage, compress for tone
3. A Midwife is caring for post partum client after third post partum day: Client history reveal that para III and has history of labour greater than 18 hour she give birth dead male neonate due to distress in labour, on physical examination pink conjunctiva moist tongue, breast firm tender, and warm and appear tense , the intervention may be:
 - A. Incising and draining the abscess.
 - B. Encouraging the mother to feed.
 - C. Restriction of breast stimulation
 - D. Administering intensive antibiotic
4. The patient hospitalized due to pelvic inflammatory disease is maintained on bed rest and is usually placed:
 - A. Semi-Fowler's position
 - B. Supine position
 - C. Prone position
 - D. Left lateral position

5. Several hours after delivery when assessing a client episiotomy the nurse find there is edema with severe ecchymosis, and the client is complaining of severe rectal pressure the fundus is firm. The vital signs are pulse 108, BP 105/60 this assessment most likely indicate
 - A. Urinary tract infection
 - B. Uterine infection
 - C. Vaginal hematoma
 - D. Postpartum hemorrhage
6. Preventive measure for cervical cancer includes all of the following except:
 - A. Education related to safe sex practice
 - B. Limiting child bearing two three years
 - C. Regular pelvic examination
 - D. Pap test
7. It helps the Midwife to differentiate Hydatidiform mole from other disorders
 - A. Expulsion of grapelike vesicles
 - B. Bright red bleeding
 - C. Continuous passage of urine
 - D. Expulsion of fetus.
8. The Midwife is responsible to teach the client for correct procedure on:
 - A. Pap test
 - B. Mammography
 - C. Breast self examination
 - D. Clinical breast examination
9. A patient with pregnancy induced hypertension probably exhibits which of the following symptoms?
 - A. Proteinuria, headaches, double vision.
 - B. Headaches, double vision, vaginal bleeding.
 - C. Proteinuria, double vision, uterine contractions.
 - D. Proteinuria, headaches, vaginal bleeding.
10. A pregnant client is diagnosed with partial placenta previa. In explaining the diagnosis the nurse tells the client that the usual treatment for partial placenta previa is which of the following?
 - A. Immediate cesarean delivery
 - B. Platelet infusion
 - C. Activity limited to bed rest
 - D. Labour induction with oxytocin

11. A postpartum patient was in labour for 30 hours and had ruptured membranes for 24 hours. For which of the following would the nurse be alert?

- A. Pelvic thrombophlebitis
- B. Salpingitis
- C. Endometritis**
- D. Endometriosis

Scenario 1: Darartu is 26 years old. She came to Jimma University specialized hospitals for delivery service. She is gravida III & para II. She is 38 weeks of gestation. The Nurse conducted PV the findings include palpating cord in front of fetal head, Membrane ruptured & Meconium stained amniotic fluid, Full cervical dilation, No suture overlapping and station zero. On abdominal Auscultation, Fetal heartbeat is decelerating.

12. Based on Scenario 1: What is the possible diagnosis?

- A. Overt UCP**
- B. Occult UCP
- C. Cord presentation
- D. Nuchal cord

13. What amount of oxytocin is required in induction and augmentation respectively?

- A. 6 IU and 12 IU
- B. 6 IU and 6 IU**
- C. 3 IU and 3 IU
- D. 6 IU and 3 IU

14. What is adequate number of contraction to stop Induction and Augmentation?

- A. 2 contraction/10 minutes lasting 20-40
- B. 3 contraction/10 minutes lasting 40-60**
- C. 4 contraction/10 minutes lasting 60-90
- D. 5 contraction/10 minutes lasting 60-90

15. The Presentation when fetus lies flexed attitude, the legs are flexed on the abdomen, and the presenting part is bulky and consists of buttocks external genitalia and both feet is:

- A. Frank breech
- B. Complete breech**
- C. Footling breech
- D. Knee presentation

16. If bleeding occurs after 28 weeks or during labour but before delivery we can suspect:

- A. Abortion
- B. Ectopic
- C. Molar pregnancy
- D. Ruptured uterus

Case Scenario 1: W/ro Sinbon is 38 years old. She comes to a health centre where you are working. Her LMP is 5/12/2007 E.C. Her previous history of gravida include, 20 weeks, 27 weeks termination of pregnancy, 34 weeks dead fetus were born and she gave birth alive three single female children's and two boys which are twin.

17. Based on Case scenario 1, what are W/ro Sinbon Gravida and Para respectively?

- A. 7 and 3
- B. 7 and 4
- C. 7 and 5
- D. 8 and 6

18. The dose of iron given during pregnancy as supplementation

- A. 325 mg tid for 7 days
- B. 325 mg daily for 3 months
- C. 325 mg bid for 3 months
- D. 325 mg tid for 3 months

19. Which drug Combination is currently used to PMTCT?

- A. AZT + 3TC + NVR
- B. TDF + 3TC + EFV
- C. TDF + 3TC + NVR
- D. AZT + 3TC + EFV

20. If the Female pelvis characterized as Heart shaped brim: Narrow fore-pelvis, The side wall is converged, The ischial spines are prominent and Pubic arch <90.

- A. Gynecoid pelvis
- B. Anthropoid
- C. Platypelloid
- D. Android pelvis

21. The effectiveness of Hormonal contraceptions may be reduced by the following drugs, except:

- A. Rifampicin
- B. Amoxicillin
- C. Carbamazepine
- D. Phenytoin

22. The sequence to delivery of fetus is 1. Move up ward, 2. Clean the mouth, 3. Delivery the head, 4. Check the cord tie on the neck, 5. Move down ward, based on this

- A. 2, 3, 5, 4, 1
- B. 3, 4, 5, 2, 1
- C. 3, 2, 4, 5, 1
- D. 1, 2, 3, 4, 5

23. W/ro Aylech is 30 years old and she is gravid IV, come to Yabu Health Center for ANC follow up. On physical examination She is 32 weeks of pregnant and FHB is positive. Her BP 120/70mmhg, PR 88 bpm, laboratory investigation, on urine analysis; Protein is positive, hgb 9 g/dl and VDRL Positive, based on this what is her diagnosis?

- A. Anemia and preeclampsia
- B. Eclampsia and syphilis
- C. Syphilis and preeclampsia
- D. Anemia and syphilis

24. W/ro Ayelech is 25 years old and she is para III. She delivered at home and come to Adola Hospital with her mother with the history of vaginal bleeding while you are working at Oby/gyne ward. When you took history from her mother, the bleeding was started immediately after the delivery of the fetus, the placenta was complete, the uterus was well contracted, and then what is the cause of bleeding?
 - A. Tissue
 - B. Tone
 - C. Thrombin
 - D. **Truama**
25. If one laboring mother comes to Shebe Health Center with history of pushing down pain while you are working at maternity ward, she is gravid II. When you perform physical examination FH is 34 weeks, FHB is positive, on vaginal examination Cervix is fully dilated and station is zero. Which laboratory request is must at this time?
 - A. Hgb
 - B. VDRL
 - C. **PITC**
 - D. BG and Rh
26. When comparing medial episiotomy with medolateral episiotomy which one is false?
 - A. Medial episiotomy is less blood loss than medolateral episiotomy
 - B. Medial episiotomy heal in less time than medolateral episiotomy
 - C. Medial episiotomy is less pain full than med lateral episiotomy
 - D. **Medial episiotomy is less extension to anal sphincter than medolateral episiotomy**
27. W/ro Chaltu is 28 years old. She wants to use natural method of family planning. She adjusts her menstrual cycle for the first six months. She saw her menstrual cycle on the 1st month on December 28, 2015, on the 2nd month January 27, 2016, 3rd on February 29, 2016, 4th on March 25/2016, 5th month on April 30/2016 and on the last month May 26/2016. Based on this when does she protect from sexual intercourse to avoid unplanned pregnancy?
 - A. 6--20
 - B. 7--20
 - C. **7--19**
 - D. 6--19
28. A woman has undergone elective abortion one week ago. Now she comes to the clinic with the chief complaint of hemorrhage. In PE cervix is closed, uterine is contracted with no tenderness. Her temperature is normal. What is the best treatment?
 - A. Doxy 100 mg bid for two weeks
 - B. Again dilate and evacuate uterus
 - C. **Observation and check of Hg and Hct**
 - D. Hormone therapy

29. Case seniero W/ro Ayantu is 32 years old comes to Asedabo Health Health center while you are working at labour ward with history of pushing down pain. She is gravid V, Para IV (Two are alive and one still birth) She has delivered her last pregnancy after two days of labour. She has history of vesico vaginal fistula repairing two years back at JUSH. After you have done physical examination you have got the following findings: BP 100/70 mmhg, PR 96 bpm, FHB 146/min cervix is 4cm dilated, Based on this what is your plan for w/ro Ayantu?
- A. Follow up the labour by partograph
 - B. Catheterize her and follow up the labour by pantograph
 - C. Catheterize her and refer to the Hospital for immediate c/s to avoid recurrence fistula
 - D. Catheterize her and refer to the Hospital**
30. The third stage of labour:
- A. The routine use of oxytocics has been shown to improve hemoglobin level**
 - B. Ergometrine should be routinely used in all patients in the third stage
 - C. Ergometrine may be used safely in management of the third stage of labour
 - D. The incidence if retained placenta after vaginal delivery is 2 per cent
 - E. Waiting 60 min rather than 30 min will have the number of women requiring anesthetics for manual removal
31. The women came with bright red vaginal bleeding; her gestational age is 30 weeks, on examination the lie is transverse, no pain during palpation thus the cause of bleeding might be:
- A. Placenta abruption
 - C. Local lesion.
 - B. Placenta previa**
 - D. Abortion
32. The four pronged National Strategies of prevention of MTCT includes all, except
- A. Primary prevention of HIV infection.
 - B. Prevention of unintended pregnancies among HIV infected women
 - C. Prevention of MTCT among HIV non-infected women**
 - D. Treatment, care and support of HIV infected women, infants and their families
33. Which one of the following intervention is important in reducing MTCT during labor and delivery?
- A. Shorten second stage by using vacuum
 - B. Perform artificial rupture of membrane to fasten the first stage of labor
 - C. Follow labour using partograph and minimize vaginal examination**
 - D. Assist delivery by performing episiotomy

34. A 25-year-old man has been unable to father children. He and his wife undergo an infertility workup. His wife's reproductive function is normal. On physical examination he has descended testes that appear normal in size. A testicular biopsy is performed and on microscopic examination only Sertoli cells are present in the seminiferous tubules. Which of the following laboratory test findings is the most likely to have?
- A. Decreased sperm count
 - B. Elevated serum human chorionic gonadotrophin
 - C. Decreased serum luteinizing hormone
 - D. Increased serum testosterone
 - E. Increased serum alpha-fetoprotein
35. A 21 year old woman comes in for first prenatal visit .On LMP she is 12 wks of she was certain. Upon examination you noted 20 wks uterus, therefore an US is performed and revealed bilaterally enlarged adnexa and a snowstorm pattern in the uterus . Based on these findings what is diagnosis of her?
- A. Ectopic pregnancy
 - B. Twin pregnancy
 - C. Molar pregnancy
 - D. Polyhydroamniotic
36. W/ ro Talile is 30 years old. She is 10 weeks pregnancy when she presents at Shenen Gibe Hospitals while you are working at maternity ward with history of light vaginal bleeding. This is Talile's second pregnancy. The pregnancy was planned and wanted. After you have completed your assessment of Talile's and your main finding including the following: Talile's BP 100/70mmhg, her PR 92bpm her temperature is 37.4She has no skin pallor. She has slight lower abdominal pain and light vaginal bleeding; her uterus is corresponding to the date; She has no uterine tenderness, no cervical motion tenderness. The cervix is closed; based on the above what is Talile's diagnosis?
- A. Threatened abortion
 - B. Inevitable abortion
 - C. Incomplete abortion
 - D. Missed abortion
37. Chaltu is 17 years old. She has normal menstrual flow for 6 months after her first menses. Now she starts with heavy bleeding which lasts many days. The pelvic findings by Ultrasonic scanning are normal. What will be the possible diagnosis of Chaltu?
- A. Puberty menorrhagia
 - B. Metropathia Haemorrhagica
 - C. Premenopausal Menorrhagia
38. All are true regarding post-menopausal bleeding except
- A. It can only be determined after 12 months of spontaneous amenorrhoea for a woman not on Hormonal replacement therapy(HRT)
 - B. Endometrial evaluation is needed for any postmenopausal woman on HRT less than 6 months with persistent uterine bleeding
 - C. The major cause of post-menopausal bleeding is endometrial cancer
 - D. Endometrial evaluation is needed for any woman previously amenorrheic on HRT who begins bleeding without apparent cause

39. Fenet is 21 years old. She has heavy menstrual flow, which lasts 11 days. She is married. She is Para I and gravid II. She is using Injection hormonal contraceptives. Which History is the most important to diagnosis the problem?
- A. Psychosocial History
 - B. Surgical History
 - C. Gynecologic History
 - D. Medical history
40. Which one of the following is not true concerning management of Vesico-vaginal fistula?
- A. It will heal if the bladder is drained continuously for 21 to 28 days
 - B. Surgery can be conducted as soon as it is diagnosed
 - C. Most of the cases is repaired vaginally
 - D. If the cause is cancer, biopsy should have been conducted before surgery is performed
41. All are true concerning Clinical Manifestations of Vaginal Prolapse except
- A. The patient may report a sense of pelvic pressure
 - B. Back pain and pelvic pain may occur
 - C. Rectocele resembles all symptoms cystocele
 - D. Prolapse can result in feelings of pressure and ulcerations and bleeding
42. W/ro Asiya is 28 years old. She is gravida II Para I. She gave birth one month ago. She complains loss of Urine during standing position. What is possible diagnosis of W/ro Asiya?
- A. Urge incontinence
 - B. Stress incontinence
 - C. Total Incontinence
 - D. Overflow incontinence
43. W/ro Bontu is 28 years old. She is gravid II and Para II. She complains pain of the lower out quadrant of her breast on left breast. Breast examination reveals, there is bloody discharge from nipple and orange peel skin of breast. Mammographic examination shows the tumor size of 2cm. The X-ray shows enlargement in supra clavicular area and pectorals muscle involvement. She is diagnosed with breast cancer. What is the Stage of the cancer?
- A. Stage III
 - B. Stage II
 - C. Stage I
 - D. Stage IV
44. Which one of the following is not true about management of Urinary incontinency?
- A. Behavioral intervention can be the only management
 - B. Behavioral intervention can be integrated to Medical management
 - C. Behavioral intervention can be integrated with surgical management
 - D. None

45. Which one of the following urinary incontinency is occur due to a problem related to postoperative period?
- A. Overflow incontinency C. Urge incontinency
B. Stress incontinency D. Total incontinency
46. If the presenting part of the fetus is 1 cm above the ischial spines, the station will be recorded as:
- A. +2 B. -1 C. +1 D. -2
47. In obstetrics ward A 28 year's old Primigravida mother was gave birth two days ago by cesarean section due to cephalo pelvic disproportion secondary to contracted pelvis. You are the one who is giving post partum care for the mother. After all you want to council her postpartum care both for her child and herself before discharge. What is the optimal birth spacing you counsel the mother to minimize adverse health effect of the child?
- A. Three years and older C. Four years and older
B. Two years and older D. One year and older
48. Which one of the following is incorrect regarding the GDM during post Partum period?
- A. Insulin is not required in the first 24 hour or only one forth of previous dose.
B. Maternal insulin requirement fall because fall in HPL after Placenta separation.
C. If the mother has elevated blood glucose and she is lactating provide Oral hypoglycemic
D. The baby born from mother with GDM may develop Hypoglycemia.
49. WHO recommendations for prevention and control of malaria during Pregnancy in sub-Saharan Africa includes all of the following except
- A. Intermittent preventive treatment.
B. Febrile malaria case management.
C. Insecticide treated bed net
D. Continues preventive treatment.
50. Among the following one considered as commonest causes of Mastitis
- A. Candida albicanus.
B. Staphylococcus aureus.
C. Escherichia coli.
D. Clostridium welchi

51. A midwife receive 5 client in postnatal ward from a duty midwife after one hour of delivery from those client in postnatal unit one women develop PPH: the client history reveled she is para six and elevated BGL after 28 weeks of gestation the whole phase of labour completed within 2 hour on physical examination that uterus soft and relaxed based of the history the midwife suspect that the risk for developing this complication might be
- A. Functional/anatomic distortion and infection
 - B. Uterine muscle exhaustion and over distention**
 - C. Uterine anomalies fibroid and multiple gestations
 - D. Intra amniotic infection due to prolonged PROM
52. A client at 36 weeks of gestation with history of one spontaneous delivery being induced the Midwife recognize that possible indication for induction might be
- A. PROM**
 - B. Prolonged pregnancy
 - C. Multiple pregnancy
 - D. Multiparity
53. Midwife understand “retained placenta”
- A. Related to time after full dilation
 - B. Related to time after birth of baby**
 - C. Abnormal throphoblast invasion
 - D. Related to time after active phase
54. A Midwife identify cord prolapsed with absent FHB cervical dilatation 8cm station +1 the appropriate intervention might be
- A. Elevate the foot of the bed
 - B. Pushing back presenting part
 - C. Wait for SVD**
 - D. Immediate cesarean section
55. Primary PPH is bleeding which occur
- A. From 24 hour to 6 week.
 - B. The first 24 month
 - C. The first 24 hour**
 - D. After 24 hour
56. Among the following, one is true about forceps and Vacuum delivery.
- A. If forceps fails vacuum can be used and Vise versa
 - B. Vacuum helps to rotate the head of the fetus
 - C. If the station is above zero Forceps can be used
 - D. Forceps can be used for pre-term labour**

57. The loading dose of magnesium sulfate is given via
- A. IV over 5 minutes, followed by deep IM injection into each buttock
 - B. IV over 5 minutes, followed by deep IM injection into one buttock
 - C. Simultaneous IV and IM injections
 - D. IV bolus, followed by deep IM injection into each buttock
58. Midwife is caring for post partum client after forceps birth with local anesthesia what the midwifery action is appropriate in the immediate post partal period?
- A. Assess the client perineum for hematoma
 - B. Check the client dressing every 30 minute
 - C. Asses bishop score for oxytocin administration
 - D. Teach the client to cough and deep breathe
59. Painful menstruation commonly associated with to pathology
- A. Secondary dysmenorrhea
 - B. Primary dysmenorrhea
 - C. Primary amenorrhea
 - D. Secondary amenorrhea
60. Which one of the following is false about post partum psychosis?
- A. The cause can be infections, drug intoxication, toxemia & blood loss
 - B. The symptoms includes hallucination, delusion, feelings of not wanting to care for the baby , suspiciousness & confusion
 - C. Suicide and infanticide are very rare
 - D. Subsequent pregnancies are associated with an increased risk to develop the disorder
61. A 28wks pregnant mother with the c/m of intractable vomiting and nausea and decreased BP and increased pulse with weight loss and dry tongue is diagnosed as
- A. Morning sickness
 - B. Mulitply pregnancy
 - C. Hydated formole
 - D. Hyperemesis gravidrum
62. Chronic hypertension with new protein urea is defined as
- A. Preeclampsia superimposed on chronic hypertension
 - B. Eclampsia
 - C. Chronic hypertension
 - D. Pre-eclampsia
63. The diagnostic criteria for mild preeclampsia is
- A. Diastolic BP $\geq$ 90-110 and Proteinuria 3+
 - B. Diastolic BP Between 100 and 110 mm Hg and Proteinuria 2+
 - C. Diastolic BP $\geq$ 90-110 and Proteinuria 2+
 - D. Diastolic BP $\geq$ 110 mm Hg or more and Proteinuria 3+

64. The treatment of metritis should include
- A. IV ampicillin or IV gentamicin or IV metronidazole
 - B. Iv ampicillin, plus iv gentamicin and iv metronidazole**
 - C. A combination of oral antibiotics
 - D. A broad spectrum oral antibiotic
65. The correct rate for ventilating a newborn is:
- A. 20 breaths per minute
 - C. 40 breaths per minute**
 - B. 30 breaths per minute
 - D. 80 breaths per minute
66. To help prevent heat loss, the newborn should be
- A. Dried thoroughly immediately after birth
 - B. Dried thoroughly after the cord has been cut
 - C. Dried thoroughly and covered with a clean cloth immediately after birth**
 - D. Covered with a clean, dry cloth after the cord has been cut
67. Newborn cord care involves
- A. Applying a dry dressing to the cord stump
 - B. Swabbing the cord stump with alcohol and applying a dry dressing
 - C. Keep the cord stump dry without putting any substance on it**
 - D. Covered with antiseptic soaked wet gauze
68. Prostaglandins should not be given
- A. Per vaginum
 - C. By IV**
 - B. By IM injection
 - D. By mouth
69. An antihypertensive drug should be given for hypertension in severe pre-eclampsia or eclampsia if diastolic blood pressure is
- A. Between 100 and 110 mm Hg
 - B. 110 Hg or more**
 - C. 115 mm Hg or more
 - D. 120 mm Hg or more
70. Eclamptic fits may occur in the
- A. Antepartum period only
 - B. Intrapartum period only
 - C. Postpartum period only
 - D. Antepartum, intrapartum or postpartum periods**
71. In a breech presentation, the fetal heart beat
- A. Can usually be heard at a location higher than expected for a vertex presentation**
 - B. Can usually be heard at a location lower than expected for vertex presentation
 - C. Can usually be heard in the same location as for a vertex presentation
 - D. Is not able to be heard

72. External rotation of the fetal head indicates that the shoulders are:
- A. In the anterior diameter of the pelvic outlet
 - B. In the posterior diameter of the pelvic outlet
 - C. In the antero-posterior diameter of the pelvic outlet
 - D. Stuck behind the symphysis pubis
73. Cervical dilation plotted to the right of the alert line on the partograph indicates
- A. Satisfactory progress of labor
 - B. Unsatisfactory progress of labor
 - C. The end of the latent phase
 - D. The end of the active phase
74. Before applying controlled cord traction during active management of the third stage of labor
- A. Oxytocin is administered intramuscularly and the attendant waits for the uterus to contract
 - B. The woman's bladder is catheterized
 - C. Pressure is applied to the fundus
 - D. All of the above
75. Plotting on the partograph should begin
- A. In the active phase of the first stage of labor
 - B. In the latent phase
 - C. When the cervix reaches full dilatation
 - D. When the woman is admitted to the labor ward
76. A woman who suffers shock as a result of an obstetric emergency may have
- A. A weak, fast pulse
 - B. High blood pressure
 - C. Normal breathing
 - D. A good urine output
77. Infection can be transmitted from clients to health care workers through
- A. Contaminated needles or other sharps instruments that pierce the health worker's skin
 - B. Splashes in the health care worker's eye of contaminated blood or body fluids
 - C. Broken skin that is exposed to contaminated blood and body fluids
 - D. All of the above
78. The single-most practical procedure for preventing the spread of infection is
- A. Wearing gloves
 - B. Wearing a mask
 - C. Hand washing
 - D. Cleaning skin with alcohol before injections

79. Following insertion of an IUCD, you should recommend that the client, even if she has no problems, have it checked after:
- A. Three days
 - B. **Three to six weeks**
 - C. One week
 - D. Three to six months
80. Iucds can be inserted during:-
- A. Any time during the menstrual cycle provided that the client is not pregnant.
 - B. Immediately after delivery
 - C. Immediately after non-complicated abortion.
 - D. **All of the above**
81. Which one of the following is incorrect about counseling?
- A. REDI framework for counseling helps in assessing and addressing the needs of the client
 - B. Clients have the right and responsibility to make decisions and carry them out.
 - C. **The steps of REDI counseling framework have to be followed strictly in sequential order.**
 - D. An important role for family planning (FP) counselor is to assist clients in anticipating and strategizing about how to overcome the barriers that might prevent them from implementing their decisions.
82. For a client who is late for her DMPA injection, the grace period is:
- A. 1 week
 - B. 2 weeks
 - C. 3 weeks
 - D. **4 weeks**
83. The IUCD may not be the best method for women who have:
- A. Gallbladder disease
 - B. No child
 - C. **Acute/sub acute pelvic infections**
 - D. Hypertension
84. All except one of the following are warning signs that you should teach to an IUCD client, which indicate that she may be having a problem with her IUCD and should seek medical attention
- A. Amenorrhea
 - B. Change in menstrual bleeding pattern in the first 3 months of use.
 - C. Sexual partner has abnormal penile discharge
 - D. **String is longer than usual**
 - E. Pain with intercourse

85. The recommended regimen for medical abortion up to 9 completed weeks in Ethiopia is
- A. Mifepristone 600mg orally followed 48 hrs later by misoprostol 400 microgram vaginally.
 - B. Mifepristone 600mg orally followed 48 hrs later by misoprostol 600 microgram vaginally
 - C. Mifepristone 200mg orally followed 48 hrs later by misoprostol 800microgram vaginally
 - D. Mifepristone 200mg orally followed 48 hrs later by misoprostol 600microgram vaginally
86. All are long acting family planning implants, except
- A. IUCD
 - B. Transdermal patches
 - C. Jaddle
 - D. Sino-implants
87. Which of the following is correct about the importance of male involvement in reproductive health?
- A. Men's role in preventing unwanted pregnancy is often limited to encouraging their wife's to use family planning
 - B. Male involvement slow the transmission of sexually transmitted infections
 - C. They make motherhood safer
 - D. All of the above
88. Which of the following is correct about the importance of male involvement in reproductive health?
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89. Which of the following Abortion care is not given at community level?
- A. Distribution of emergency contraceptives
 - B. Referral to post-abortion and safe abortion services
 - C. Training of Community level workers and junior health professionals
 - D. None of the above

90. During maternal and child health care in the community, which is the best intervention to reduce maternal and child mortality and morbidity.
- A. Promotive care
 - B. Curative care
 - C. Preventive care
 - D. A and C
91. The one included in the basic safe abortion care services is:
- A. Blood transfusion
 - B. Removal of retained products greater than 12 week
 - C. Administer uterotonics
 - D. Perform laparotomy
92. Concerning adolescent health, when we do say adolescents are healthy?
- A. When they feel guilty of antisocial behaviors
 - B. When they are able to cope with environmental stressors life
 - C. When they are unable to engage in the developmental tasks of adolescence
 - D. When they experience a sense of illness and worry
93. Early sexual debut is one of adolescent health problems. One is not true about the reason of this problem?
- A. Hormonal change leading to early sexual maturation and early menarche
 - B. Socio-economic problems
 - C. Permissive attitude of society towards premarital sexual relations for girls
 - D. Conflict and forced migration
94. All are correct reasons about the limited contraceptive use among adolescents except one.
- A. Lack of awareness to anticipate the consequences
 - B. Feeling on not being vulnerable
 - C. Lack of life skill life skill to negotiate sexual indulgence
 - D. User friendliness of reproductive health services
95. All are the problems of adolescents but which one exposes them to morbidity and mortality in developing countries?
- A. Malnutrition
 - B. Addictive Behavior
 - C. Unsafe sex
 - D. Gender based violence
96. All are true about the importance of school health services but one is not.
- A. It has an influence of health on the ability to learn
 - B. Most problems arises as a result of attending school
 - C. Address large group at one place
 - D. It focuses on preventive health care

97. Which type of the female genital mutilation could lead to higher risk of obstetric complications?
- A. Clitordectomy
 - B. Excision
 - C. Infibulation
 - D. Unclassified
98. Which of the following is/are not true about the concepts of reproductive health?
- A. It should primarily focus on women in child bearing age
 - B. It is integral to human development
 - C. It is the Rights of every Human being to be have access to RH
 - D. It addresses health from womb to the tomb
99. What is the importance of serial measurement on child growth monitoring except one?
- A. To detect the position of the child on the growth curve
 - B. To detects the direction of growth and explores hidden problems
 - C. To makes right decision regarding the interventions required
 - D. To assess of normal growth of infants and young children
100. Women's policy state different issues about women, among these issues which is not correct?
- A. Women shall have equal to men in the enjoyment of human rights
 - B. Women have the right to be free from the influence of HTP
 - C. Women have the right to maternity leave without remuneration
 - D. Women have equal rights as men with regard to marriage
101. Following are the target diseases of EPI except
- A. Tb
 - B. Mumps
 - C. Measles
 - D. Tetanus
102. Vaccine can be stored at health facility level for,
- A. 1 month
 - B. 2 months
 - C. 3 months
 - D. 6 months
103. Status epilepticus is a condition where the seizure last for ,
- A. 5 minutes
 - B. 30 minutes
 - C. 45 minutes
 - D. 60 minutes
104. Following are the causes of Acute Rheumatic Fever except
- A. Previous infection with group B streptococcus
 - B. Substandard health practices
 - C. Overcrowded living condition
 - D. Poor socioeconomic status and living condition

105. Which of the following would the nurse need to keep in mind as a predisposing factor when formulating a teaching plan for child with UTI?
- A. A shorter urethra in females
 - B. Frequent emptying of the bladder
 - C. Increased fluid intake
 - D. Ingestion of acidic juice
106. Which of the following nursing diagnosis would be inappropriate for the infant with gastro esophageal reflux disorder
- A. Fluid volume deficit
 - B. Risk for aspiration
 - C. Altered nutrition less than body requirement
 - D. Altered oral mucous membrane
107. Following are the medical complications of the child who is referred for in-patient therapeutic management of SAM except
- A. Bilateral pitting edema
 - B. Severe vomiting
 - C. Extensive skin lesion
 - D. Fever 38 o c
108. Recommended duration of out-patient antibiotic therapy for child with SAM,
- A. 7 days
 - B. 10 days
 - C. 14 days
 - D. Until therapy finishes
109. The definitive diagnosis for dehydration in marasmic patient is from,
- A. A skin pinch
 - B. Sunken eyes
 - C. History
 - D. Mental status

110. For treatment SAM in infant less than 6 months the nurse must prepare diluted F100 by following technique,
- A. Use 100 ml F100 already prepared and add 35 ml of water
 - B. Use 200 ml F100 already prepared and add 35 ml of water
 - C. Use 100 ml F100 already prepared and add 70 ml of water
 - D. Use 300 ml F100 already prepared and add 35 ml of water
111. Which of the following functions or activities requires recruiting and placing qualified personnel needed for the organization so that it may achieve its objectives and goals?
- A. Planning
 - B. Organizing
 - C. Controlling
 - D. Staffing
 - E. Leading
112. All are the primary responsibility of the first level manager except
- A. Directing the day-to-day activity of the organization
 - B. Establishing organizational goals and strategic plan
 - C. Supervising the work of non-managerial personnel
 - D. None of the above
113. Which of the following is a potential environmental obstacle to delegation?
- A. Standards
 - B. Insecurity
 - C. Inexperience
 - D. All of the above
114. Which of the following characteristics is NOT related to Laissez-faire leadership?
- A. Very little guidance from leaders
 - B. Complete freedom for followers to make decisions
 - C. Leaders provide the tools and resources needed
 - D. There is little time for group decision-making
 - E. Group members are expected to solve problems on their own
115. The Midwife is studying the difference between a manager and a leader. Which of the following is the major difference between managers and leaders?
- A. Leaders do not make good managers
 - B. Managers seldom are leaders
 - C. Managers always have legitimate power
 - D. Leaders always have legitimate power

116. He recalls that the management process involves five functions which follow the following sequence.
- A. Planning, staffing, organizing, controlling, directing
 - B. Planning, organizing, directing, staffing, controlling
 - C. Organizing, planning, directing, staffing, controlling
 - D. Planning, organizing, staffing, directing, controlling
117. Midwife Edris is aware that there are three levels of skills, which he should possess as manager. These are:
- A. Conceptual skills, literary skills, technical skills
 - B. Political skills, conceptual skills, human skills
 - C. Interpersonal skills, technical skills, human skills
 - D. Conceptual skills, interpersonal skills, technical skills
118. A type of interpersonal power that is based on the possession of unique skills, knowledge and competency of the manager is
- A. Reward power
 - B. Referent power
 - C. expert power
 - D. Coercive power
119. Which management function requires managers to make decisions on the four fundamental elements of objectives, actions, resources, and implementation?
- A. Controlling
 - B. Planning
 - C. Organizing
 - D. Leading
120. Organizing involves the following:
- A. Motivating employees
 - B. Defining goals
 - C. Monitoring employees' activities
 - D. Grouping tasks into departments
 - E. Determining appropriate tasks and resources



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TAJAAJILA KENNINU

- ✓ **Barnoota Dabalataa Kennu**
- ✓ **Exit Exam/ COC'f isin qopheessuu**
- ✓ **Leenjiwwan gaggabaaboo kennuu fi kkf**



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